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Liana Haygert Pithan

**PERFORMATIVE RECONFIGURATION OF PROFESSIONAL AUTHORITY
ON DIGITAL PLATFORMS:**

A substantive theory of medical influencers in Brazil

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Liana Haygert Pithan

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Tese apresentada ao Programa de Pós-Graduação em Administração da Escola de Administração da Universidade Federal do Rio Grande do Sul como requisito parcial para a obtenção do título de doutora em Administração.

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BANCA EXAMINADORA

Orientadora – Profª. Dra. Lisiane Quadrado Closs – PPGA / UFRGS

Prof. Dr. Bruno Felix Guilherme de Araújo – PPGA / Fundação Dom Cabral

Prof. Dra. Marcia Cristiane Vaclavik – PPGA / UFRGS

Prof. Dr. Wilson Aparecido Costa de Amorim – PPGA / USP

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RESUMO

Plataformas de mídia social operam como infraestruturas de gestão do trabalho criativo cujas lógicas de dataficação, quando colidem com profissões de alto prestígio e jurisdição protegida por lei, como a medicina, revelam a profunda capacidade desse modelo de negócio de reorganizar o mundo do trabalho. No Brasil, a expansão do ensino médico gerou um excedente de profissionais indiferenciados em busca de diferenciação. Nesse cenário, as plataformas emergem como mercados paralelos de credenciais. Esta tese investiga como influenciadores digitais médicos gerenciam a impossibilidade estrutural de conciliar o Profissionalismo Organizacional de Plataforma (POP), cujo mecanismo central é a Credencial Performativa, e o Profissionalismo Ocupacional Médico (MOP), regulado pelo Conselho Federal de Medicina (CFM) sob o imperativo da Proteção da Autoridade Fiduciária. O estudo se fundamenta na ontologia do Realismo Crítico e no ciclo morfogenético (Bhaskar, 2008; Archer, 2020), operacionalizado por meio da Teoria Fundamentada Construtivista (Charmaz, 2014), dentro da lente analítica da Sociologia das Profissões. A pesquisa examinou materiais instrucionais do Instagram, TikTok e YouTube, documentos normativos do CFM, e analisou sistematicamente os rastros digitais naturalísticos de 26 mega-influenciadores médicos (MMI) brasileiros com mais de um milhão de seguidores em ao menos uma dessas plataformas. Esta tese demonstrou que, quando uma profissão organizada em torno da autoridade fiduciária é submetida à lógica de credenciamento performativo das plataformas de mídia social, a autoridade profissional não declina nem se hibridiza; ela se reconfigura, de forma contínua e performativa, em direções dialeticamente opostas conforme a orientação teleológica do agente. A teoria substantiva da Reconfiguração Performativa da Autoridade Profissional explica que, diante da impossibilidade estrutural de adesão simultânea ao POP e ao MOP, os MMI reconfiguram continuamente sua autoridade como uma performance responsiva por meio de dois vetores teleologicamente opostos: o Vetor 1 (Conversão da Autoridade Profissional em Ativos Comerciais) direciona a ação para a conversão comercial, e o Vetor 2 (Profissionalização da Comunicação em Saúde) se volta para a preservação institucional da profissão. Ambos os vetores mobilizam transversalmente os mesmos Mecanismos Operacionais de Adaptação a Plataformas (OMPA). Essa reconfiguração não é um processo em equilíbrio, pois o Vetor 1 praticado em escala erode as condições institucionais que tornam o Vetor 2 possível, produzindo elaboração estrutural em vez de reprodução da autoridade médica. A dataficação da autoridade médica e a emergência de um sistema paralelo de validação evidenciam que a plataformização reconfigura mesmo a ocupação mais protegida do trabalho especializado.

Palavras-chave: Influenciadores digitais. Plataformas de mídia social. Sociologia das profissões. Teoria Fundamentada. Autoridade médica.

ABSTRACT

Social media platforms operate as infrastructures of creative labor management whose datafication logics, when they collide with high-prestige professions operating under legally protected jurisdictions, such as medicine, reveal the profound capacity of this business model to reorganize the world of work. In Brazil, the expansion of medical education has generated a surplus of undifferentiated professionals seeking differentiation. In this context, platforms emerge as parallel credential markets. This thesis investigates how medical digital influencers manage the structural impossibility of reconciling Platform Organizational Professionalism (POP), whose core mechanism is the Performative Credential, and Medical Occupational Professionalism (MOP), regulated by the Federal Council of Medicine (CFM) under the imperative of Fiduciary Authority Protection. The study is grounded in the Critical Realist ontology and the morphogenetic cycle (Bhaskar, 2008; Archer, 2020), operationalized through Constructivist Grounded Theory (Charmaz, 2014), within the analytical lens of the Sociology of Professions. The research examined instructional materials from Instagram, TikTok, and YouTube, normative documents from the CFM, and systematically analyzed the naturalistic digital traces of 26 Brazilian medical mega-influencers (MMI) with more than one million followers on at least one of these platforms. This thesis demonstrated that, when a fiduciary authority-based profession is subject to social media platforms' logic of performative credentialing, professional authority does not decline or hybridize; it reconfigures, continuously and performatively, in dialectically opposing directions depending on the agent's teleological orientation. The substantive theory of Performative Reconfiguration of Professional Authority explains that, given the structural impossibility of simultaneous adherence to POP and MOP, MMI continuously reconfigure their authority as a responsive performance through two teleologically opposed vectors: Vector 1 (Converting Professional Authority into Commercial Assets) directs action toward commercial conversion, and Vector 2 (Professionalizing Health Communication) is oriented toward the institutional preservation of the profession. Both vectors transversally mobilize the same Operational Mechanisms of Platform Adaptation (OMPA). This reconfiguration is not a process in equilibrium, because Vector 1 practiced at scale erodes the institutional conditions that make Vector 2 possible, producing structural elaboration rather than the reproduction of medical authority. The datafication of medical authority and the emergence of a parallel validation system evidence that platformization reconfigures even the most protected occupation in specialized work.

Keywords: Digital influencers. Social media platforms. Sociology of professions. Grounded theory. Medical authority.

DECLARATION OF ARTIFICIAL INTELLIGENCE USE

The use of artificial intelligence (AI) tools in this doctoral thesis happened in different research stages.

Data construction involved the development of Python scripts, written with the support of Gemini and ChatGPT, to capture data and content produced by the investigated medical influencers on social media platforms, as described in subsection 5.4.3.2.

Claude Pro and QuillBot Premium were used to review the language and assess the coherence, clarity, and cohesion of the arguments expressed in English. More specifically, Claude identified expressions that might lose their intended meaning or sound awkward when translated literally from Portuguese. Each suggestion had to be evaluated individually to determine whether the change was to be implemented or a localized solution was to be developed. After this translation process with Claude, the text was grammar checked by QuillBot. This was an iterative process and involved the manual reading of small sections of the thesis. Such revision ensured the final version remained faithful to the original formulations.

The analytical work remained independent of automated intervention. The coding of social media data, the development of categories, and the formulation of the substantive theory resulted from manual analysis. This effort involved co-authors during Phase 1 and proceeded individually in Phases 2 and 3. Full responsibility for the content, theoretical interpretations, and intellectual integrity of this doctoral thesis rests with the author.

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LIST OF ABBREVIATIONS AND ACRONYMS

- ANVISA** – Agência Nacional de Vigilância Sanitária (Brazilian Health Regulatory Agency)
- API** – Application Programming Interface
- CFM** – Conselho Federal de Medicina (Federal Council of Medicine)
- CGT** – Constructivist Grounded Theory
- CNPJ** – Cadastro Nacional da Pessoa Jurídica (National Registry of Legal Entities)
- CODAME** – Comissão de Divulgação de Assuntos Médicos (Commission for the Disclosure of Medical Matters)
- CRM** – Conselho Regional de Medicina (Regional Council of Medicine)
- CTPS** – Carteira de Trabalho e Previdência Social (Work and Social Security Card)
- DSA** – The European Union’s Digital Services Act
- ENAMED** – Exame Nacional de Desempenho dos Estudantes de Medicina (National Exam for the Performance of Medical Students)
- GT** – Grounded Theory
- GTM** – Grounded Theory Methodology
- IG** – Instagram
- MMI** – Medical Mega-Influencers
- MOP** – Medical Occupational Professionalism
- OAB** – Ordem dos Advogados do Brasil (Brazilian Bar Association)
- OMPA** – Operational Mechanisms of Platform Adaptation
- POP** – Platform Organizational Professionalism
- PROFIMED** – Exame Nacional de Proficiência em Medicina (National Exam of Proficiency in Medicine)
- RQE** – Registro de Qualificação de Especialista (Specialist Qualification Registry)
- SEO** – Search Engine Optimization
- STF** – Supremo Tribunal Federal (Supreme Federal Court)
- SUS** – Sistema Único de Saúde (Brazilian Unified Health System)
- TT** – TikTok
- V1** – Vector 1 (Converting Professional Authority into Commercial Assets)
- V2** – Vector 2 (Professionalizing Health Communication)
- YT** – YouTube

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INTRODUCTION

Industries built on creative work were among the first to absorb the consequences of platform logic, but its reach has since extended to professions with legally protected jurisdictions and consolidated social prestige, reorganizing labor well beyond the sectors where its influence was initially felt. Social media platforms are organizations that manage the productive labor of those who use them, even when that management operates through indirect means. Proprietary organizations inscribe their values and interests into their platforms' codes, which means having the power to define and impose rules on those who produce within them (Dolara; Schrape, 2023). It is within this organizational space that the creator economy has taken shape.

The creator economy is structured around the individuals who create, distribute, and market original content through digital platforms. This activity generated a market that transformed online content production into a structured form of work, remuneration, and entrepreneurship (Ma; Gui; Kou, 2023). The most significant change within this economy from 2019 onward was the rapid growth of the segment oriented toward teaching something, from parenting advice and history lessons to parenting guidance (Hund; Grohmann, 2022). Healthcare professionals are part of this growing group, and their participation intensified from 2020 onward with the COVID-19 pandemic (Farsi, 2021), although the phenomenon of medical digital influencers had already been studied by medical researchers since the 2010s (Alhafez; Rubio-Rico; Diez-Bosch, 2023). Alongside the growth of medical influencers, marketing agencies specializing in this segment have emerged.

The engagement of individuals from highly institutionalized professions in digital content creation corroborates Grohmann's (2021) claim that dependence on digital platforms to obtain work or carry out occupational activities tends toward generalization. The force of this phenomenon is neither anecdotal nor transitory. Medicine is a case in point, and if an occupation with that institutional standing does not escape platformization, this reveals the full extent of this business model's capacity to capture and reshape occupational life. Drawing on this reasoning, this thesis defined medicine as its empirical domain.

Brazilian physicians who are mega-influencers, those with more than one million followers (Hawley; Ismail, 2023), were chosen as the empirical focus because this threshold places them in the elite of the creator economy in terms of monetization expectations. At this scale, the prescriptions of platform logic are structural and continuous. A physician does not reach that level of visibility without sustained alignment with the behaviors that serve

platform business objectives and adherence to the ethos of content creation. This is the condition that places the medical influencer under two competing normative demands at once.

Medical mega-influencers are at the top of a larger stratification of physicians with a digital presence; the structural conditions examined in this thesis do not necessarily apply to physicians in the early stages of audience building or without consolidated monetization.

The physician who operates as a mega-influencer accumulates two roles that pull in different directions. As a physician, the individual follows autonomous conduct norms and exercises discretionary judgment, distinct from professionals subject to organizational control. These represent two distinct normative configurations. According to Evetts' (2013) ideal types, occupational professionalism is a discourse constructed within the professional group, based on shared values and beliefs, technical and ethical principles of conduct, and autonomy to control one's own work. Organizational professionalism is a management control discourse that defines 'professional' behaviors aligned with corporate objectives, defining standardized procedures, performance monitoring, and hierarchical authority. The medical mega-influencer operates in response to both, and their incompatibility is a contingent condition activated when a physician voluntarily takes on both roles.

This thesis therefore poses the following question: how do physicians who simultaneously operate as mega-influencers manage the tensions between the visibility imperatives of platforms and the conduct norms of a highly institutionalized and legally protected profession?

The **general objective** is to investigate how the imperatives of social media platforms bear upon medical professional practice, analyzing how physicians who simultaneously operate as mega-influencers manage the demands of platform visibility and the norms of medicine's institutional authority. To answer this guiding question, the research pursued four specific objectives: (1) to characterize the disciplinary logic and competencies required by the instructional materials of Instagram, TikTok, and YouTube, identifying how they configure an organizational professionalism logic for creators; (2) to reconstruct the normative-regulatory framework established by the CFM governing physician engagement in digital media; (3) to establish the structural incompatibility between the two normative arrangements through systematic comparison of their respective prescriptions; and (4) to construct, from the data, a substantive grounded theory explaining how medical mega-influencers reconfigure medicine's institutional authority as a responsive performance under the condition of structural incompatibility between them.

The study is grounded in critical realist ontology (Bhaskar, 2008; Archer, 2020) and operationalized through constructivist grounded theory (Charmaz, 2014). Critical realism provides the stratified ontological framework that distinguishes observable practices from the generative mechanisms that produce them. Constructivist grounded theory provides the iterative, abductive method for constructing conceptual categories from data, separating structural conditioning from agentic response to examine how institutional norms are instantiated, ignored, or reconfigured.

The investigation unfolded across three research cycles structured by Archer's morphogenetic sequence. The first phase examined the instructional materials produced by Instagram, TikTok, and YouTube for content creators, in collaboration with Willian Fernandes Araújo and Mateus Dalmoro (Pithan; Araújo; Dalmoro, 2025). The second phase examined the normative framework established by the CFM concerning physician engagement in digital media and contrasted its prescriptions with those identified in Phase 1 to establish the structural incompatibility between them. The third phase investigated how 26 MMIs navigate, exploit, and selectively comply with their divergent prescriptions in their digital practices, through the analysis of profiles, metadata, captions, and 58 YouTube video transcripts selected through purposive sampling.

This thesis develops the substantive theory of **Performative Reconfiguration of Professional Authority**. The theory explains that, given the structural impossibility of simultaneous adherence to Platform Organizational Professionalism (POP) and Medical Occupational Professionalism (MOP), medical mega-influencers continuously reconfigure medicine's institutional authority as a responsive performance through two teleologically opposed vectors: one Converting Professional Authority into Commercial Assets, the other Professionalizing Health Communication. Although the two vectors diverge in teleological orientation, both are operationalized through the same set of instruments, demonstrating that the reconfiguration of authority is a continuous and inescapable process of occupational transformation. This reconfiguration is not a process in equilibrium. When Vector 1 is practiced at scale, it erodes the institutional conditions that make Vector 2 possible, producing structural elaboration rather than the reproduction of the medical profession's authority.

The main argument of this thesis is that when digital platforms impose a logic of performative credentialing on a profession organized around fiduciary authority, professional authority does not decline or hybridize; it reconfigures, continuously and performatively, in dialectically opposing directions depending on the agent's teleological orientation.

The originality of this investigation resides in three dimensions. Theoretically, the thesis expands the applicability of Evetts's (2013) typology to digitally mediated work environments. Whereas the author's typology describes two ideal types of control and work organization, this research demonstrates that the same agent, under the same structural condition, orients toward either according to the teleology pursued, producing distinct reconfigurations of professional authority. Methodologically, the sequential analysis of platform prescriptions, occupational norms, and agentive responses across three distinct corpora produces a comparative design capable of isolating the generative conditions that underlie the observed tensions, rather than merely describing their surface manifestations. Socially, the thesis documents a process with direct consequences for public health. The datafication of medical authority, in which legitimacy through performance metrics supersedes legitimacy through credentials, produces structural conditions for the circulation of health information disconnected from scientific consensus at a mega-influencer scale.

Seven chapters organize this thesis. Chapter 2 defines platforms as organizational work management spaces, examining their infrastructure and logics of exploitation, and the invisible management regime that governs content creators. Chapter 3 presents the sociology of professions as an analytical lens, reviewing the neo-Weberian framework centered on concepts of professional project, jurisdiction, and abstract knowledge. Chapter 4 examines the historical formation and regulatory structure of the first professions established in Brazil, with emphasis on medicine as a legacy profession heavily regulated by the state. Chapter 5 describes the methodological journey, which includes ontological positioning, Charmaz's (2014) constructivist grounded theory, and three cycles of data construction and analysis. Chapter 6 presents and discusses the results of the three research phases: section 6.1 maps POP; section 6.2 maps MOP; section 6.3 establishes the structural incompatibility between the two regimes through cross-regime comparison; section 6.4 presents the substantive theory of the Performative Reconfiguration of Professional Authority, detailing the two opposing vectors, the Operational Mechanisms of Platform Adaptation (OMPA) that cut across both vectors, the conditions of the Brazilian medical market that explain why this reconfiguration gains structural traction, and the structural implications of the reconfiguration process for the medical profession as an institution. Finally, Chapter 7 presents the concluding remarks, discussing the implications of the datafication of medical authority for public health.

2 PLATFORMS AS ORGANIZATIONAL SPACES FOR WORK MANAGEMENT¹

Far from being neutral spaces for human interaction, digital platforms are reprogrammable infrastructures organized through data collection, algorithmic processing, monetization, and distribution to facilitate and shape human interactions, actions, and activities (Poell; Nieborg; Van Dijck, 2019), which produce the resources central to the business. Thus, platform owners inscribe their interests and values in technical infrastructure, the general foundation for activities and interaction, becoming rule-setting and rule-enforcing actors (Dolata; Schrape, 2023).

The extent of platforms' political influence is evident in incidents such as the Cambridge Analytica scandal. The digital strategy that influenced the vote for Donald Trump for president of the United States in 2016 was only possible because Meta provided the political consulting firm with data from millions of Facebook users (Cadwalladr; Graham-Harrison, 2018). All phenomena that arise on platforms have an organizational substrate because these platforms manage users' behaviors, actions, and activities to match the business' values and goals.

Platforms actively resist legislation that would regulate them and hold them accountable for what happens in their digital space, influencing public perceptions to boycott discussions and lobbying to dissuade public authorities (Haeck, 2024; Paul, 2024; Rudnitzki; Lobato, 2023). Different countries and regions have different approaches and levels of agility in dealing with it. The European Union's Digital Services Act (DSA) went into effect in 2024, requiring big tech companies to better regulate illegal and harmful content on social media platforms (Udrescu; Jacqu  , 2023). While in Brazil, the *Marco Civil da Internet*, a 2014 law that establishes principles, guarantees, rights, and obligations for Internet use, has become outdated due to rapid changes in the digital environment, including its inability to address social media content and its creators (Ag  ncia Senado, 2024). Platforms are, in practice, the first to draft their rules and guidelines. This thesis uses the terms 'social media platform' and 'platform' interchangeably.

2.1 Social media, labor, and the limits of existing literature

¹ Note on prior publication: This chapter was originally developed as a theoretical essay co-authored with Lisiane Quadrado Closs (Pithan; Closs, 2024). The published version incorporated valuable suggestions from both reviewers, one of whom agreed to have his name disclosed: Samir Adamoglu de Oliveira, from Universidade Federal da Para  ba. The essay has been reformulated to integrate this thesis as a chapter, including the addition of Tables 1 and 2 and connections to Goffman's theory (1982; 2002).

Platforms profit primarily by charging fees for intermediating transactions through software and digital content stores (such as Apple Store and Google Play), e-commerce (like Amazon), and transportation and delivery applications or by collecting and selling user data to advertisers, as exemplified by Facebook, Instagram, YouTube, TikTok, and Google search engine (Kenney; Zysman, 2016). The core feature of the business models of platforms for crowdwork, mobility providers, accommodation, and delivery services is the extensive outsourcing of labor and means of production (Casilli; Posada, 2019; Dolata; Schrape, 2023), enabling platform owners to reduce labor expenses by approximately 30% (Srnicsek, 2017). Using user labor to produce content represents an unprecedented level of capitalism's capacity to exploit non-workplace labor. Users are essential contributors, from user-generated content on social media to peer evaluations on travel or e-commerce websites (Jarrett, 2022).

Even if an organization denies users the status of workers, it must still control them. Therefore, it retains authority over functions such as data collection, service pricing, and revenue collection while delegating responsibility for functions such as working methods, schedules, and performance evaluation to users and complementors (Vallas; Schor, 2020), those who complement products and services provided by platforms (Poell; Nieborg; Duffy, 2022), like drivers and content creators.

There is a rich and growing body of academic work on platforms; however, each research tradition focuses on a specific aspect of platformization (Poell; Nieborg; Duffy, 2022). Business and management studies and economics are foundational research areas on digital platforms, providing valuable insights into the new multi-sided markets, their strategic use, innovation, and service provision intermediation by platforms (such as delivery, ride-hailing, and microtasks). However, they do not include social media among digital labor platforms, nor do they treat creators who are paid or seek to be paid for their digital production as workers (Aloisi; Stefano, 2020; Hagi; Wright, 2019; Keegan; Meijerink, 2023; Kuhn; Maleki, 2017).

Scholars in other fields, such as critical political economy, creative digital labor, and media and cultural studies, have not made this omission. However, while they investigate the power dynamics between platforms and creators, they do not address work management.

Because changes in one dimension of platformization cannot be properly understood without examining related changes in other dimensions (Poell; Nieborg; Duffy, 2022), observing the new occupation of content creation needs to begin by recognizing that its practices, ethos, and identity are conditioned by the organizations from which it emerges and is managed, just as administrators and journalists are conditioned by their industry.

If platforms are not evident workspaces, work management remains unexplored. The well-known term “algorithmic work management” (Duggan *et al.*, 2020; Lee *et al.*, 2015) exemplifies the strategy of merely adding technological adjectives to earlier concepts, lacking a deeper conceptual construct and offering limited analytical value. This term reifies an instrument by making it an agent, obscuring organizational agency; perpetuates the notion of algorithms as inscrutable black boxes (Bucher, 2018); and fails to encompass the hidden management of a workforce not formally recognized as such. Rather than adopting neologisms, Pithan and Closs (2024) employ the concept of governance, understood as the institutionalized coordination that defines relationships, regulates access, structures permitted actions, and provides the legal basis for commercially exploiting users’ activities through terms and conditions (Dolata; Schrape, 2023). This choice reflects a theoretical commitment to examining organizational agency rather than reifying technological instruments.

2.2 Infrastructure, market, and organizations

A platform is a set of techniques, technologies, and digital interfaces provided to users to build social interactions and economic activities on a stable substrate (Kenney; Zysman, 2016). The owning company creates the platform as a mode of socio-technical intermediary and business arrangement (Langley; Leyshon, 2017). It enables, organizes, and directs the value creation of users and complementors. The platform earns revenue from the actions of third parties, either by charging for intermediate transactions or by selling user data. Before the first decade of this century, the web ecosystem was decentralized, with independent websites spread across the public Internet.

Platforms have captured the dispersion of the global digital network, centralizing, controlling, and monetizing the exchanges that take place on them. The platformization of the web, marking the rise of the platform as the dominant infrastructural and economic model of the social web, began with Facebook between 2006 and 2007. Facebook became a platform for third-party developers by giving them access to its users’ data and a set of programming tools, an Application Programming Interface (API). Programmability enables a platform to expand and embed itself in other domains, decentralize data production to complementors, and then recentralize data collection (Helmond, 2015). APIs promote content sharing, connect services, and function as regulatory instruments for platform-to-third-party relationships, business models, and data collection tools. After APIs, “social networks” have become social media platforms.

The primary source of competitive advantage, capital, power, and influence in a platform business is the positive network effect, defined as the exponential growth of a system's value as its user base increases (Parker; Van Alstyne; Jiang, 2016; Vlist, 2022). Five platform firms own the largest, most profitable, and most widely used systems in history by concentrating on web-based intermediation through firms such as Alphabet, Amazon, Apple, Meta, and Microsoft (Vlist, 2022). The platforms' narratives about their long-term surveillance of user habits, fueled by the network effect, give them an alleged unbeatable advantage, preventing the launch of competitors (Giblin; Doctorow, 2022).

There is a distinction between the operating company, its platform, and how the former coordinates, controls, and exploits the latter. Corporations are organizational nuclei whose purpose is to run a profitable business. They establish rules for structuring, selecting, monitoring, and evaluating what happens on platforms. Platforms are social action spaces with a solid technical base and a reliable institutional foundation where actors engage in variable actions and activities that produce resources that the corporation transforms into commodities. The institutionalized mechanisms of coordination, control, and exploitation of social actions and economic processes on the platform link it to the corporation. Non-negotiable contractual relationships, established through detailed terms and conditions set by the corporation, establish these mechanisms (Dolata; Schrape, 2023).

Platform businesses have a unique capability to technically and economically implement widespread commodification of behavioral traces. Profit-driven strategies for valuing aspects of society, particularly people's daily behavior, now allow for previously unthinkable uses (Couldry; Mejias, 2019). According to Dolata and Schrape (2023), the commodification of user behavior takes place in institutionalized coordination, the interplay between the two levels of the platform architecture, the operating company, and its platforms.

According to Srnicek (2017), platform capitalism is a historical extension of capitalism in which data, its surveillance, and extraction serve to train algorithms, enable the coordination and outsourcing of workers, and allow for the optimization and flexibility of production processes. The outsourcing of labor to users is an intensified and radicalized continuation of the deregulation and flexibilization of work and labor relations (Dolata; Schrape, 2023).

Besides monopolies (when sellers have power over buyers), large platforms are also monopsonies (when buyers have power over sellers). In monopsony, a company separates producers from consumers, capturing a disproportionate share of value from the relationships between them. In this chokepoint capitalism, the monopsonists drive the amount paid to

workers and suppliers below what they would receive in a competitive market. Market strangulation, rather than the platform model itself, makes these companies liable for predatory practices (Giblin; Doctorow, 2022).

Platforms enroll users through a participatory economic culture to create multi-sided markets, coordinate network effects, increase scale, and extract revenue from data circulation (Langley; Leyshon, 2017; Srnicek, 2017), positioning the bulk of the workforce as independent contractors rather than employees (Jarrett, 2022). Whoever signs up for a platform accepts its terms and conditions, transferring rights to the data generated to the company (Dolata; Schrape, 2023). Thus, the power relationship between the organization and the other actors on the platform is inherently asymmetrical (Cutolo; Kenney, 2021).

2.3 Structural principles of social media platforms and exploitation logics

Built around the convergence of content sharing, communication, and interpersonal connection (Burgess; Marwick; Poell, 2018), platforms are peculiar value extractors from sociality. Despite rhetorically promising to give users the power of choice, platforms structure the process, format the options, define default settings, and direct users to specific paths, granting third parties an extremely limited version of autonomy while leaving their autonomy intact (Graham; Henman, 2019). As platforms shape and manage permitted activities, they morph into systems that design and manipulate connections.

Platforms' exploration logic is indirect, extracting users' behavioral traces as data to attract advertising revenue (Dolata; Schrape, 2023). The organization creates digital spaces where third parties produce what it sells (Fuchs, 2013), and its technical arrangement gives people the illusion of autonomy while directing them to achieve business goals in the same way that it manages employees (Jarret, 2022).

The four principles of programmability, popularity, connectivity, and datafication structure platforms. "Programmability" means being able to guide and manage what users contribute through set code instructions (Van Dijck; Poell, 2013), allowing them to easily join the platform and start adding value for the business, driving the organic, scalable, and almost unlimited growth of the network effect (Parker; Van Alstyne; Jiang, 2016). Codes direct the flow of communication and prioritize certain types of content over others. The volume of interactions on platforms exceeds the capacity of only human moderators to ensure compliance with public legislation and platform guidelines, mandating automated moderation (Katzenbach; Ulbricht, 2019). Therefore, the programmed architecture mediates content,

experiences, and relationships (Van Dijck; Poell, 2013), which impacts the likelihood that audiences will find and engage with posts (Beer, 2013).

“Popularity” is the power of agenda-setting or pushing certain topics to the forefront. It is evident in rankings and recommended content, whose metrics reflect the platform’s measures and are sensitive to user actions to promote or denounce a specific item. Underlying this principle is scale, where the more popular a post, the greater the number of users (data producers) it attracts. The time users spend socializing on platforms is surplus production and, therefore, unpaid labor (Jarret, 2022).

“Connectivity” refers to the socio-technical capacity of platforms to connect content to user activities and advertisers. Even if users have considerable influence over content contributions, platforms always mediate and define how connections take shape. Connectivity is a strategic tactic that effectively enables human connectedness while pushing automated connectivity. Finally, “datafication” consists of the translation of life and human attitudes into quantifiable and analyzable digital data, based on the capacity of network infrastructures to convert into numerical records various aspects of the world that were previously unmeasured (Van Dijck; Poell, 2013). This translation, in turn, gives platforms the ability to develop predictive and real-time analytics techniques to track, diagnose, and predict behavior, allowing for the immediate redirection of content and advertising to participants (Lemos, 2021).

2.4 Platform governance as reflective and relational coordination

Platform-owning organizations claim that they are already self-regulating and that external regulation would harm the economy or freedom of expression, fueling opposition to the implementation of local laws (Rudnitzki; Lobato, 2023). However, platforms are largely responsible for the dissemination of hate speech and disinformation, failing to moderate such content, according to Ceylan, Anderson, and Wood (2017). Their study of Facebook users reveals that 15% of the most frequent sharers spread 30–40% of fake news because the social media’s algorithmic recommendation system boosts the visibility of users who share content frequently, whether sensationalist or false. Political and social falsehoods often remain online under the guise of protecting free speech, while lies about other topics, such as health, are more easily eliminated (Waddell, 2020). This regulatory vacuum is not incidental, it is structurally produced by the same asymmetry of power that governs the relationship between platforms and the workers who populate them.

As platforms have spread, so has their power to reorganize, disperse, and recompose work. Their self-definition as technology companies that facilitate transactions between independent workers and clients, rather than employers who control their employees, hides a concentrated power and information asymmetry that exists in their relationship with workers (Garcia-Calvo; Kenney; Zysman, 2023). Platformization (the increasing reliance on platforms for getting, doing, and maintaining work) tends to generalize as various labor activities rely more heavily on digital infrastructure and its logic (Casilli; Posada, 2019; Grohmann, 2020).

Garcia-Calvo, Kenney, and Zysman's (2023) systemic perspective on platform work expands previous views, including a wide range of legacy industries affected by platforms. From their sociological perspective, there are three broad categories of platform workers. The first includes personnel and contractors directly tied to the platform company. The second encompasses work mediated by the platform, such as service providers hired and paid through the platform (e.g., digital influencers and app developers), vendors who sell goods or services either in person (e.g., Uber and Airbnb) or remotely (e.g., Upwork), content creators who may become paid if successful, and prosumers, that is, everyone who interacts with a platform and incidentally provides data. The third category involves work affected by platforms, including professional services such as advertising and journalism, indirect workers who assist content creators (e.g., lawyers, scriptwriters, web designers), workers in other service sectors (e.g., restaurants and real estate), and manufacturing sectors transformed by platforms.

Given the structuring principles of the platforms (programmability, popularity, connectivity, and datafication) and the scale of human labor mobilized, management is mostly automated. What makes this possible are algorithms, defined as mathematical instructions coded for transforming input data into a desired output, built on specific interests, strategies, policies, and stakeholders (Gillespie, 2018a), reflecting the beliefs, biases, values, and cultural assumptions of those who write them (Bucher, 2018). Algorithms shape where and to whom data flows as a new type of power that operates within people's lives (Beer, 2013).

The two types of algorithms are pre-programmed, which always produces the same output for a given input, and machine learning, which learns from examples to change behavior, improve performance, and predict outputs (Bucher, 2018). On social media platforms, every click and every query increases the tool's functionality, allowing algorithms to better tailor content and advertisements to individual users' preferences. Although algorithms play a vital role, they cannot function without user-generated data (Gillespie, 2018a), implying that they are powerful but still just tools.

It is necessary to avoid the simplistic technological determinism as a driver in explaining phenomena; instead, it is necessary to examine the human and institutional choices behind the mechanisms (Gillespie, 2018a). Therefore, Pithan and Closs (2024) argue that the technological fetishism (Feenberg, 2001; Harvey, 2003) inherent in the concept of “algorithmic work management” is detrimental to understanding peculiar platforms’ work management. Furthermore, it reinforces the perception that the algorithm is a black box. This metaphor favors ignorance instead of knowledge, serving as “strategic unknowns” for companies to avoid liability when platform issues arise (Bucher, 2018, p. 56). New terms are unnecessary when governance suffices.

Hofmann, Katzenbach, and Gollatz (2017) argue that the erroneous association of governance and regulation considers only a limited set of ordering processes and interprets existing rules and institutions as functional outcomes of problem-solving activities. While regulation includes operations aimed at influencing a specific behavior in an area to produce a predictable outcome through standards, regulatory infrastructures, and coordination, the authors characterize governance as reflexive coordination, focusing on “critical moments” when routine activities become problematic and require revision. Thus, regular coordination necessitates coordination, emphasizing interaction, cooperation, and negotiation to potentially redefine the rules (Hofmann; Katzenbach; Gollatz, 2017).

Platform governance is also relational because software-coded institutional logic is social, capable of shaping behaviors, preferences, and content creation, while being redefined by the responses governance provoked (Napoli, 2014). These codes emerge from various relationships; they communicate or act on behalf of someone or something, and their existence is always relational (Laapotti; Raappana, 2022).

Gorwa’s (2019) more straightforward definition of platform governance recognizes three primary forms. These are governance *by* platform (self-governance), governance by external parties (governance *of* platform), and multi-stakeholder governance (co-governance). The last one is a combination of the previous two, in which the company forms councils to gather input from the government and civil society. Governance *of* platforms through specific laws is uncommon, as they typically govern themselves before regulators intervene. This approach does not imply that platforms have unrestricted power, as they are subject to regulatory scrutiny and must follow local laws. As long as they respond to public pleas and court orders to remove harmful and criminal conduct, platforms are not liable for user misbehavior. Nonetheless, Gorwa (2019) argues that these responses seem to be strategically crafted to avoid potential external regulation while preserving the highly profitable status quo.

Platforms establish their own rules and regulations for digital spaces, but stakeholders also influence them, making governance a complex relational construct (Vlist, 2022). As a result, notions of platforms as self-governing and open to reflective coordination coexist.

2.4.1 Governance strategies: Regulation, curation, and moderation

Platform services such as social media significantly influence and mediate individual behavior, so they engage in governance at the individual user level (Gillespie, 2015). As we are dealing with individual-level management here, it is ‘governance by platforms’ that manages users’ work. This governance is operationalized through three independent, overlapping, and constantly updated strategies of regulation, curation, and moderation.

Regulation sets the formal and technical arrangement in which cultural production takes shape by establishing documented norms such as Terms of Service and Privacy Policies, as well as operational documentation (Gillespie, 2018b; Siciliano, 2021; Poell; Nieborg; Duffy, 2022). Table 1 presents the types of documents that comprise the regulation, categorized by this thesis according to their main legal, disciplinary, and operational functions. They work together to structure access, prescribe acceptable conduct, and orient activity on the platform. Community standards, the most visible disciplinary mechanism, prioritize behavioral regulation over content (Bateman; Thompson; Smith, 2021), whereas instructional materials guide content creators, emphasizing audience metrics and entrepreneurial alignment (Araujo, 2021; Siciliano, 2021).

Curation refers to topics’ categorization and selection by platform to steer user interaction. Traditional curation relies on experts and institutions to uphold cultural norms and professional conduct, while automated curation combines algorithm-driven and user-driven selection. According to their techno-commercial strategies, platform algorithms trigger and filter user activity (choices, clicks, searches, and shares) to influence content, service, visibility, and availability (Van Dijck; Poell; Waal, 2018). Curation illustrates how data-hungry platforms’ business models relate to social phenomena. While extremist content recommendations are rare for casual viewers on YouTube, they are common for those who already consume it (Chen *et al.*, 2023), steering them toward a narrower ideological range (Brown *et al.*, 2022) and creating echo chambers for political extremism, conspiracy theories, white nationalism, the alt-right, and QAnon (Haroon *et al.*, 2023). Because automated curation by algorithms prioritizes user engagement and permanence, it feeds them more of what they already watch, regardless of content.

Table 1 — Types of regulatory documents and policies

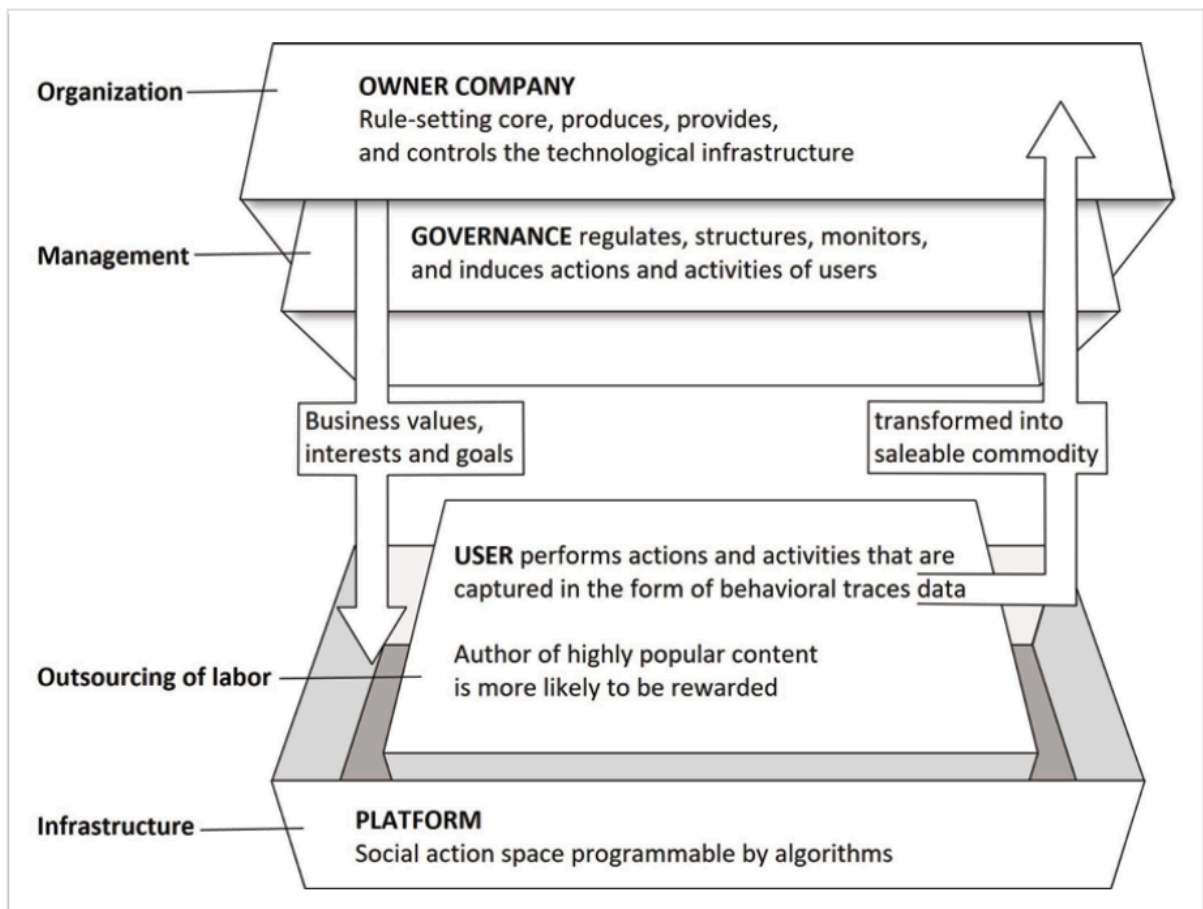
Legal and Contractual Documents	
Terms of Service (ToS)	This is the legal basis for the platform-user relationship, establishing guidelines and limiting corporate liability. Violation of these terms may result in account suspension or termination. They are legally binding, and acceptance is required for using the platform (Belli, 2021).
Privacy Policy	It describes how the platform collects, uses, stores, and shares users' personal information. It specifies the types of data collected, their uses, and who may have access to them (Cunningham; Craig, 2021).
Cookies Policy	It explains how tracking technologies are used to monitor user activity and maintain continuous surveillance of browsing habits and preferences (Bucher, 2018).
Regulations and Discipline	
Community Guidelines	It addresses the rules of behavior and content that users must follow, including prohibitions and regulations. Violations result in penalties, including permanent bans (Cotter, 2019).
Monetization Policy	The platforms establish explicit eligibility criteria for revenue sharing, create advertiser-friendly standards with more stringent rules, and exclude controversial or sensitive topics even if they do not violate general community guidelines (Caplan; Gillespie, 2020; Hödl; Myrach, 2023).
Technical and Operational Documents	
Recommendation Systems Description	Platforms provide details about how algorithms select and order content in user feeds. These descriptions emphasize watch time, relevance, and engagement to influence creator strategies (Ducato, 2021).
Creator Directions	Platforms educate creators by providing dedicated learning centers, tutorials, and best-practices guides. These instructions specify platform-beneficial behaviors (Araujo, 2021; Siciliano, 2021; Pithan; Araujo; Dalmoro, 2025).

Finally, moderation structures content availability, consisting of pre-screening, rejection, removal, banning, demotion, or demonetization of content and accounts. This eliminates violations of the platforms' regulations and content that cause stakeholder shock or outrage (Gillespie, 2018b; Poell; Nieborg; Duffy, 2022), whether reported by users themselves or ordered by a court. Platforms are often criticized for moderating too little, neglecting serious cases, or too much, filtering out historically or culturally significant topics (Van Dijck; Poell; Waal, 2018). In countries without specific laws, for example, authorities must persuade

platforms to fight fake news, even when it harms victims of a tragedy (Strickland; Doria, 2024). Given this complexity, human moderation can never be fully replaced, but companies increase automation (Gorwa; Binns; Katzenbach, 2020; Wright, 2022) and ask their users to flag inappropriate content (Van Dijck; Poell; Waal, 2018).

Pithan and Closs (2024) depict social media platforms as an organizational form of production, management, and work in Figure 1. The owning corporation defines the social rules governing the platform in accordance with its values, interests, and objectives, giving it an action-oriented institutional basis. Governance regulates, structures, monitors, and induces actions and activities of users' activities by inducing them to take actions that leave behavioral traces, whose data are captured and transformed into saleable commodities, especially to generate advertising revenue. Therefore, the primary business resource originates from the outsourcing of productive tasks to users, who are rewarded only when their "productivity" is immense.

Figure 1 — Social media as an organizational form of production, management, and work



Source: Pithan and Closs (2024, p. 14)

2.5 Invisible management and visibility-driven work

Content creators are those who try to make a living by producing digital content across multiple platforms, learning how to deal with innovative technologies and algorithms, and negotiating their identities within commerce circuits, all while building a community of followers around themselves and their activities (Arriagada; Ibáñez, 2020). A notorious subgroup of content creators is digital influencers, the quintessential internet celebrities with positive notoriety, talent, and skills (Abidin, 2018); thousands of admirers; and a personal brand produced as a commodity (Karhawi, 2016), who are successful in monetizing their online presence (Duffy, 2020b; Arriagada; Bishop, 2021).

Self-branding (process) involves constructing and adjusting one's public self-presentation based on market analysis and audience feedback to position and maintain a positive image compatible with the desired reputation (Gorbatov; Khapova; Lysova, 2018). A self-brand or personal brand (product) is a professional distinction that generates a visible and measurable reputation, favoring advertising contracts, paid audience memberships, and product sales (Brooks; Drenten; Piskorski, 2021). To convert visibility into income, self-branding is operationalized through identity self-presentation strategies and the building of communities around themselves and their content, based on periodicity, thematization, and a publicly attributed and recognized reputation (Karhawi, 2021).

Through Goffman's (2002) dramaturgical framework, self-branding can be understood as the strategic management of impressions in which creators execute identity work for dispersed audiences. Unlike traditional face-to-face interactions, in which performers can adjust their presentations based on immediate feedback, platform creators face what Goffman (2002) refers to as 'audience segregation' challenges, in which they must address multiple audiences (followers, potential sponsors, the platform's automated system, and professional peers) whose expectations may conflict. Creators are also required to perform relational labor because establishing affective connections with audiences (performing authenticity, vulnerability, and accessibility) transforms passive viewers into engaged consumers (Baym, 2018). Furthermore, emotional labor is central to the feminized work of digital influence, where the appearance of genuine intimacy is itself the commodity that sustains monetization (Duffy; Wissinger, 2017).

According to Sedda and Husson (2023), in platform capitalism, visibility becomes the primary object of work activity instead of a byproduct of labor. For content creators, managing their image and pursuing metric visibility are examples of aspirational labor (Duffy, 2016), in which current effort is invested in the hope of future recognition and monetization.

Most aspiring content creators seek enough visibility to turn it into direct compensation through monetization on platforms or an indirect reward through agreements with brands (see Table 2) or job opportunities outside the digital sphere (Villegas-Simón, 2022). In the attention economy of platforms, the currency is platform metrics, as creators need verifiable evidence of their ability to influence (like subscribers, followers, comments, and reactions) to monetize (Duffy, 2020b). So, platform metrics function as “sign equipment” (Goffman, 2002, p. 66), the visible evidence that validates a performer’s claimed identity. For content creators, follower counts, engagement rates, and verification badges provide tangible proof of their capacity to influence.

Table 2 — Instagram, TikTok, and YouTube revenue sources and monetization mechanisms

Sources of Revenue	Revenue Mechanisms	IG	TT	YT
Earnings from platforms	Revenue sharing from advertising			X ¹
	Performance-based incentives, bonuses for content	X ²		
	Incentives, bonuses for high-quality content		X ³	
	Incentives, bonuses for effect creation, popular music usage		X	
Fan payments	One-time support (virtual gifts, badges, financial values)	X	X	X
	Monthly payments (subscriptions / memberships)	X	X	X
	One-time paid content (single purchase of content packages)		X	
Brand partnership	Branded Content / Direct Advertising	X	X	X
	Affiliate Marketing / Third-Party Product Sales	X	X	X

¹ Watch Page, Shorts, and YouTube Premium revenue sharing

² Available only in the United States and South Korea as of the date of reading (January 5, 2026)

³ Based on thousands of views and engagement metrics

Creative expression is difficult to measure and control, so it requires unusual management approaches that organize the work activity and simultaneously preserve laborers’ illusion of autonomy. Although the organization does not formally impose rules about work hours and pace, they do exist. The “invisible management” operates indirectly, such as through guidelines, policies, and instructional material. These mechanisms control user work by prescribing the scope of acceptable activity and punishing violators, functioning just as well as daily task assignments in a “Taylorist office” (Jarrett, 2022, pp. 74-76).

While these three governance strategies overlap in practice, examining their distinct influences on creative work reveals how platforms exercise control through multiple, reinforcing mechanisms.

Influence of Regulation: Visibility is presented as a value achieved only by obeying the norms designed to govern behavior. While disciplinary rules, such as the Community Guidelines, serve as a warning that violations will result in invisibility, regulatory and instructional tools identify behaviors that can increase visibility. In their instructions, platforms recommend that creators tailor their content to audience trends to gain visibility and equate work worthy of payment with viewers' gains, encouraging production under more profitable standards for their businesses. By presenting metric results as a mere reflection of audience choices, platforms commit the "algorithm fallacy," as Araujo (2021, p. 30) calls the discourse that covers up the algorithm's bias and the organizational objectives. Frequent updates to monetization policies, guidelines, and tools lead to insecurity and constant relearning through trial and error (Arriagada; Ibáñez, 2020), which increases creators' vulnerability and asymmetry between effort and reward (Burgess; Green, 2018; Cunningham; Craig, 2021) and makes overworking inevitable (Arriagada; Ibáñez, 2020). Duffy (2020, p. 103) refers to this structural instability as "algorithmic precarity," which is at most momentarily viable. For influencers with institutionalized and regulated primary occupations, this precariousness is aggravated by the simultaneous incidence of the professional body's sanctioning system, which operates with different temporal and normative logics.

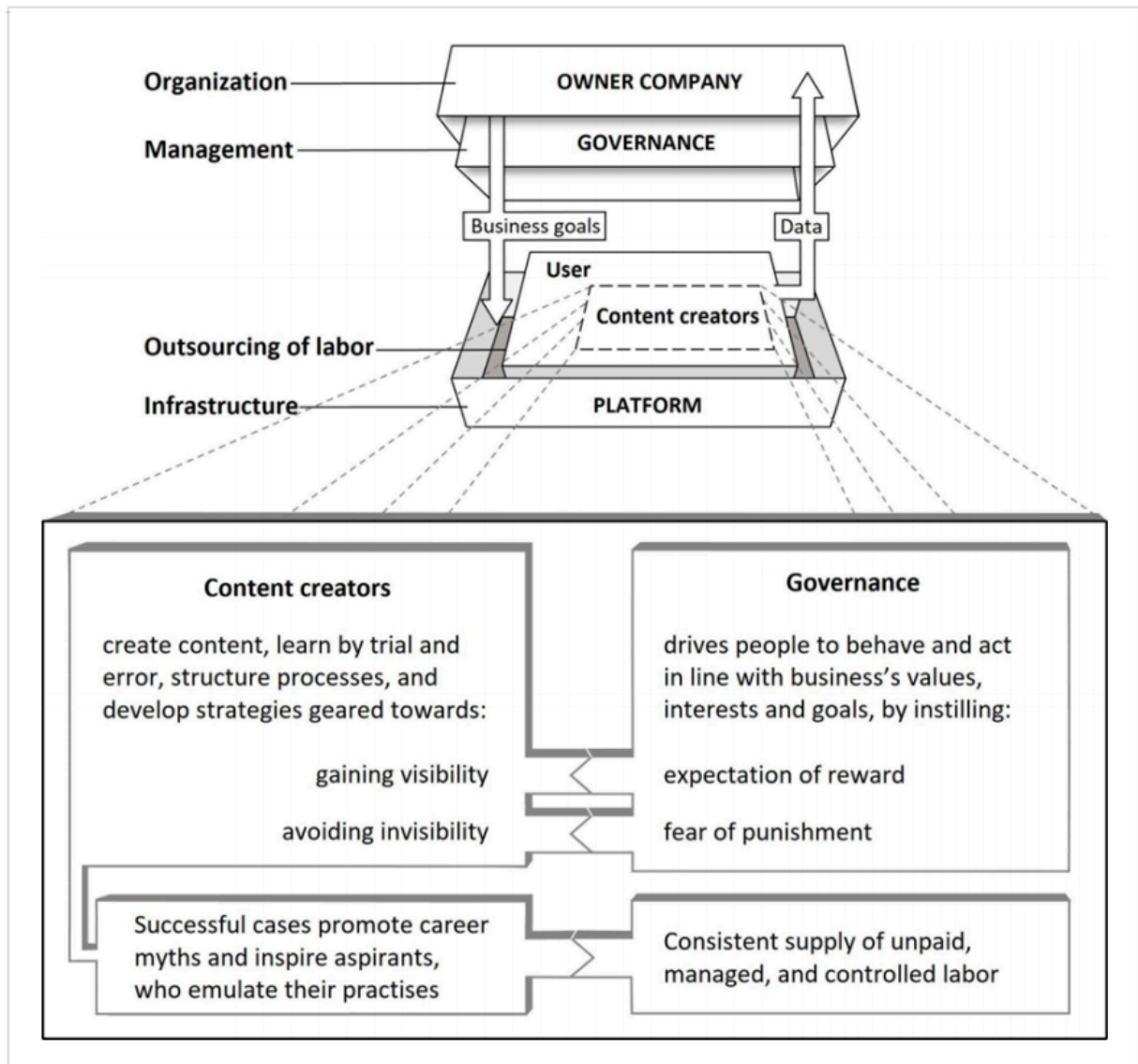
Influence of Curation: The automated recommendation and ranking system's criteria determine whether content is successful or not. This shapes creative work around metric visibility and prioritizes what platforms can measure over reputation (Abidin, 2018; Villegas-Simón, 2021). Because its operation is still opaque, creators build mental models of how it works (Bucher, 2018; Bishop, 2019) and, in a constant effort to decode it, adjust their aesthetic choices, production rhythms, and even the way they interact with audiences to fit what they think curation favors (Cotter, 2019; Duffy; Meisner, 2023).

Consistency and frequency become prerequisites for platform-driven visibility, and overwork is once again normalized. Even when creators try to "beat the algorithm," they are defeated because their strategies ultimately entail producing with the increased frequency desired by the organizations (Arriagada; Ibáñez, 2020, p. 9). According to Pithan and Closs (2024), curation is an effective organizational control tool that fosters self-management, self-control, and self-exploitation. While the paid traffic doesn't generate direct compensation through reward programs (Instagram, 2024; TikTok, 2024; YouTube, 2024), it does boost discovery. This mechanism reveals the double exploitation by the platforms since they charge for visibility while simultaneously commodifying the attention these creators have purchased.

Influence of Moderation: The opaque way moderation rules are applied makes creators monitor and censor themselves, changing video metadata to avoid triggering automatic moderation and blocking comments to prevent punishment for messages from the public (Ma; Kou, 2021). On the other hand, creators use resistance tactics that platforms call manipulation, such as clickbait titles, search engine optimization, bots and automation software, and the organization of groups for reciprocal engagement (O'Meara, 2019; Poell; Nieborg; Duffy, 2022). Platforms condemn the use of click farms, which sell engagement (Grohmann *et al.*, 2022), but some tactics, such as SEO (Search Engine Optimization) and clickbait, are on the verge of acceptance (Poell; Nieborg; Duffy, 2022). While promising to punish creator-invented strategies for increasing visibility, platforms provide automated visibility boosts when creators pay to convert content into an advertisement (Petre; Duffy; Hund, 2019). Minority creators (non-white individuals, LGBTQI+ people, women, and those without large followings) report being disproportionately harmed by automated moderation (Duffy; Meisner, 2023), suffering shadowbanning, a sudden drop in their visibility without explanation (Cotter, 2019), and having content removed for coordinated complaints from other users who stalk them (West, 2018).

Celebrity creators who successfully navigate governance strategies to gain visibility (and avoid invisibility) play an active role in perpetuating the supply of free labor. According to Duffy and Wissinger (2017), these champions of data productivity portray the career as fun, free, authentic, and viable, propagating the “creativity dispositif” that establishes models, norms, and standards while shaping aspirant expectations of achievement. These mythologies, highlighting pleasurable aspects while concealing negative ones, play an ideological role by influencing aspirants to join the pool of free labor in anticipation of future rewards. Governance thus directs labor through reward promises and punishment threats, sustaining the digital capitalism cycle. Figure 2 depicts how governance guides workers through the promise of rewards and the threat of punishment (Pithan; Closs, 2024).

Figure 2 — Content creators' behavior in response to governance



Source: Pithan and Closs (2024, p. 18)

2.6 Artifactual and contractual power in platformized management

Kenney, Bearson, and Zysman (2021) identify two key aspects of platform power, namely the artifactual, inherent in software and algorithms, and the contractual, defined by terms of service, guidelines, policies, and instructions. Pithan and Closs (2024) point out that the artifactual and contractual aspects are intertwined in governance strategies to meet business needs in two ways, with the first being that platforms allow self-management without outside interference. Platforms have unparalleled autonomy because they lack the accountability and regulatory obligations that legacy media companies face, and they are not bound by formal employment relations with their core workforce of content producers. Yet stakeholder concerns about harmful or criminal user behavior must be addressed to preserve

legitimacy and advertiser confidence. This is accomplished through regulation (establishing rules and prohibitions) and moderation (threatening sanctions for violations), implemented primarily through automated systems to minimize costs while maintaining the appearance of responsible governance.

Second, platforms increase data productivity by maximizing user retention, engagement, and advertiser satisfaction. This is achieved through curation (algorithmic recommendation systems that condition visibility on specific user behaviors) and by tying creator rewards explicitly to metric performance. What platforms can measure becomes what creators optimize for, and compliance with platform regulations becomes mandatory to avoid algorithmic penalties or outright demonetization.

Content creators exemplify these dynamics particularly clearly. As detailed in section 2.6.1, the three governance strategies converge on a single structural binary: visibility is promised to those who comply and withheld from those who do not. Through experimentation with regulation, curation, and moderation, creators develop artifact-specific strategies that ultimately serve platform productivity goals. This gives rise to the practices, values, and discourses that shape the new occupation.

These practices, values, and discourses constitute the occupational culture of a new form of work. Content creators are not employees, yet they are subject to organizational control. They possess autonomy over creative choices, yet these choices are constrained by algorithmic and contractual governance, which can limit their ability to fully express their creativity. They claim professional status but lack the institutional foundations (formal credentials, collegial control and ethical codes developed by practitioners) traditionally associated with professions.

Understanding the professional identity and behaviors of content creators requires handling ambiguity. The sociology of professions addresses occupational autonomy, organizational control, and the conflict between professional self-regulation and management discipline. However, classical professionalism, developed to explain the power and privileges of established professions like medicine and law, must be adapted to platform ecosystem jobs. The following chapter examines how the sociology of professions theorizes this tension, establishing the analytical foundation for understanding physicians who navigate incompatible professional regimes on social media platforms.

3 THE SOCIOLOGY OF PROFESSIONS AS AN ANALYTICAL LENS²

Chapter 2 established platforms as organizational spaces that manage creative labor through governance. Revising the neo-Weberian analytical framework centered on concepts of professional project, jurisdiction, and abstract knowledge, this chapter examines how such organizational pressure threatens professional autonomy. The sociology of professions is a useful lens here because it shows that professional status is vulnerable to external forces (Larson, 2017) like technology and organizations (Abbott, 1988).

As explained in the previous chapter, platforms are infrastructures of management and power. Social media platforms force traditional medical authority, grounded in credentials and educational legitimacy (Freidson, 1989), to coexist with digital authority based on performance metrics. If prevalent, the logic of the platforms destabilizes the professional autonomy of judgment (Freidson, 2007) and the jurisdictional unity of medicine (Abbott, 1988), as values aligned with the business logic of social media infiltrate even the most established professions.

Medical influencers respond to a dual professional regime. As content creators, they align with social media governance and the organizational professionalism these platforms prescribe. As physicians, they adhere to the values, beliefs, and ideologies of their occupational professionalism (Evetts, 2013). This dual role creates tension because the criteria by which a physician earns authority in clinical practice, credentials and discretionary judgment, differ from, and at times contradict, the criteria by which platforms reward medical content, engagement metrics and posting frequency.

However, professions cannot disregard platforms as digital arenas for social action, especially when professionalism is perceived as a performance (Goffman, 2002) manifested through interaction among professionals, clients, cases, stakeholders, and context (Noordegraaf, 2020). From Goffman's (2002) dramaturgical perspective, professional identity is an ongoing accomplishment. Physicians must continuously 'perform' competence, authority, and trustworthiness through situated interactions. Therefore, responding to public demands for free information and transparency gives professionals the opportunity to claim moral and cognitive authority in the digital space, fostering public trust. Actively participating in social media would demonstrate, in the words of Abbott (1988), a profession's ability to

² Note on prior publication: Excerpts from this chapter were formulated for three works previously presented in conference proceedings (XXV SemeAd 2022, EnANPAD 2024, and AoIR2025), co-authored with Lisiane Quadrado Closs, Wilian Fernandes Araujo, and Mateus Dalmoro (Pithan; Closs, 2022; Pithan; Closs, 2024; Pithan; Araujo, Dalmoro, 2025). Therefore, although it has a unique structure and exclusive formulations, this chapter may contain phrases already used in these papers.

adapt and survive.

The sociology of professions provides essential notions for understanding the vulnerability of the professional status and that adapting to social transformations is both a form of resistance and a risk. This theoretical foundation elucidates how outside pressures from organizations can greatly affect professions, revealing that values connected to platform business logic have even affected the most entrenched professions.

This review sets the relevance of neo-Weberian concepts for analyzing how external forces threaten professional control. The chapter then examines the contemporary shift from analyzing professions as a structural category to professionalism as a normative process, particularly as it is shaped by organizational contexts. Lastly, it addresses how professionalism is put into practice and controlled through competence frameworks, setting the methodological foundation for the empirical analysis.

3.1 Functionalism and symbolic interactionism in early frameworks

The social sciences have long attributed special significance to professions, as studying how they reflect and shape aspects of society, such as the division of labor, authority, identity, and ethics, is fundamental to the interpretation of social dynamics and interactions between individuals and institutions (Kurtz, 2022). For Émile Durkheim and Max Weber, occupations and professions were important phenomena to comprehend modern society's structures (MacDonald, 1995; Rodrigues, 2002; Kurtz, 2022). While Durkheim viewed occupational groups as a link between individuals and society, Weber focused on the influence of work ethic on contemporary capitalism (Dubar; Tripier; Boussard, 2015; Schmitz, 2014).

Functionalism initially predominated in the discipline (until the end of the 1960s) and coexisted with the symbolic interactionism of the Chicago School, followed (in the 1970s) by a critical movement opposing the functionalist view (MacDonald, 1995; Rodrigues, 2002; Saks, 2016). Functionalism addresses professions in terms of social differentiation, role division, and socialization. These special occupations are considered ethically positive entities that embody core societal values and incorporate socially functional traits such as altruism and are therefore considered a moral basis for modern society (MacDonald, 1995, p. 2).

The functionalist Talcott Parsons, whose work drew on Weberian concepts, was the first to demonstrate the relationship between the capitalist economy, the rational-legal social order, and modern professions (Rodrigues, 2002; Evetts, 2013). He conceptualized the professional role through dimensions that articulated social norms and cultural values, such as dual competence involving both practical and theoretical knowledge and a sense of

disinterestedness that combined affective neutrality with client-oriented service. Parsons argued that, despite having the same rational-legal principles as the managerial hierarchy of bureaucratic organizations, professions differ by seeking a shared normative end through collegiate organization (Evetts, 2013; 2018).

Although Parsons went beyond the taxonomy of other functionalists, he was criticized for omitting conflicts based on interests and professional power as drivers of change (Saks, 2016). Furthermore, his work was predominantly deductive, lacking empirical grounding, and his conceptual system often justified the status of established professions based on the ideology of those same groups (Dubar; Tripier; Boussard, 2015).

The interactionists' procedure runs counter to the functionalist approach. Symbolic interactionist Everett Hughes, through empirical research conducted with his students at the Chicago School from the 1940s onward, fundamentally challenged the functionalist model by questioning its valorative assumptions (Rodrigues, 2002). Hughes (1964, p. 44-45) viewed professions as "not so much a descriptive term as one of value and prestige," rejecting the false question of whether an occupation qualifies as a profession. Instead, he investigated the circumstances under which occupational groups attempt to transform themselves into professions.

His research demonstrated that all occupations (from physicians and lawyers to janitors and exotic dancers) develop collective pretensions, systems of mutual protection around mistakes, and claims to dignify their work, though they differ in their success in obtaining societal recognition (Hughes, 1964; 1993). In this perspective, Hughes's followers analyze professionalization as a process of mobilizing resources to ascend the occupational hierarchy by obtaining the honorific title of a profession and achieving what Hughes (1993) describes as occupational mobility. This processual approach introduced key analytical concepts such as 'license' or the authorization to practice; 'mandate,' which represents the mission and obligation to define proper client conduct (Hughes, 1993); 'career,' understood both as an objective sequence of positions and a subjective self-interpretation (Hughes, 1937); and 'guilty knowledge,' which involves handling the sacred and taboo matters of society (Hughes, 1964; 1993).

Expanding the procedural framework, Rue Bucher and Anselm Strauss conceived of professions as flexible professional segments in constant flux, rather than homogeneous communities sharing unified identities and interests, as the functionalists assumed (Dubar; Tripier; Boussard, 2015; Rodrigues, 2002). These segments are defined by divergent missions, methodologies, clients, and career paths, and this internal diversity leads them to

continuous conflict over jurisdictional boundaries, resources, and the legitimate mandate of the profession. Freidson (1996) identified the structural source of this tension as a division between a cognitive elite of academic authority and practitioners in their daily work. Because these groups operate under different standards, there is inherent conflict within the professional structure. In addition to reflecting status variations, internal differentiation serves to generate or absorb disturbances in interprofessional relationships, according to Abbott (1988), allowing professions to respond to external challenges with internal reconfigurations instead of frontal confrontation.

From an interactionist perspective, professionalization involves internal differentiation marked by hierarchies among elite groups of researchers in universities, professionals, and administrators, each group claiming to represent true professionalism while marginalizing the others (Dubar; Tripier; Boussard, 2015; Freidson, 1996; Rodrigues, 2002). While these interactionist insights remained focused on workplace negotiations and internal professional dynamics (Saks, 2014; 2016), subsequent theorists, such as Freidson (2007) and Larson (2017), expanded this processual logic to analyze broader public and legal contexts, shifting the unit of analysis from the profession itself to interprofessional relations, as Abbott (1988) does. The shift from internal negotiations to interprofessional relations requires a broader approach to power.

Interactionism anticipated power-centered perspectives by exposing how functionalists reified professional self-representations (Hughes, 1964). However, MacDonald (1995) and Saks (2016) believe the interactionist view is limited by its micro-focus on workplace negotiations and the subjective construction of meaning, overlooking larger power dynamics, historical processes, and how state recognition affects professional success. Although interactionism's emphasis on professionalization as a strategic and symbolic action has influenced later theories on professional power and social closure, analytical tools are insufficient to explain why some occupational projects secure state-sanctioned monopolies while others fail. Since the 1970s, authors have focused on this issue, shifting the emphasis from microsociological to macrostructural analyses of power and social closure (Saks, 2010).

The evolution of the discipline demonstrates that professions are projects vulnerable to external forces. For medical influencers, one form of exposure to vulnerability is the governance logic of platforms, potentially incompatible with medical professionalism. The next section explores the neo-Weberian framework, which offers the analytical vocabulary needed to examine the tension between professional projects, autonomy, jurisdiction, and knowledge.

3.2 Neo-Weberian perspectives on closure, monopoly, and control

Critical perspectives in Anglo-American sociology of professions encompass diverse paradigmatic and methodological orientations. Although this heterogeneous body can be classified as a power paradigm (Rodrigues, 2002), this thesis adopts the neo-Weberian designation used by Dubar, Tripier, and Boussard (2015), MacDonald (1995), and Saks (2016). Neo-Weberian authors draw on symbolic interactionism and Marxism, yet analytical tools from Weber's work remain central, enabling adaptability across organizational contexts without constituting theoretical inconsistency (Dubar; Tripier; Boussard, 2015).

Weber rarely used the term "profession," conceiving professionalism as associated with rational action in relation to ends and values (Schmitz, 2014, p. 22), which provides the basis for understanding professions as groups that strategically seek to institutionalize authority through exclusionary strategies (Weber, 1922/2002). Work as a vocation (*beruf*), which was once rooted in the Protestant ethic, is transformed by economic rationalization that leads to increasing bureaucratization and the rise of the expert, whose legitimacy no longer rests on charismatic or traditional grounds; professional authority is now based on a belief in competence attested by diplomas and loyalty to established norms (Weber, 1905/2004; Weber, 1922/2002).

Central to this approach are Weber's concepts of power, as "the probability that an actor within a social relationship is in a position to carry out his own will despite resistance" (Weber, 2004, p. 33). This is operationalized through social closure, the process by which groups restrict access to resources and opportunities for eligible individuals, aiming to maintain prestige, social honor, and economic chances (Weber, 2010). Through these exclusionary strategies, professions transform *de facto* power into legitimate rational-legal authority and, in this way, monopolize their advantages in competitive markets (Weber, 1922/2002). But monopoly is not a perpetual status. It requires the continuous exercise of linking social closure to the practical domain of labor.

Professionalism involves a cognitive dimension where abstract knowledge serves as a strategic resource for autonomy and the control of jurisdictions (Abbott, 1988; Dubar; Tripier; Boussard, 2015; Rodrigues, 2002). Alongside this, a political dimension exists where service provision becomes a point of negotiation between the state and the professions to maintain practice exclusivity (Saks, 2010). The pursuit of legitimacy and market protection is operationalized through the professional project (Larson, 2017), which involves the struggle for control of jurisdictions (Abbott, 1988) and the attainment of autonomy (Freidson, 2007).

Given that the system occupied by professions and occupations is strained by external, relational, and internal forces, the meanings of professionalism also change, especially in the face of modern organizational (Evetts, 2013) and interactional contexts (Goffman, 2002; 1967/1982). This vulnerability becomes particularly visible when alternative modes of institutionalizing expertise emerge, as the following sections demonstrate through the concepts of professional project, autonomy, jurisdiction, and knowledge.

3.2.1 Concepts of profession, professional project, autonomy, and jurisdiction

The neo-Weberian and other critical approaches maintain Hughes's notion that "a profession is an occupation that has achieved a special standing among occupations" (Hughes, 1993, p. 465). This thesis adopts Abbott's (1988, p. 318) definition of professions as "exclusive occupational groups applying somewhat abstract knowledge to particular cases." Monopoly represents merely one form of jurisdictional settlement, being rare, difficult to maintain, and not universally sought (Abbott, 1988). Beyond rejecting market exclusivity as essential, Abbott (1988) and his peers converge in classifying professions as a unique and privileged occupational type.

According to Freidson (1989, p. 425), a profession is "a kind of occupation whose members control recruitment, training, and the work they do," having the right to supervise and evaluate its members' technical activities. This professional autonomy excludes repetitive tasks, applying only to activities that require discretionary judgment based on theoretical knowledge. Furthermore, professional authority is achieved through persuasion, as the group must convince society that its specialized knowledge deserves exclusive control and that this monopoly will serve the public interest, not purely selfish ends (Freidson, 2007).

Larson (1979; 2017) defines a professional project as the conversion of an occupational group's limited resources (particularly knowledge and skills) into social and economic rewards. She believes that social closure mechanisms like accreditation maintain scarcity by restricting supply in the labor market. This process combines accredited cognitive exclusivity with market closure to ensure prestige within the social stratification system. However, the professional project is susceptible to threats, requiring the constant invocation of the legitimizing ideology before the state and the public. Consequently, professions are special communities of discourse that possess the authority to speak about and for their respective fields, constructing their meaning and maintaining internal unity to defend their credibility (Larson, 2018). Therefore, addressing specialized knowledge and promoting professional ideology on social media can be understood as a form of discourse useful for

maintaining the collective professional project, though individual practitioners may instrumentalize this rhetoric for personal ends.

Gaining control over a territory of work involves more than publicly defending one's arguments; it also entails disputes over jurisdictional control, in which professions compete with one another to take on tasks and solve problems. According to Abbott (1988), a "jurisdiction" represents the link between a profession and its work, anchored in formal and informal structure and social and cultural dimensions. The social dimension of the acquired jurisdictional rights may include a monopoly on practice, the right to self-discipline, and control over professional training, recruitment, and licensing. The cultural dimension is supported through three essential stages, including the classification of the problem, known as diagnosis; the reasoning process, or inference; and the final execution of action, defined as treatment. While diagnosis expands the knowledge base and treatment follows instructions derived from it, inference remains the most exclusive to professional tasks. It uses the diagnosis to generate treatment options and predict results, requiring a cognitive structure that society recognizes as legitimate for solving the issues at hand.

3.2.2 Knowledge as a necessity and ideological tool for legitimacy

Professions became possible once knowledge emerged as a sociocultural entity independent of established social institutions (such as monarchies and religions) and when the market became a societal element enabling private provision of knowledge-based services (Freidson, 1995). In this context, knowledge is essential for highly specialized work because it ensures that tasks and solutions to complex problems rest on rational, logical, and scientific principles and values (Abbott, 1988). Thus, knowledge allows professions to dispute jurisdiction, claim autonomy, and secure social legitimation necessary for realizing the professional project. As a synonym for competence, knowledge reinforces belief in professional rational-legal authority.

Abbott (1988) contends that abstract knowledge is the only one capable of rationalizing professional work while justifying it through cultural values. Although some occupations obtain licenses or create ethical codes (real estate agents, for example), only professions grounded in abstract knowledge systems can defend and expand jurisdiction by claiming new domains and tasks. Abstract and concrete knowledge form a continuum with varying degrees, the former context-independent and the latter context-dependent. A profession risks losing credibility at either extreme, as excessive abstraction may appear as mere formalism, while excessive concreteness is often dismissed as craft (Abbott, 1988).

Higher education institutions serve as the primary means of professionalization because they provide scientific knowledge training and legitimacy through credentials (Carvalho; Santiago, 2016). Academia is uniquely suited to professional field expansion, as exemplified by medicine's appropriation of alcoholism and mental illness from the church and police. For Larson (2017, p. XXV), the modern university provides the structural link between market and technical-theoretical knowledge, two previously disconnected entities. The university combines the production of knowledge with that of "the production of the producers," which standardizes expertise so that it can be sold as a recognizable commodity.

The professional group sets the training's content and duration (if there is no organized theoretical body, one will be constructed) to meet work needs and to prove that their activity is a profession. Long-term learning and training help to develop professional identity, prevent the dissemination of knowledge to the outside, and maintain supply control (Freidson, 2007).

A profession's prestige and power are determined by the nature of its knowledge, its social evaluation, how it is applied to serve clients and expand jurisdiction (Abbott, 1988), and how access to it is protected (Freidson, 1995). But although necessary, certified knowledge and credentials are insufficient to typify a profession (Larson, 2018); they are means and resources, not attributes (Hughes, 1993), because they can only have an impact if clients, the state, and broader society believe in their worth (Abbott, 1988; Freidson, 1989).

Therefore, the professional ideology centers on the belief that specialized knowledge and skills require higher education, specialized training, and experience, thus justifying trust, autonomy, and monopoly. Professional knowledge and skills lack inherent power; their generic resource is persuasion, a complex task including "rhetorical claims and institutional practices that ordinary occupations do not ordinarily display" (Freidson, 1989, p. 427). Maintaining professional unity and consensus on its premises, therefore, requires a continuous discourse in defense of its ideology (Larson, 1979).

External forces pose systematic challenges to this defense. Alternative modes of institutionalizing expertise, such as the commodification of knowledge and organizational employment, compete with professionalism, according to Abbott (1988). Commodification embeds expertise into products, such as standardized forms or computer systems, and threatens professional jurisdiction by eliminating the need for professional mediation and making knowledge accessible to non-specialists. Organizational employment poses a greater threat since it subordinates professional work to organizational hierarchies and routines, reducing professions' workplace control over their tasks (Abbott, 1988). According to Larson (2017), market rationality and organizational bureaucracy disrupt the constitutive link

between educational credentials and market position in the division of labor. Credentialing loses its capacity to secure collective mobility and occupational closure; expertise circulates outside the profession's monopolistic control, and professionals risk subordination to organizational imperatives and capitalist profit objectives.

3.2.3 Defining professional and occupational boundaries

The seminal works cited above provide sufficient descriptions to establish dividing principles between professions and other occupations, even if this distinction has supposedly been overcome (Evetts, 2013). Professions are service-based occupations requiring advanced education, training, and experience, though this specification becomes unnecessary as the discipline shifts focus toward professionalism, a transition detailed in the next section. Crucially, this expertise claims legitimacy through a 'service ideal' or ideology where the professional is expected to prioritize the client's welfare and public good over economic self-interest (Freidson, 1996; Larson, 2018). For Freidson (2001), who also had approached profession phenomenologically as something that is realized or achieved by different social actors, researchers must specify how this concept is applied.

This thesis emphasizes the capacity of social media platforms to affect fully constituted professions that possess high levels of field control and organized autonomy. The selection process for this thesis moved from classic professions through "imperial professions," namely, engineering, law, and medicine (Coelho, 1999), to the operational choice of medicine. As health is a universal need, the medical market is potentially unlimited, while the state provides social closure through licensing and registration (Larson, 2017; Saks, 2016). This monopoly relies on linking a specific object of work, the human body, with deep and formally rich knowledge. Such a cognitive abstraction ensures exclusive control over diagnosis and treatment against rival occupations (Abbott, 1988; Dubar; Tripier; Boussard, 2015).

The ability to abstract knowledge from concrete contexts defends professional jurisdictional boundaries while simultaneously building hierarchies of prestige within professional fields linked to the purity or impurity of the work. "Purity" is characterized as abstract labor that was previously being mediated by lower-status professionals, whereas "impurity" is found in frontline work that comes into direct contact with social disorder or organizational and commercial logics. This distinction, formulated by Abbott (1981), is based on Douglas's (1966/2001) argument that anything that crosses categorical boundaries or does

not fit into any category is considered impure or dangerous. Impurity, then, is a violation of cultural categories.

Specialists deal with previously refined, technically well-defined problems, while emergency physicians or general practitioners deal with complex and ambiguous human situations. The problem is purified in the referral chain and arrives refined and delimited to professionals with higher interprofessional status. “Such successive purification is, in fact, the norm of professional life. The intern prepares the body, the resident opens it, the cardiac surgeon works with the heart alone,” exemplifies Abbott (1988, p. 827). Prestige, then, derives from the ability to isolate knowledge.

However, public reverence operates according to an inverted logic. The population values precisely those professionals in direct contact with disorder, who are frontline practitioners whom the profession itself considers impure and of lower status (Abbott, 1981). This paradox becomes especially important in digital settings, because public involvement, not peer recognition, determines visibility and authority, a dynamic that shapes medical communication on social media, where the structural conditions of frontline work are unavoidable. Social media is inherently “frontstage” (Goffman, 2002, p. 78), a public space where individuals act and present an image of themselves that conforms to social expectations, as opposed to the “backstage,” the private space where they can be themselves. Medical influencers must therefore deal with the tension between peer recognition, which values purified work, and public recognition, which values direct engagement.

Although many physicians work within organizational settings, medicine retains the characteristics of a liberal profession, making it the main field for identifying traits of occupational professionalism. In contrast, content creation for social media is an emergent occupation born from and shaped by the logic of social media platforms. As established in Chapter 2, the conversion of leisure into labor within these systems is conditioned by the platforms’ own definitions of professionalism. While medicine retains institutional foundations, content creation for social media lacks them and is directly subject to the governance and business interests of specific organizations.

Medicine and content creation relate to their respective work domains differently. Abbott (2005) explains that actors connect to tasks through ‘ligation,’ an interaction process that simultaneously defines the actor, the location, and the relationship between them. In traditional medicine, this connection occurs through permanent credentials, producing an exclusive and stable jurisdiction. I argue that continuous performance metrics function as ligation for content creators, they do not produce stable jurisdictions but rather what Abbott

(2005) calls settlements, locations without rigid or exclusive task boundaries where positions remain fluid and require constant renegotiation.

This structural distinction between jurisdiction and settlement explains why medicine qualifies as a profession while content creation does not and why the analytical focus must shift from a profession as a structural category to professionalism as a normative process. When professions operate within organizational logics, whether in hospitals or on platforms, the analysis must move toward examining the origin and operation of the norms that govern each field, as theorized by Evetts (2013; 2018) and others.

3.3 The turn from profession to professionalism

All attempts to characterize professions are ideal types, that is, heuristic constructs that guide hypothetical approaches to the historical development of certain occupations. However, according to Nunes and Souza (2017), the common traits that helped explain the rise of traditional professions in the 20th century are now insufficient to account for contemporary transformations driven by external factors such as technology and globalization, as well as new forms of identity driven by consumption. Turning analytical attention toward professionalism offers a better path for understanding these changes.

Professionalism highlights the importance of trust in economic relations within modern societies characterized by advanced divisions of labor. In everyday language, to say that a group is professional means that it prioritizes clients, maintains confidentiality, refrains from using knowledge fraudulently, and thereby deserves laypeople's trust (Evetts, 2006). Professionalism may thus be a distinct form of decentralized occupational control, that is, self-regulation embedded in each practitioner through shared norms and values.

Professionalism is theorized through distinct lenses across the literature. Freidson (2007) posits professionalism as a third logic of work organization, an ideal type whose normative values, centered on logic and integrity, organize work as an alternative to rational-legal bureaucracy (managerialism, where managers control work) and the free labor market (consumerism, where customers' preferences control work). The institutional arrangement stems from the belief that professional work is so specialized that it is inaccessible to untrained individuals and cannot be standardized or commodified. Digital platforms, however, are simultaneously market mechanisms and algorithmic standardization, a combination that Freidson's typology did not anticipate.

Conversely, Larson (1979) sees professionalism as an ideology that both describes real social relations and produces imaginary narratives to ensure systemic conformity. This

ideology expresses bourgeois individualism built around the private appropriation of socially generated skills. In this view, the idea that science and rational knowledge are neutral serves to justify the professional privileges and authority, which in turn explains the inequalities within the occupational stratification system (Larson, 2017).

In the 1990s, researchers reconsidered the positive contributions of professionalism to social systems, marking a return to its analysis as a normative value (Evetts, 2006). Julia Evetts's constructivist approach gained traction by shifting the focus from defining a profession to analyzing how discourses of professionalism impact occupational change (Nunes; Souza, 2017). As the positive public image of professionals persists, organizations mobilize employees' desire for recognition by defining professionalism as a set of virtues and principles that guide worker conduct with corporate goals. Through mechanisms of occupational identity and self-control, professionalism becomes simultaneously an individual aspiration and a powerful governance tool (Aldridge; Evetts, 2003).

Evetts (2013) has drawn attention to the growing centralization, bureaucratization, and rationalization of professional work through her ideal types of occupational professionalism and organizational professionalism, built on Fournier's (1999) understanding of professionalism as a disciplinary mechanism for governing autonomous labor. These ideal types distinguish the origin of professionalism discourse, how it translates into work control and practitioner autonomy, and the mechanisms through which it operates (as synthesized in Table 3). Saks (2016) notes that Evetts used Weberian concepts of rational-legal authority instead of mainly focusing on social closure and also drew on Durkheim's views of organic solidarity and occupations as moral communities and sources of identity (Evetts, 2013).

Occupational professionalism is a discourse constructed from within the group, built through long-term education, training, and socialization that fosters strong collective identities. In this model, professional self-regulation is supported by codes of ethics and peer supervision, where autonomy to control one's work implies a relationship of trust between practitioners, employers, and clients. Groups skilled at using discourse to construct their identity and bargain with the state are often those that secure market privileges and, consequently, substantial returns in terms of salary, status, and authority (Evetts, 2013).

In contrast, organizational professionalism is a discourse of control used by managers to reconcile individual aspirations with corporate goals. Within organizations, those in higher hierarchical positions set the standards for actions, aligning the conduct of subordinates with business interests (Evetts, 2018). This model is based on rational-legal authority and a top-down approach. Instead of relying on peer trust, it holds professionals accountable

through standardized procedures and performance monitoring (Evetts, 2013). Corporate objectives regulate and replace the practitioner's control in professional-client interactions, limiting discretionary judgment (Evetts, 2018). Workers welcome the discourse of professionalism if they perceive it as a means of obtaining rewards or status, even as it constrains their autonomy (Aldridge; Evetts, 2003).

Table 3 — Evetts' ideal types of professionalism

Dimension	Organizational Professionalism	Occupational Professionalism
Discourse	Discourse of control is used increasingly by managers in work organizations	Discourse constructed within professional groups
Authority	Rational-legal forms of authority; Hierarchical structures of authority and decision-making	Collegial authority
Work control	Standardized procedures; Managerialism	Discretion and occupational control of the work; Controls are operationalized by practitioners
Trust & accountability	Accountability and externalized forms of regulation, target-setting, and performance review	Practitioners are trusted by both clients and employers; Professional ethics monitored by institutions and associations
Theoretical basis	Linked to Weberian models of organization	Located in Durkheim's model of occupations as moral communities

Source: Adapted by the author from Evetts (2013, p. 787)

Evetts (2011, p. 407) states that “since the exercise of professionalism is now organizationally defined and includes the logics of the organization and the market,” it no longer works as a separate logic from managerialism and commercialism, as Freidson's (2007) ideal-type model suggested.

While organizations use management to control practitioners' work, practitioners and professional associations use management strategies to advance their careers as well as improve the occupation's status and standing (Evetts, 2018). This overlap suggests that contemporary professionalism is shaped by the environment of organizations, even in established professions (Choroszewicz, 2023), leading to questions about how professionals handle conflicting expectations when the organization's and the profession's definitions of professionalism do not match (Evetts, 2006).

To understand how organizational professionalism functions, one must analyze the competence frameworks that define and measure what it means to be professional in organizational contexts. This is the subject of the section that follows.

3.4 Competence as a disciplinary regime for governing autonomous labor³

Fournier's (1999) analysis of professionalism as a disciplinary apparatus demonstrates how organizational professionalism governs conduct in practice. It is done through the notion of competence, a concept that Fournier draws from Foucault's liberal governmentality. As a disciplinary regime, the competence-based model defines the domain of possible actions by defining which knowledge and practices are valid and legitimate. The competence-based model extends beyond specific tasks to the subjectivity of individuals, who internalize these self-regulating standards as part of their own professional identity.

Competencies, referring to both tasks and attitudes, shape the subjectivity of individuals, internalizing managerialism within workers' subjectivities (De Gaulejac, 2007). Both professional discourse (Fournier, 1999) and competency (Zarifian, 2003) are intended to regulate the increasing uncertainty of work in flexible organizations. However, this model has contradictions and may lead to the individualization of suffering and self-blame when performance goals are not met (Dejours, 2012; De Gaulejac, 2007). Prescribed work (its defined tasks and rhythms) often clashes with real working conditions, forcing workers to adjust, circumvent, or even violate procedures to complete their tasks, which Dejours (2012) refers to as living labor. In this context, performance evaluations and analogous procedures do not measure labor in its entirety; rather, they are control tools that pressure individuals toward self-evaluation and self-sanctioning (Pagès *et al.*, 2006). The managerialist ideology, which obscures contradictions and allocates responsibility to individuals, favors a "symbolic management of suffering," mobilizing individual aspirations to secure adherence to organizational objectives (Pagès *et al.*, 2006, p. 75). Consequently, resistance becomes individualized, and suffering is silenced⁴ (Dejours, 2012; De Gaulejac, 2007; Pagès *et al.*, 2006).

The main rationale of liberal governmentality for Foucault is to govern through freedom, a logic Fournier (1999) applies to professionalism as a form of governing autonomous labor. Management does not impose conduct but rather offers it as a means of individual self-development, with the competence-based model promoting the appropriate

³ Note on prior publication: This subchapter was developed as a literature review of an article previously presented at AoIR2025: The 26th Annual Conference of the Association of Internet Researchers (Pithan; Araujo; Dalmoro, 2025).

⁴ This framework is based on organizational studies of managerial ideology in traditional employment settings. Its application here is theoretical: Phase 1 findings revealed that platform instructional materials use analogous symbolic harm management mechanisms directed at content creators in general (see section 6.1.3). The empirical claims of this thesis do not address whether and to what extent these mechanisms cause experienced suffering among MMIs, who are structurally privileged as content creators.

mode of conduct (in which the customer reigns) “rather than simply a way of performing one’s job” (Fournier, 1999, p. 296). Individual development is defined as taking responsibility and being flexible, adaptable, and persistent in achieving goals and dealing with changing business priorities. Organizations avoid the consent-control dilemma by defining expectations as competencies, making “appropriate conduct” the condition for recognition and advancement (Fournier, 1999, p. 286).

In the company empirically studied by Fournier (1999), the competence framework involved listening, self-criticism, flexibility, sensitivity to human needs, and the ability to mobilize collective energy. This displacement enables organizations to practice indirect control over workers. New employees are called upon to act in the name of a supposed customer demand (either external or internal) and to pursue their self-improvement, just as traditional professions claim their authority based on service to the public.

3.4.1 From qualification to competence as performance validation

Although neither Fournier nor Evetts mentions a specific theory of competence, Fournier draws on Foucault’s *technologies of the self*, treating competence as a disciplinary mechanism that constitutes professional subjectivity. French scholars, by contrast, approach competence as a management model addressing workplace indeterminacy. Yet these perspectives converge in practice, as Evetts’s organizational professionalism becomes operational through the concrete tools Zarifian (2003) describes, such as performance observation, situational evaluation, and managerial validation of workers’ capacities in action.

The French conception associates competence with people’s achievements, with what they produce or deliver. Individual competence is the mobilization and combination of resources (such as knowledge, know-how, and emotional capabilities) to perform work activities in a given context in response to a particular demand (Durand, 2015; Le Boterf, 2003; Zarifian, 2003). Such an approach requires workers to take initiative, assume responsibility, and learn from experience to expand their repertoire of actions (Zarifian, 2003). Organizations, in turn, expand their competencies by incorporating those of their workers. They use strategic vision, structure, processes, and organizational identity (shared values, rites, taboos, and common beliefs) to influence individual attitudes in a direction that is in the interest of all (Durand, 2015).

The concept of competence replaces notions of qualification. While qualification is anchored in education, attested by diplomas and collectively negotiated, competence is validated by management based on performance and on the individual acquisition of

knowledge, know-how, and interpersonal skills (Dubar; Tripier; Boussard, 2015; Zarifian, 2003). Evaluating whether someone is competent requires observing a person's actions, results, and ability to reflect on their work (Le Boterf, 2003).

In social media, this transition marks a shift from the expert system to an informal reference system, in which trust no longer resides solely in institutional accreditation but becomes mediated by user experience and digital visibility as signals of quality (Arcidiacono; Pais; Piccitto, 2023).

3.4.2 The professional as a self-enterprise through identity and mobility

This transition happened due to the convergence in the 1980s of the diffusion of the competence-based model with the simultaneous identity wave, which was marked by the valorization of the individual as the agent of their trajectory and by the internalization of the demands for subjective mobilization, competence, and personal fulfillment as conditions for recognition at work (Dubar; Tripier; Boussard, 2015). For these authors, competence operates as a new form of socialization, with workers embracing competence discourse because it offers the promise of personal realization through the identity of a winner or self-entrepreneur. This phenomenon explains why people are less likely to resist competence-based management compared to direct control models, given that the motivation to engage is voluntary and based on personal goals rather than being forced.

The new way of managing workers is no longer about helping them get internal promotion; instead, it focuses on external mobility and building networks that allow them to self-realize and be employable. It is not just about developing skills but about realizing one's full potential by managing oneself like a business, making personal identity a self-enterprise (Dubar; Tripier; Boussard, 2015). This shift of professional identity into self-employment aligns with the notion of 'sincere' and 'cynical' performers (Goffman, 2002). The competence-based approach encourages workers to fully adopt corporate values so that acceptable professional behavior and genuine self-expression become indistinguishable (the sincere actor who believes in their performance). However, the constant self-monitoring demanded by competence frameworks risks generating cynical performers who carefully manipulate impressions but lack belief in the reality they project.

The professional is redefined as a versatile and self-developing agent, responsible for their work and for their career, which is conceived as lifelong personal development (Dubar; Tripier; Boussard, 2015). Within this logic, competency frameworks become the operational mechanism of organizational professionalism. When organizations, including digital

platforms, define professional competencies, they prescribe both performance and appropriate professional identity and conduct (Evetts, 2013; Fournier, 1999). This situation raises critical questions when platform organizations prescribe competencies for established professions like medicine, leading to possible tensions between the professionalism defined by peers and the professionalism defined by the organization.

3.5 Professionalism as situated performance

Content creators are the concrete embodiment of the professional redefined by the competence model, as described above. This is particularly true for the subtype of digital influencers, characterized by online celebrity status, a distinct self-brand, and relationships with commercial sponsors (Abidin, 2016; Duffy, 2016). The competence of these creative workers is defined, evaluated, and validated by social media platforms based on their capacity to generate business results, namely, to prompt data production by the audience. Therefore, the platform organization is the normative instance that defines professionalism as an extension of its own economic rationality, driven by the commodification of attention.

While social media platforms grant creators autonomy, the visibility imperative constrains autonomous professional action, as discussed in Chapter 2. Provided it does not violate any law or platform regulation, any action by a content creator that produces maximum value in terms of engagement and audience metrics, and thus meets the social media organization's objectives, is deemed acceptable. For example, a Brazilian plastic surgeon who posted TikTok videos dancing while exhibiting skin and fat removed from a patient was faithful to the platform's "organizational professionalism" but violated the standards of the medical profession. As a consequence, the Regional Medical Council initiated disciplinary proceedings (Martins, 2021). This case demonstrates that the uncritical pursuit of digital popularity can lead medical influencers to practices contrary to professional norms (Ng *et al.*, 2024). Their digital self-presentation requires negotiating between the roles of health service provider and content creator, but negotiating roles differs from balancing demands or partially adhering to one regime or another.

Empirical research on health professionals specifically documents two distinct dynamics operating within this negotiation. Context collapse, the structural property of platforms that concentrates heterogeneous audiences in a single public space (Marwick; boyd⁵, 2010), renders the separation of professional and personal spheres unviable even when

⁵ dana michele boyd is a social media and youth culture scholar who legally changed her name to be written in lowercase for reasons she has articulated on her website <https://www.danah.org/name.html>.

practitioners actively attempt to maintain it (Maggio *et al.*, 2023). Responding to this structural condition, some health professionals engage in what Atef, Fleerackers, and Alperin (2023) term context collusion, the deliberate integration of multiple identity facets (accessible, knowledgeable, pedagogical, and popular) into a unified account, generating a fused role that could not be sustained in institutionally segregated settings. The two dynamics are not sequential, as context collapse operates as a structural constraint independent of the agent's intention, while context collusion operates as a strategic response that transforms an imposed condition into a resource for audience engagement.

The two dynamics are not sequential, as context collapse operates as a structural constraint independent of the agent's intention, while context collusion operates as a strategic response that transforms an imposed condition into a resource for audience engagement; this thesis treats the former as the generative condition shaping medical influencers' performance, given that Goffman's framework of impression management already accounts for the strategic integration Atef *et al.* (2023) describe.

Unlike traditional professional settings, where practitioners can segregate audiences (Goffman, 2002), performing differently for clients, peers, and regulators, creators must address an imagined audience that simultaneously includes potential patients, other professionals, platform systems, sponsors, and casual viewers. To fulfill tasks with contradictory conditions prescribed by platforms, content creators employ creative, deviant, and subversive tactics (Pithan; Araujo; Dalmoro, 2025); thus, performance itself constitutes a simultaneous response to both occupational and platform expectations.

The role of a medical influencer can be conceived as a performance and not as exclusive loyalty to one regime or another. The physician plays the role of influencer (and the influencer, the role of physician), responding to the specific context and adapting to the infrastructure of the platforms and the expectations of other actors. This means, in Goffman's (2002) terms, engaging in impression management and forming a visible identity in dress, attitude, vocabulary, and empathy, as well as performing socially acceptable ways of acting and interacting. These acceptable behaviors, and not technically correct alternatives, determine how the professional is perceived, evaluated, and legitimized (Noordegraaf, 2020).

Professionalism emerges in the space of social action (Goffman, 2002); it is constituted through interactions in which external actors, audiences, and situational contingencies participate in defining legitimate professional conduct (Noordegraaf; Brock, 2021). Professionalism is a performative act, and its key dimensions, which are expertise, autonomy, and authority, must be continuously enacted (Noordegraaf, 2020). Expertise is

performed for dispersed publics, autonomy is exercised within platform-imposed constraints, and authority is claimed and evaluated beyond formal credentials. This situated and publicly evaluated accomplishment aligns with Larson's (2017, p. xx) view of professional design and her description of professions as special communities of discourse.

Social media is thus an inescapable arena for making a rhetorical defense of the profession, for validating its autonomy to act, and even for justifying monetization in the digital sphere (Pithan; Closs, 2025). However, this performance is inherently unstable. The contradictions between professional values and the logic of metric visibility expose the individual to unprecedented risks.

With the theoretical foundation established, the thesis continues the analysis of this dual professional regime in the following chapters. Chapter 4 examines the historical formation and regulatory structure of the first professions established in Brazil, with emphasis on medicine as a legacy profession strongly regulated by the state.

4 THE MAKING OF REGULATED PROFESSIONS IN BRAZIL

Following the theoretical lens of the sociology of professions established in Chapter 3, this chapter explores the historical development and regulatory framework of the first professions established in Brazil, focusing on medicine as a legacy profession heavily regulated by the state. The imperial state established the so-called imperial professions (law, engineering, and medicine) throughout the 19th century (Coelho, 1999; Vargas, 2010). The initial landmark dates back to the transfer of the Portuguese Court to Brazil in 1808, when the first schools of surgery and medicine were created in Bahia and Rio de Janeiro, followed later by the establishment of law courses in 1827 (Santos, 1995; Coelho, 1999). The institutionalization of these careers was based on educational credentialism, establishing the higher education diploma as a mandatory legal requirement and a market reservation mechanism since the period after Independence (Coelho, 1999).

The Brazilian model is similar to the French model in that it bases professional status on the possession of official academic degrees that confer legal privileges and privileged access to positions in the state bureaucracy (Coelho, 1995; Barbosa, 1998). In the United States and England, professions emerged from civil society seeking cultural authority through voluntary associations; in Brazil, they historically constituted themselves as extensions of the state itself, creating an elite in which political and administrative activity often overlapped with liberal technical practice (Coelho, 1999).

The Brazilian structure consolidated, especially after 1930, when a “regulated citizenship” emerged (Santos 1995, p. 19). Within this rigid corporate system, councils function as federally autonomous entities, possessing delegated police powers for inspection and punishment and ensuring market reserves through legal enforcement (Santos, 1995; Vargas, 2010). This logic functioned differently for liberal professions and for the working class. For the working class, access to social security and other social rights was conditional upon formal inclusion in the productive structure through the registration of a recognized profession in the National Work and Social Security Card (CTPS). Professional elites organized themselves into autonomous councils outside the union system, such as the Brazilian Federal Council of Medicine (Conselho Federal de Medicina–CFM) and the Brazilian Bar Association (Coelho, 1999; Santos, 1995). This configuration perpetuates a stratified social hierarchy that shields traditional careers from market competition and social mobility (Pereira Neto, 2000; Vargas, 2010).

The state conducted the professionalization project vertically, creating official colleges and imposing the legal diploma (Coelho, 1999; Santos, 1995), and hired physicians, engineers, and lawyers for its bureaucracy (Pereira Neto, 2000). Because of this connection, professional elites were able to maintain their status and power despite shifts in the political system by influencing laws to safeguard their market reserves (Coelho, 1995; Coelho, 1999).

4.1 Professional strategies for market closure

Physicians sought to expand their authority by intervening in the definition and control of public problems such as alcoholism, prostitution, and crime. Their goal was to shift the approach to these issues from the legal sphere to the medical one and to influence the reform of penal codes (Coelho, 1999). This jurisdictional expansion operated by transforming social issues into pathologies that required technical treatment instead of just punishment, opening a dispute with jurists over the ownership of these problems and the responsibility for their solution (Coelho, 1999). The legal profession has expanded its domains by bringing judicialization into everyday life, inserting complex legal rules into social relations, and making citizens dependent on the mediation of lawyers to resolve conflicts that were previously dealt with outside the forensic sphere (Barbosa, 1998).

To preserve the value of fees and social prestige, corporations acted politically to limit the supply of new professionals, pressuring the state to veto the opening of higher education courses and preventing diploma recognition (Vargas, 2010). In Brazil, the university degree represented the mechanisms of social closure (Weber, 1922/2002) and artificial scarcity (Larson, 2017), which were ensured by long, expensive courses with rigorous entrance exams (Coelho, 1995).

The diploma also served as a social stratification marker, legitimizing an occupational hierarchy in which ‘learned’ knowledge predominated over practical or popular knowledge and conferring cultural authority on its holders to define societal problems (Barbosa, 1998). Credentialism has become a fundamental requirement for access to positions in the state bureaucracy and for the reproduction of elites, ensuring that the most valued qualifications remain concentrated in the highest income strata, even with the expansion of higher education (Coelho, 1999; Vargas, 2010).

4.2 Elite associations and professional stratification

The Imperial Academy of Medicine, the Brazilian Bar Association [*Instituto dos Advogados Brasileiros*], and the Polytechnic Institute/Engineering Club were clubs aimed at

influencing legislation and securing monopolistic privileges, not representative class associations. The Imperial Academy of Medicine was the imperial government's advisory body on public health and epidemic control issues, bringing together professors and physicians from the Court with the goal of assuming state authority over education and combating competitors such as homeopaths (Coelho, 1999; Santos, 1995).

While this restricted group controlled the official associations and claimed to be financially disinterested, most professionals, including unlicensed legal practitioners and lesser-renowned physicians, struggled economically and relied on daily technical jobs for survival, often being stigmatized by corporate leaders as merchants or executors of low-level tasks (Coelho, 1999; Santos, 1995). The population, in turn, rejected aggressive and painful therapies; they preferred treatment from healers and homeopaths, who were considered safer. In addition to being more financially accessible and culturally closer to patients, alternative therapists offered less invasive treatments, which were more socially accepted given the skepticism regarding the effectiveness of medicine (Coelho, 1999; Pereira Neto, 2000).

The medical profession refused to guarantee a cure, admitted therapeutic limitations, and focused on the dignity of the scientific procedure. Promises of results were considered incompatible with the prudence and uncertainty of clinical practice at the time. The Imperial Academy of Medicine argued that the freedom to practice a profession without the control of the faculties generated anarchy and threatened public health (Coelho, 1999; Santos, 1995). Using science rhetorically to disqualify competitors, the institution labeled healers and homeopaths as ignorant individuals who did not master academic knowledge (Coelho, 1999). Common practices in indigenous and African cultures were classified as quackery, including the use of unknown substances in prescriptions, the sale of secret remedies, and the application of therapies based on religious beliefs or nature (Pereira Neto, 2000). Thus, allopathic medicine sought to establish a monopoly by linking its practice to truth and reason, while portraying competitors as dangerous sects or scientific heresies (Pereira Neto, 2000).

But although the imperial elite copied French scientific rhetoric to differentiate themselves from healers and homeopaths, the practice had little therapeutic efficacy, and its image depended more on social and political prestige than on effective cures (Coelho, 1995; Santos, 1995). During the First Republic, this scenario evolved into a conflict where hygienists used state power to impose sanitary rules, such as the mandatory notification of diseases, clashing with clinicians who feared losing the trust of their private clientele and their liberal autonomy (Pereira Neto, 2000). Internal disputes became evident at the 1922 National Congress of Practitioners, when the category was divided into three identities, the generalist,

the specialist, and the hygienist. The generalist treated medicine as a liberal art and emphasized altruism and the relationship with the patient; the specialist claimed status based on scientific rationality; and the hygienist claimed prestige for caring for collective health (Pereira Neto, 2000). This division reflects the symbolic stratification of medicine, where distance from the individual patient conferred status. General practitioners maintained direct contact (impure work), specialists distanced themselves through technical fragmentation, and hygienists at the top dealt with health in an abstract and collective way, according to Abbott's (1981) distinction of purity and impurity.

Abbott (1981) also observed that professions experiencing intraprofessional status regression (that is, declining prestige among peers) tend to respond by dispersing their charisma outward, toward the public, via mass media. In the mid-twentieth century, the American Medical Association's use of television and radio demonstrated this dynamic, where public visibility compensated for the loss of collegial authority. The digital platforms of the twenty-first century represent a structurally analogous moment in Brazilian medicine, with the algorithmic recommendation system serving as the arbiter of charismatic dispersal rather than the broadcaster or the newspaper column, with consequences for jurisdictional authority that Abbott (1981) himself warned would be eroding in the long run.

4.3 Credentials, jurisprudence, and market protection

The creation of the Central Board of Hygiene in 1851 removed sanitary control from the municipal councils and required the official registration of diplomas for anyone wishing to work as a physician or pharmacist (Coelho, 1999). Although the Imperial Academy of Medicine centralized oversight, it did not receive from the government the power of a medical tribunal to judge and punish competitors on its own (Coelho, 1999).

The 1891 Constitution guaranteed the free exercise of any moral, intellectual, and industrial profession [*livre exercício de qualquer profissão moral, intelectual e industrial*], but the Constituent Assembly rejected amendments that called for unrestricted freedom. The conflict pitted credentialists, who defended diplomas as protection against technical incompetence, against anti-credentialists, who argued that academic medicine lacked exact theories and operated empirically as much as popular practices, making the legal privilege unjustifiable (Coelho, 1999).

The Rivadávia Reform of 1911 attempted to establish occupational freedom by removing the official character of education, but the Supreme Federal Court (STF) ruled in favor of requiring diplomas for professional practice, establishing jurisprudence that the

constitutional guarantee of freedom of work did not eliminate the need for technical qualification titles, thus validating the Penal Code's restrictions on practitioners and healers (Coelho, 1999).

The expansion of higher education during this time resulted in a surplus of graduates that the market could not absorb, forcing many physicians, lawyers, and engineers to take jobs outside their fields or in lower-paying positions to survive. This saturation changed the role of the diploma, which went from being a mere mark of social distinction to a technical certificate required for fierce competition, prompting professionals to seek refuge in the state bureaucracy, although the number of public positions was insufficient for everyone (Coelho, 1999). Faced with the risk of impoverishment and loss of autonomy, the categories reacted by creating economic defense entities, such as the Brazilian Medical Union in 1927, and pressured the government for laws that guaranteed market reservation for holders of academic degrees (Santos, 1995; Coelho, 1999).

4.4 The federal council of medicine

Market saturation and corporate mobilization for economic protection culminated in the institutionalization of professional control through the creation of the Federal Council of Medicine in 1945, restructured in 1957 as a federal autarchy (Santos, 1995). The law transferred from the state to the profession itself the responsibility for maintaining the registry of qualified physicians and functioning as a disciplinary tribunal, removing direct government intervention in ethical judgments.

The council's power to regulate comes from creating rules, like the Code of Medical Ethics, and overseeing ethical issues through regional councils, which can lead to penalties that range from private warnings to losing the right to practice medicine altogether (Salmeron, 2022; Santos, 1995). The central difference between the Brazilian model and that of voluntary associations is that the organization of professions in Brazil after 1930 was structured by the state through the creation of public law autonomous entities with compulsory membership for the legal practice of the activity. While associations such as the American Medical Association in the United States emerged from private initiative and market dynamics before seeking state support, the CFM (Federal Council of Medicine) was born already embedded in the bureaucratic structure of the state to exercise a delegated public function of control over who can or cannot work (Coelho, 1995; Santos, 1995).

4.5 The logic of non-commercialization: ethics, advertising, and disciplinary control

Medicine has legitimized its authority and monopoly based on the ideal of service, the commitment to doing a good job, and the well-being of the patient. This moral mandate suggests that, since they do not use their specialized knowledge in a mercantile way, physicians deserve public trust and the autonomy to self-regulate, distinguishing themselves from activities primarily aimed at profit (Barbosa, 1998; Coelho, 1995). This distinction historically differentiates the social status of physicians in relation to other health occupations, such as apothecaries (Coelho, 1995).

The principle of non-commercialization is formalized in codes of ethics that attempt to discipline the conduct of professionals in the market. In practice, this normative structure often clashes with how work in health is organized under capitalism and the way payments are made for services, which pushes for more financial and the use of expensive technologies, making it hard to follow the ethical guidelines that are supposed to be in place (Santos 1995).

The first Medical Code of Ethics appeared in Brazil in 1929, translated from a document approved by a Latin American congress and defining self-promotion as a violation of professional honor (Vieira *et al.*, 2024). The most recent version of the Medical Code of Ethics modernized the norms to include the use of social media and technologies while maintaining the historical prohibition against sensationalist and mercantilist advertising (CFM, 2019). The document's fundamental principles establish that medicine is a means-based activity [*atividade meio*], not an end product for commercialization (CFM, 2019). However, the principle of non-commercialization faces a structural contradiction. Professions, as we know them, were only possible when the market economy became a social feature to the point of enabling the private provision of knowledge-based services (Freidson, 1995), but professional projects depend on groups distinguishing themselves from purely mercantile logic (Freidson, 2007; Larson, 2017). In the case of Brazilian physicians, working for a salary or through health plans restricts their autonomy because institutional protocols, payment systems, and management structures limit their ability to make choices about diagnosis and treatment (Schraiber, 1993). On digital platforms, automated recommendation systems subordinate decisions about the creation of medical content to optimization for visibility, which can be considered an example of organizational control over professional autonomy.

With Resolution No. 2336/2023, the CFM has gradually expanded the regulatory scope to encompass medical communication in digital contexts. The regulation requires sobriety and combats sensationalism, but it relaxes previous provisions by allowing the

disclosure of consultation fees and the use of patient images, including ‘before and after’ images, as long as they are educational and do not promise results (CFM, 2023).

The opening of disciplinary proceedings does not depend on a formal complaint. The Code of Ethics assigns to the regional medical councils the legal competence to oversee professional practice and initiate investigations (CFM, 2019). However, the application of penalties faces problems with delays in the internal procedures of the councils, which maintain outdated judgment structures in light of the growth of the medical category (Salmeron, 2022).

The CFM’s defense of professional autonomy leads to disputes with other regulatory bodies of the state, such as the National Health Surveillance Agency (Anvisa). According to Paumgarten (2022), the council engaged in a dispute opposing the agency’s ban on amphetamines used for weight loss and defending the prescription of drugs without proven efficacy during the Covid-19 pandemic. The professional body argued that the physician’s autonomy to decide on treatment prevails over market-driven health control, emphasizing the political use of self-regulation to expand or protect the scope of clinical practice against external restrictions.

4.6 Contemporary challenges to professional closure

The balance of power between the interests of the profession, the state, and the private education market has altered historical patterns of credential control and social closure in medicine in Brazil. The shortage of physicians, linked to prestige and reward (Larson, 2017), was maintained in Brazil by state control and stimulated by the historical pressure from the CFM (Federal Council of Medicine) against the opening of new medical schools and vacancies. The creation of the ‘Programa Mais Médicos’ in 2013, however, led to an increase in the number of professionals in the country. The law that establishes the program aimed to address the shortage of professionals in rural areas, indigenous areas, remote regions, and the poor peripheries of large cities; strengthen the infrastructure of primary health care; and reorganize medical training to prioritize general practice instead of just hospitals (Santos *et al.*, 2019).

The program encouraged an increase in medical schools (Pinto; Côrtes, 2022), and the number of schools increased from 252 to 448 between 2014 and 2024, with the private sector holding 91.5% of the new places opened. In the last five years of the decade analyzed by Medical Demography, 116,500 new physicians entered the market (Scheffer, 2025). The ethical stance against commoditizing the profession has not prevented the commercialization

of medical education. The expansion of private institutions in professional training makes medical credentials an accessible commodity for those who can afford them, so that market logic threatens the quality standards defined by peers (Freidson, 2001). Furthermore, the number of residency positions has not kept pace with the growth in undergraduate programs, creating a gap between the number of newly graduated physicians and the available specialization positions. This gap expands the share of general practitioners (consistent with the program's goals) while raising the status of the specialist credential (Abbott, 1988; Larson, 2017).

Research on the program's effects reports that it reduced the extreme shortage of physicians in vulnerable municipalities and expanded access to the Unified Health System (SUS) (Santos *et al.*, 2019), but it faced corporate resistance focused on maintaining market reserves and the social status of the profession (Matias; Verdi; Finkler, 2023).

The CFM, in turn, criticizes the opening of schools, classifying it as indiscriminate and claiming that the colleges lack infrastructure. The board argues that the measure failed to retain professionals in the interior and demands the approval of a mandatory National Proficiency Exam to guarantee the quality of care (CFM, 2025). This demonstrates a loss of power for the profession to influence public authorities in regulating the supply of labor and the weakening of the traditional credential, the university degree. Thus, the board demands the requirement of a second credential after graduation as a way to reclaim the dynamics of social closure, in Weber's terms (2010).

The mandatory proficiency test (Profimed) advocated by the CFM is the subject of a bill under consideration in the National Congress that creates an exam similar to the Brazilian Bar Association (OAB) exam, which conditions the release of professional registration on newly graduated physicians passing the test (Cunha, 2026). The CFM (2026) argues that Profimed will prevent physicians without the required qualifications from serving the population. In practice, the exam will create a new *ex post facto* control bottleneck, since the CFM has lost the ability to block the entry of the "production of producers" (Larson, 2017, p. 17).

With the goal of measuring student performance and the quality of medical courses and serving as a unified step for medical residency selection, the Ministry of Education instituted the Enamed exam in 2025 (Brasil, 2025b). The results of the first year of application showed insufficient grades in 30% of the colleges (grades 1 and 2 on a scale of 1 to 5), punished by the federal government with sanctions that include cuts in vacancies,

suspension of the financing fund (Fies) for students enrolled in private universities, and prevention of new admissions (Brasil, 2026).

The institutions with insufficient grades are concentrated in private for-profit colleges and municipal public colleges. Nearly 90% of municipal courses and more than half of private for-profit courses fell into these lower ranges (Brasil, 2026). Although CFM praises Enamed for exposing poor quality education, it criticizes the fact that the graduates from schools with grades 1 and 2 are still able to register for employment despite the exam results (CFM, 2026). While Enamed (government) follows the bureaucratic logic of efficient resource distribution, Profimed (CFM) corresponds to the professional logic of self-regulation and market closure (Freidson, 2001).

Against this backdrop, contemporary disputes over credential control and market saturation shape the conditions under which Brazilian physicians operate as content creators.

5 METHODOLOGICAL ROUTE

This chapter describes the methodological route through which the research was conducted. It begins by establishing the ontological and epistemological positioning that grounds the study, proceeds to justify the choice of constructivist grounded theory as the analytical method, and concludes by detailing the three cycles of data construction and analysis through which the substantive theory was developed.

5.1 Ontological and epistemological positioning

This research adopts critical realism as its ontological foundation (Bhaskar, 1975/2008), which emerged contemporaneously with Burrell and Morgan's (1979) paradigmatic framework but was not included in their typology. Critical realism posits that reality exists independently of our knowledge of it, yet our understanding of that reality is socially constructed. It recognizes that social reality is stratified into three overlapping domains (Bhaskar, 1975/2008). The “domain of real” consists of generative mechanisms and structures possessing causal powers, whether or not exercised. The “domain of actual” comprises events generated when mechanisms activate in specific contexts. The “domain of empirical” includes experiences and observations accessible to researchers. Because “real structures exist independently of and are often out of phase with the actual patterns of events” (Bhaskar, 1975/2008, p. 2), researchers must go beyond surface-level data to uncover the underlying causes that shape social life.

Table 4 — Bhaskar's stratified ontology: the three domains of reality

	Domain of Real	Domain of Actual	Domain of Empirical
Mechanisms	✓		
Events	✓	✓	
Experiences	✓	✓	✓

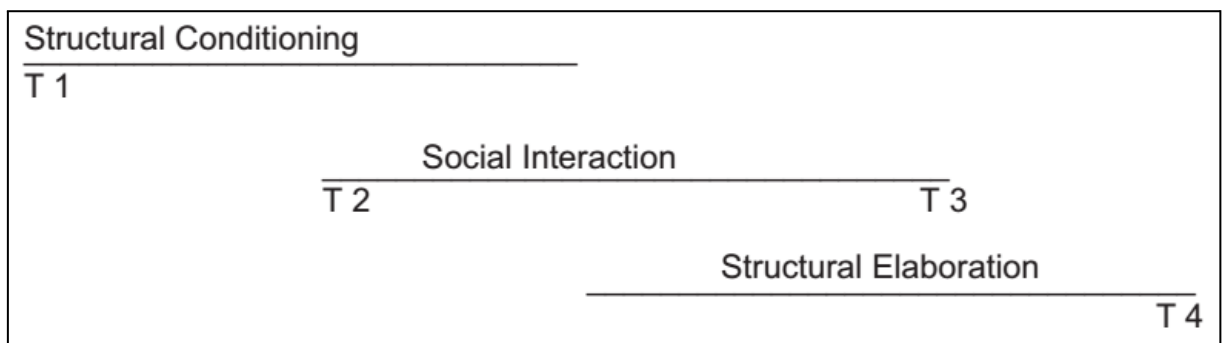
Source: adapted from Bhaskar (2008, p. 2)

Within Bhaskar's (2008) stratified ontology, represented in Table 4, the platform as an organizational form of production, management, and work and the pre-existing institutional structure of the medical profession are the generative mechanisms in the domain of the Real in this research. The incompatibility between these regimes, activated when physicians act simultaneously in both, comprises the domain of the Actual, while the naturalistic digital

traces documented in the corpus compose the domain of the Empirical, as will be explained in section 5.4.3 of this chapter.

In the social world, structures are simultaneously produced by human action and possess the causal power to constrain or enable future actions (Carter & New, 2004). They possess “anteriority” and are “relatively enduring” (Carter; New, 2004, p. 4), meaning they predate the individuals acting within them. Their causal powers include enablement and constraint, such as how “structures of discrimination will constrain the discriminated and enable the discriminators” (Carter; New, 2004, p. 4). To analyze the interplay between these structures and human action, this research employs Archer’s (1995/2020) analytical dualism, which artificially separates structure and agency in time. Archer’s morphogenetic sequence distinguishes four analytical moments: structural conditioning (*T1*), social interaction (*T2-T3*), and structural elaboration or reproduction (*T4*). Through the morphogenetic cycle, pre-existing structures (*T1*) condition social interaction (*T2-T3*), which ultimately leads to either the reproduction or the transformation (structural elaboration) of those conditions at *T4* (Figure 3).

Figure 3 — Archer’s basic morphogenetic sequence



Source: Archer (2020, p. 142)

Charmaz (2014, p. 342) acknowledges that “the emerging significance of critical realism as an approach to take with grounded theory exemplifies the versatility of the method.” She establishes that constructivist grounded theory “distinguishes between the real and the true” and assumes the existence of an obdurate reality (Charmaz, 2000, p. 523). This approach combines ontological realism (the existence of structures with causal powers), epistemic relativism (knowledge is historically and socially situated), and judgmental rationality (the ability to rationally arbitrate between opposing arguments) (Archer, 2020).

According to epistemic relativism, knowledge is historically and socially situated, produced by researchers whose positions shape what can be observed and interpreted (Bhaskar 1975/2008; Charmaz, 2014). The concept does not imply that epistemological relativism licenses the view that all knowledge claims are equally valid; rather, validity must be assessed through judgmental rationality, the capacity to evaluate competing explanations against evidence and theoretical coherence (Archer, 2020; Bhaskar, 1975/2008). In this study, epistemic relativism is operationalized through the reflexive construction of data from naturalistic digital traces (Charmaz, 2014), whereas judgmental rationality governs the retroductive inference from observed contradictions to the generative mechanisms that caused them (Kempster; Parry, 2014). As Kempster and Parry (2014, p. 87-88) demonstrate, critical realist grounded theory enables researchers to “explain what causal powers shape” phenomena and “explain how context affects such causality” through examining “causal powers operating in real, actual, and empirical” domains.

Social media platforms are treated in this research as organizational forms of production, management, and work whose contractual and artifactual power constitutes a generative mechanism possessing “tendencies which may be possessed unexercised, exercised unrealized, and realized unperceived” (Bhaskar, 1975/2008, p. 7). The medical profession constitutes a preexisting institutional structure with causal powers that precede and condition the actions of individual physicians. Following Archer’s (2020) analytical dualism, both are activity-dependent since their causal powers require human agency for instantiation, enabling workers to interpret and adapt prescriptions to contingent conditions.

While Charmaz (2014) refers to abductive reasoning as the logic of discovery in grounded theory, within the critical realist framework, this inferential process is specifically operationalized as “retroduction”, moving “from experiences in the empirical domain to propose possible structures and mechanisms in the real domain that could have produced them” (Kempster; Parry, 2014, p. 89). The observable contradictions in medical influencers' digital practices are the empirical starting point; retroduction moves from these contradictions toward the unobservable mechanisms that generated them. The morphogenetic cycle indicates that platform systems and medical regulatory frameworks shape practice without fully controlling it.

The structural conditioning (*T1*) that precedes the social interaction of medical influencers (*T2–T3*) comprises the platform's generative mechanisms and the preexisting institutional structure of the medical profession, whose incompatibility is activated in the domain of the Actual when physicians operate simultaneously under both. Whereas abduction

moves from observed data toward the most plausible hypothesis, retroduction moves from an observed event toward the unobservable mechanism that generated it, and it is this distinction that justifies the use of critical realism as an ontological complement to constructivist grounded theory. The morphogenetic cycle then traces how physicians interpret and navigate these structural conditions during the interaction phase, and whether the outcome at T4 constitutes structural elaboration or reproduction. The reflexive deliberations of agents mediate this process, allowing them to define their social projects while operating under the simultaneous constraints of platform prescriptions and medical occupational prescriptions.

5.2 Grounded theory

Grounded theory is a qualitative research method for constructing conceptual frameworks or theories about a social phenomenon from the data collected rather than testing previously developed hypotheses. Comparative in nature, it stands out for its exploratory, iterative, inductive-abductive, systematic, flexible, and theory-building orientation. Researchers engage in the field, observing how participants construct meanings and actions, and then analyze data as they collect it, not afterward, coding and categorizing it to identify patterns and explain the phenomenon. In the simultaneous stages of data construction and analysis, one informs and shapes the other (Charmaz, 2014). The following conditions make grounded theory an appropriate methodological option (Creswell, 2014):

- 1) There is no theory explaining the topic under study.
- 2) Existing models were developed and tested on populations other than those of interest (for example, theories explaining labor related to social media content creation refer only to the creative class).
- 3) Existing theories (e.g., sociology of professions) do not address or predict variables such as those studied here (professionals from groups with monopolistic control over cognitive capital now freely share specialized knowledge publicly online).

Grounded theory methodology (GTM) denotes the methodological path, while a grounded theory is the resulting theoretical framework (Bryant; Charmaz, 2019). Consistent with the research object, epistemological positioning, and methodological choices, this study employs Charmaz's (2014) constructivist grounded theory. The next section clarifies the purpose and type of theory this research aims to develop.

5.2.1 Substantive and middle-range theories

A theory is a connected set of claims related to empirical phenomena that serve as a conceptual model for understanding and explaining social reality, predicting outcomes, and producing new insights (Creswell; Creswell, 2021). Unlike grand theories that make universal claims, middle-range theories address problems within limited domains, according to Robert Merton's concept (Charmaz, 2014). Glaser and Strauss distinguished between substantive and formal middle-range theories; the former seeks to explain specific social phenomena within particular contexts, while the latter emerges from comparative analysis of several substantive cases and achieves broader generalization (Charmaz, 2014; Urquhart, 2019).

Given that this research investigates social and relational phenomena among a specific group in a particular context, the outcome is a substantive theory. This is consistent with Charmaz's (2014) view that constructivist grounded theory generates context-sensitive theoretical interpretations rather than context-free generalizations.

5.2.2 Grounded theory origins and variants

In 1967, Barney Glaser and Anselm Strauss published 'The Discovery of Grounded Theory', which challenged the ideas that only quantitative methods are valid and that theory must be kept distinct from empirical study (Charmaz, 2014). Their method provided a methodical path for generating qualitative theories. Glaser, from Columbia University, brought methodological rigor and systematic procedures, while Strauss, from the Chicago School, focused on action, process, and meaning. He saw social phenomena as dynamic processes instead of static structures (Charmaz, 2014; Charmaz; Thornberg; Keane, 2018; Flick, 2020).

From the original GTM, various versions came out that had different philosophical structures and methodological rules (Kenny; Fourie, 2015). The three main ones are Glaserian (classical), Straussian, and constructivist. Despite these differences, all variants share core characteristics, namely that grounded theory elucidates processes, begins with inductive logic, encompasses simultaneous data collection/construction, analysis, and theory construction, uses constant comparison and memo writing, employs theoretical sampling, and focuses on generating grounded theory (Charmaz; Thornberg; Keane, 2018).

This study employs Charmaz's (2014) constructivist grounded theory (CGT). The constructivist version retains GTM's flexible guidelines while adopting a relativistic epistemology that acknowledges the situated nature of knowledge production. CGT assumes that researchers construct categories instead of discovering them, that they seek interpretive

understanding rather than decontextualized generalizations, and that both researchers and participants influence the data and analyses (Charmaz, 2014; Charmaz; Thornberg; Keane, 2018). The following section details CGT's distinctive premises and procedures.

5.2.3 Constructivist grounded theory

Despite its interpretive and partially subjectivist aspects, the GTM never rejected the foundations of the positivist paradigm with its objectivist underpinnings, according to Charmaz (2000). Due to the quantitative hegemony of the time, the influence of Glaser, or an attempt to silence those who claimed qualitative methodology could never match quantitative methodology, the GTM combined its interpretive purposes with the empiricism, logic, rigor, and systematic analysis associated with quantitative investigation.

Charmaz (2000) introduced constructivist grounded theory (CGT) to the research debate, repositioning the methodology within a relativistic epistemology. She chose the term constructivist to acknowledge subjectivity and the researcher's involvement in the construction and interpretation of data, as well as to distinguish her approach from conventional social constructionism (Charmaz, 2014).

CGT is influenced by pragmatism and symbolic interactionism while retaining flexible guidelines based on a relativistic epistemology (Charmaz; Thornberg; Keane, 2018). This perspective seeks interpretive understanding that accounts for context rather than abstract generalizations independent of time, space, and situation. It also claims that researchers construct data and theories from their engagement with participants and emergent analyses (Charmaz, 2014). Charmaz (2000, p. 523) clarifies that constructivist grounded theory "distinguishes between the real and the true" and "remains realist because it addresses human realities and assumes the existence of real worlds." Therefore, rather than prioritizing the researcher's viewpoints, constructionists regard participants' perspectives and voices as essential to both the analysis and its presentation (Charmaz, 2008).

CGT emphasizes reflexivity and acknowledges the perspective of the researcher, the research situation, and debates about the representation of participants. This approach recognizes how historical, social, and situational factors influence these actions and the researcher's role in shaping the data and analyses (Charmaz; Thornberg; Keane, 2018). The following sections describe how the data for this thesis were constructed and how their analysis and coding were performed. CGT's coding process proceeds in two sequential phases: initial coding, which involves line-by-line or incident-by-incident examination of the data generating provisional codes that stay close to the data; and focused coding, which

selects the most analytically significant initial codes and applies them more systematically across the full corpus to construct broader categories (Charmaz, 2014).

5.3 Researcher immersion and ethnographic sensibility

This section describes the preliminary immersion phase that preceded and accompanied systematic data construction, demonstrating how informal field observations created the sensitizing conditions required for grounded theory analysis. I immersed myself in the field by following the content of medical influencers on Instagram, TikTok, and YouTube. I started by following about ten influencers purely based on suggestions from the platforms, and once the sample was consolidated, I extended the practice to follow all 26 subjects selected for analysis, according to the criteria explained in subsection 5.4.3. I watched videos, read posts, observed interactions, and wrote down impressions in memos, but I did not systematically catalog each piece of content.

This immersion allowed me to gain a better understanding of the participants' practices by revealing longitudinal patterns that would otherwise be invisible in point analyses. This immersion is consistent with Hine's (2015) concept of connective ethnography, in which the researcher's journey through online and offline spaces actively shapes the field. Rather than establishing boundaries beforehand, I followed the connections made relevant by the platforms' recommendations and the participants' own practices, viewing social media interaction as an embedded phenomenon (Hine, 2015). Furthermore, this approach follows the principles of netnographic observation, in which the researcher serves as a finely tuned instrument for capturing cultural nuances that automated data mining may overlook (Kozinets et al., 2014). Immersion in comment sections on all three platforms revealed audience dynamics that informed subsequent analytical decisions.

Observed patterns included signs of inauthentic engagement, interpersonal conflicts among audience members in videos where influencers disputed peers' medical claims, and rhetorical disagreements over cognitive authority between physicians. These observations helped shape the analytical categories in Phase 3, as they directed my attention to how influencers claim legitimacy and manage audience reception.

Audience members' complaints about self-managed health packages (saying they weren't receiving support, that protocols weren't working, or reporting poor outcomes after in-person consultations) highlighted the tension between the rhetoric of health education and monetization through the sale of digital products for consumers to guide their self-management of health.

Rather than forming analytical findings, these observations served as early indicators of the structural contradictions that systematic data construction would later investigate. Throughout this immersion phase, my analytical focus was on how physicians perform professionalism in the face of contradictory institutional demands, rather than cataloging platform features or individual influencer content. This distinction guided the transition from informal observation to systematic data construction, as described in the following sections.

5.4 Data construction and analysis

The literature review established that medical influencers work under a dual professional regime. Occupational professionalism is regulated by the CFM and sustained by ethics and autonomy, while the organizational professionalism of platforms is defined by prescribed competencies and validated through performance and visibility metrics. To investigate how these professionals perform in the face of contradictory demands, the research needed to map the two normative regimes before analyzing practices.

The methodological choice for analytical dualism justifies the temporal detachment of structure and agency during data construction. This artificial separation enables the researcher to examine the autonomous properties of institutional norms in an initial stage before observing their instantiation in professional practice. These three research cycles follow Archer's (2020) morphogenetic sequence to identify how structural conditioning (*T1*) provides the environment for the subsequent social interaction (*T2–T3*). This thesis required examining (1) the competency prescriptions of Instagram, TikTok, and YouTube platforms; (2) the regulations of the Brazilian Federal Council of Medicine (Conselho Federal de Medicina–CFM), comparing the two of them; and (3) the performances of 26 digital medical mega-influencers. The impossibility of direct access to subjects through interviews, as detailed in section 5.4.3, determined the data construction strategies of the third stage.

The analysis in the three phases of this research followed the coding procedures of Charmaz (2014), while the structure for systematically visualizing the progression of the data followed Gioia, Corley, and Hamilton (2013). In all phases, the initial codes were treated as first-order concepts, which were abstracted into second-order themes and distilled into aggregate dimensions. This combination allowed this research to maintain the constructivist emphasis on the 'doing' of professionalism (represented by codes in gerunds) while providing the rigorous visual demonstration required to map the structural clash between the two normative regimes. In both phases, initial codes were treated as 1st-order concepts

(informant-centric), which were abstracted into 2nd-order themes and distilled into aggregate dimensions.

The three-component data construction was determined by the empirical need identified in the literature review. No consolidated model exists for organizational professionalism on social media platforms or for Medical Occupational Professionalism applied to this context. Without establishing these analytical foundations, it would be impossible to systematically map the contradictions between the two regimes or to analyze how medical influencers manage them in practice. The decision to use multiple sources, based on their availability and suitability to the research topic, relies on the flexibility in construction strategies allowed by the GT (Charmaz, 2014; Glaser, 2007).

The organizational professionalism of platforms was identified by analyzing the competencies prescribed by Instagram, TikTok, and YouTube in their instructional areas for those who want to pursue a career as content creators. This investigation was developed in collaboration with Willian Fernandes Araujo and Mateus Dalmoro (Pithan; Araujo; Dalmoro, 2025) and served as a reference point for constant comparison in subsequent stages. A section ahead provides a detailed account of the data construction, coding process, and data structure.

To understand how Medical Occupational Professionalism is discursively articulated in digital contexts, this study analyzed three CFM documents: the Medical Code of Ethics, CFM Resolution No. 2336/2023, and the medical advertising manual. These documents represent a formal effort to negotiate medical identity within an unavoidable digital landscape, offering a discursive representation of the occupation that can be understood and contrasted with platform prescriptions to reveal structural tensions. Sections below provide a detailed overview of the data structure.

To investigate how medical influencers cope with the professional tensions of social media activities, the first option considered was to employ intensive interviews. Using different means of contact, such as email, direct messages via social media, and phone messages, I addressed the interview request to the first ten physicians identified as eligible (with regular activity on Instagram, TikTok, and YouTube, and over 1 million followers on at least one platform), but there was no response, not even a refusal. Alternative ideas, such as scheduling consultations and attempting to convert them into interviews, or selecting physicians with fewer followers (presumably more accessible), were discarded due to costs, ethical concerns, and the need to maintain the criterion of addressing cases of maximum possible ‘professionalization’ of digital practice.

According to Glaser (2007, p. 1-2), researchers must deal with exactly what is happening, not with what they would want to happen, and “what is going on in the research scene is the data, whatever the source.” Therefore, I worked with what was available, including social media profiles and content; professional websites; closed WhatsApp/Telegram groups administered by the physicians' teams; and transcripts of interviews they granted to third parties.

According to Glaser (2007, p. 3), data strategically presented by participants to appear socially appropriate (properline data) reveal fundamental processes of social construction, confirming that “socially structured, vested fictions run the world.” Physicians' self-presentations on social media are precisely this kind of data, capturing how MMI publicly manage the tensions between two professional regimes as that management unfolds, rather than as subjects would retrospectively reconstruct it under interview conditions. What direct observation of platform performance yields is therefore not a substitute for interview data but a methodologically distinct access point, one that reveals the structural logic of the reconfiguration process rather than the practitioner's own account of it.

5.4.1 Data for examining the Platform Organizational Professionalism

Social media platforms are technologically mediated spaces owned by organizations to facilitate and shape human interactions that produce the saleable commodity, namely the behavioral data of participants. Because they depend on users (in this context, prosumers) to produce value, organizations need to manage their work, especially the work of content creators, who attract thousands of prosumers to deliver data (Garcia-Calvo; Kenney; Zysman, 2023) and who propagate the positive myths about the career that attract aspirants (Duffy; Wissinger, 2017). Thus, these individuals are the engine of the self-renewing supply of data producers.

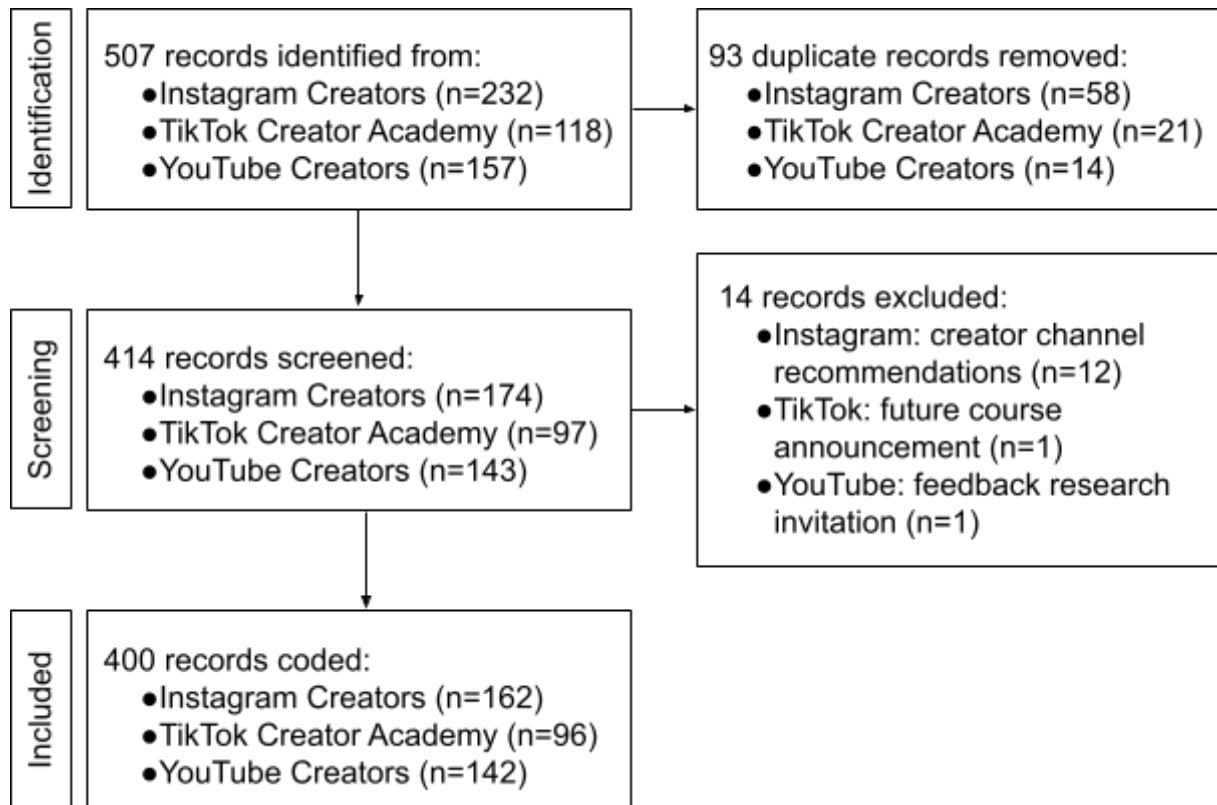
Regulation, curation, and moderation (the three governance strategies) exercise the role of work management. Among the regulatory instruments, dedicated learning centers are fundamental to the discussion of this thesis. These are sessions with educational material aimed at those who want to make a living from content creation, containing text and video tutorials and best practice guides. These instructions specify what individuals should know and do and how they should behave to achieve success. The platforms present these competencies as a prerequisite for creators to monetize their activity. Therefore, these instructions embody the essence of the professional discourse.

To understand how these platforms articulate competencies within an organizational discourse of professionalism, Araujo, Dalmoro, and I analyzed Instagram, TikTok, and YouTube instructional materials for content creators (Pithan, Araujo, Dalmoro, 2025). These three were selected because they are the most popular among Brazilian creators (Navarro, 2024) and are identified in global scholarship as important platforms that structure the creative economy, concentrating dominant models of content production, metric visibility, and interaction between creators and audiences (Durmuş-Şenyapar, 2025). They all have revenue-sharing programs for creators, although Instagram has not yet expanded it to Brazil, as of the date of this research.

Data construction from instructional material took place between September 30 and October 5, 2024, beginning with access to all texts and video transcripts available on the homepages and subsections of creators.instagram.com/, tiktok.com/creator-academy/, and youtube.com/creators/. We copied all instructional units found in these sections, including their homepages, subsections, and linked resources (such as help centers). Rather than employing theoretical saturation as a stopping criterion, we selected all available instructional content from these official creator education sections. Since the instructional corpus was finite and publicly accessible, we adopted an exhaustive selection strategy initially. However, consistent with grounded theory, the analysis process remained iterative. We treated the entire corpus as a field of data wherein we engaged in theoretical sampling within the copied documents, returning to specific sections (e.g., monetization rules vs. community guidelines) to refine emerging categories and check analytical hunches as the dimensions of Platform Organizational Professionalism (POP) crystallized (Charmaz, 2014).

The corpus of Phase 1 comprised instructional units, defined as self-contained pieces of text or video directed at content creators, extracted from the official educational sections of Instagram, TikTok, and YouTube. We copied 507 such units into an interactive online spreadsheet; after eliminating 93 duplicate records and removing 14 records for irrelevance, 400 were retained for coding (as shown in Figure 4). The excluded records were not instructions to creators, consisting of recommendations of Instagram creator channels (n=12), an announcement of an upcoming TikTok course (n=1), and an invitation to a YouTube feedback survey (n=1).

Figure 4 — First phase document selection and inclusion process



Source: Pithan, Araujo, and Dalmoro, 2025, p. 2

Since competency is the mobilization of resources necessary for performing activities in a given work context and according to specific demands (Durand, 2015; Le Boterf, 2003; Zarifian, 2003), we sought descriptions of processes in the instructional units. To identify them, we followed the format recommended by Brandão (2017) for writing competencies, in which expected behavior is described with a verb and object of the action, including, when possible, a condition and quality criterion for execution. This action-oriented definition of competency aligns with Charmaz's (2014) methodological instruction to code with gerunds. By focusing on what the platforms instruct creators 'to do' (e.g., optimizing, complying), we avoided static topical coding and captured the active, processual nature of the platform's professional demands. These descriptions were shortened to a few words so that the codes could be named.

Each instructional unit was treated as a single incident for the purposes of initial coding, consistent with Charmaz's (2014) incident-by-incident approach. Initial and focused coding followed an inductive logic, and codes emerged directly from the data. The instructional quality of the data, which was meant to be clear and useful, made it easier to code consistently. Instead of a pure inductive emergence, the analysis involved a dialogue between the data and existing theory, where observations stimulated theoretical questions and

concepts sensitized the interpretation of the data (Charmaz, 2014). For example, when the platform instructions prescribed the continuous optimization of metrics, we recognized parallels with Fournier's (1999) concept of disciplinary professionalism, which guided subsequent analyses of how platforms govern creative labor.

The analysis consisted of three single-platform rounds using Google Sheets. Each platform had a dedicated tab that was duplicated for independent coding. In each phase, Araujo and I independently performed initial and focused coding directly on the 400 instructional units, recorded in separate tabs, and reached categorization by consensus in a shared tab. Given the instructional and prescriptive nature of the data, independent coding by two researchers frequently resulted in similar or identical codes. Dalmoro then reviewed the description of decisions and the definitions of codes and categories recorded in memos in a duplicated tab, proposing adjustments and improvements in the competency approach. The final data structure of each phase was used as a point of constant comparison in the coding of the next platform. Figure 5 represents the phases of the analysis process.

Figure 5 — First phase analysis process

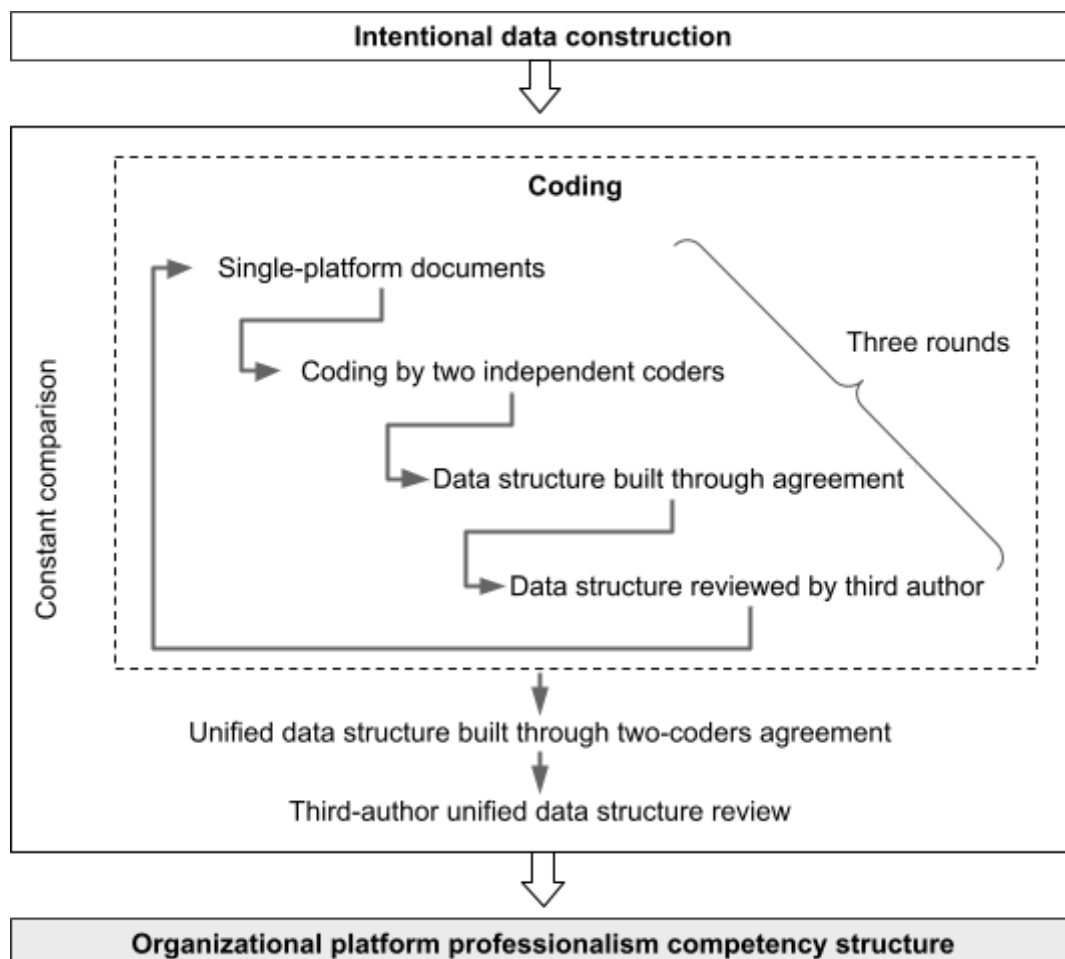
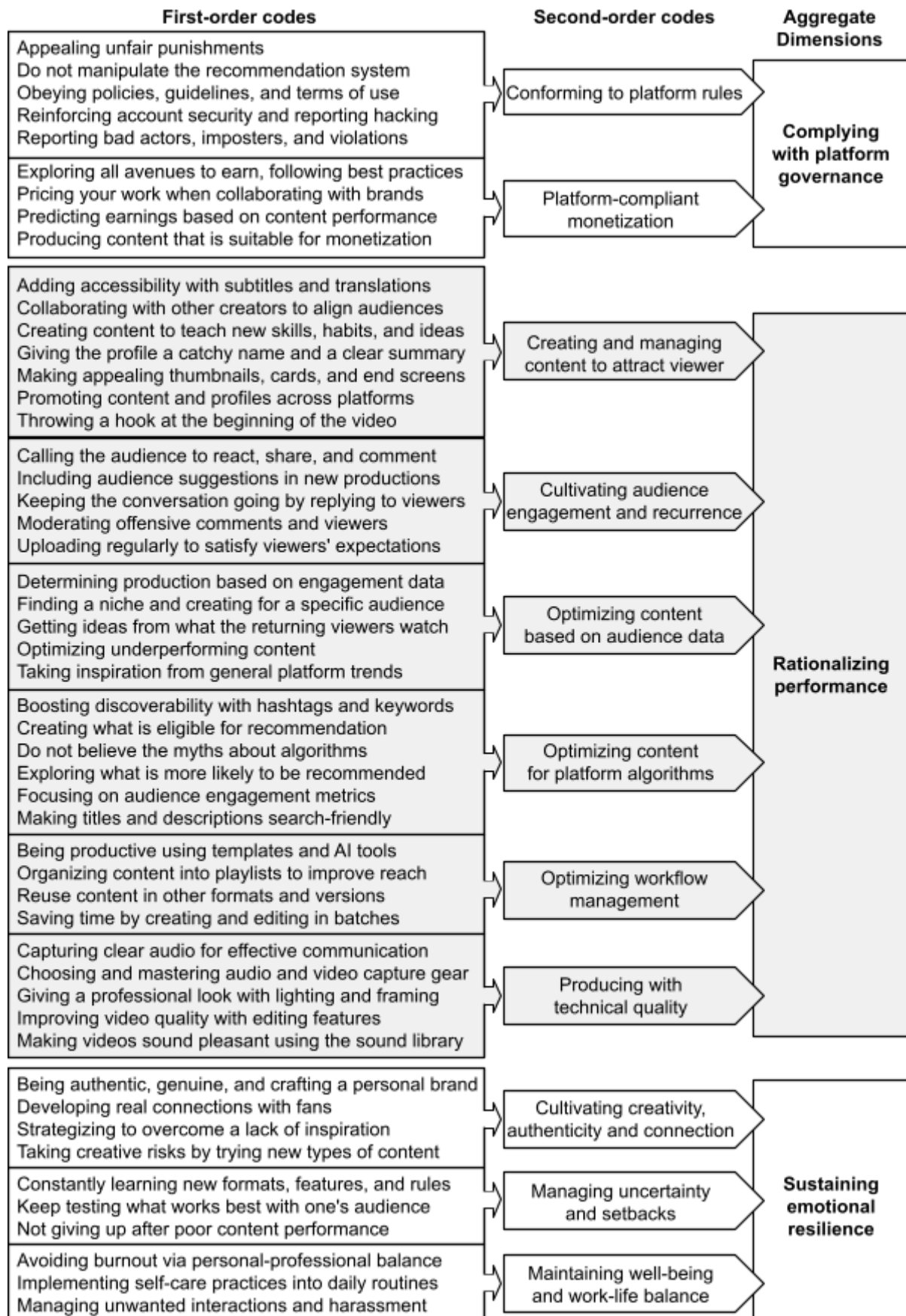


Figure 6 — First phase data structure



Source: Adapted from Pithan, Araujo, and Dalmoro, 2025, p. 3

At the end of the iterative coding cycles, we identified 51 individual competencies as first-order codes (e.g., giving the profile a catchy name and a clear summary, calling the audience to react, share, and comment, boosting discoverability with hashtags and keywords). These were grouped into second-order codes according to the expected results of these competencies for the content creator (e.g., monetization, engagement, workflow management, well-being). The competency results were then grouped into three analytical dimensions (Gioia; Corley; Hamilton, 2013) that define the creator's role in relation to platforms, their content production, and their career. These aggregate dimensions of interdependent competencies that comprise organizational professionalism are Complying with platform governance (related to following rules), Rationalizing performance (content production and audience management), and Sustaining emotional resilience (involving creative and well-being challenges). Finally, the integration of these three dimensions revealed a recursive mechanism that we termed the Performative Credential. This core category, the central concept that integrates the analysis and accounts for the variation in the observed patterns, explains how professional legitimacy is continuously granted and revoked by the platform's sociotechnical systems, contrasting with the stability of traditional occupational credentials.

Figure 6 shows the complete data structure, as recommended by Gioia, Corley, and Hamilton (2013). The findings are presented in the results' discussion chapter 6. The following sections address the data construction for the second and third phases of analysis.

5.4.2 Data for examining Medical Occupational Professionalism

To further understand how Medical Occupational Professionalism (MOP) is discursively articulated in digital contexts, this thesis examined the Brazilian medical code of ethics, the resolution on medical advertising, and the advertising manual.

The medical code of ethics (CFM, 2019) compiles the set of principles and responsibilities that regulate professional conduct. It sets the premise that medicine cannot be practiced as a commercial enterprise and must prioritize the patient's well-being before profit. CFM Resolution No. 2336/2023 regulates medical publicity and advertising, establishing the ethical boundaries between marketing for customer acquisition and educational communication in digital environments. Its creation involved three years of debates and public consultation to modernize ethical standards for the "era of digital medicine and constant technological evolution" (CFM, 2023, p. 9). The regulation authorizes the use of the physician's own social networks to form, maintain, and expand their clientele, provided they follow strict rules against sensationalism and unfair competition. The medical advertising

manual (CFM, 2024) instrumentalizes the application of the resolution through interpretative comments, functioning as pedagogical material that translates the abstract norm into concrete examples of self-presentation and the performance of authority on social media.

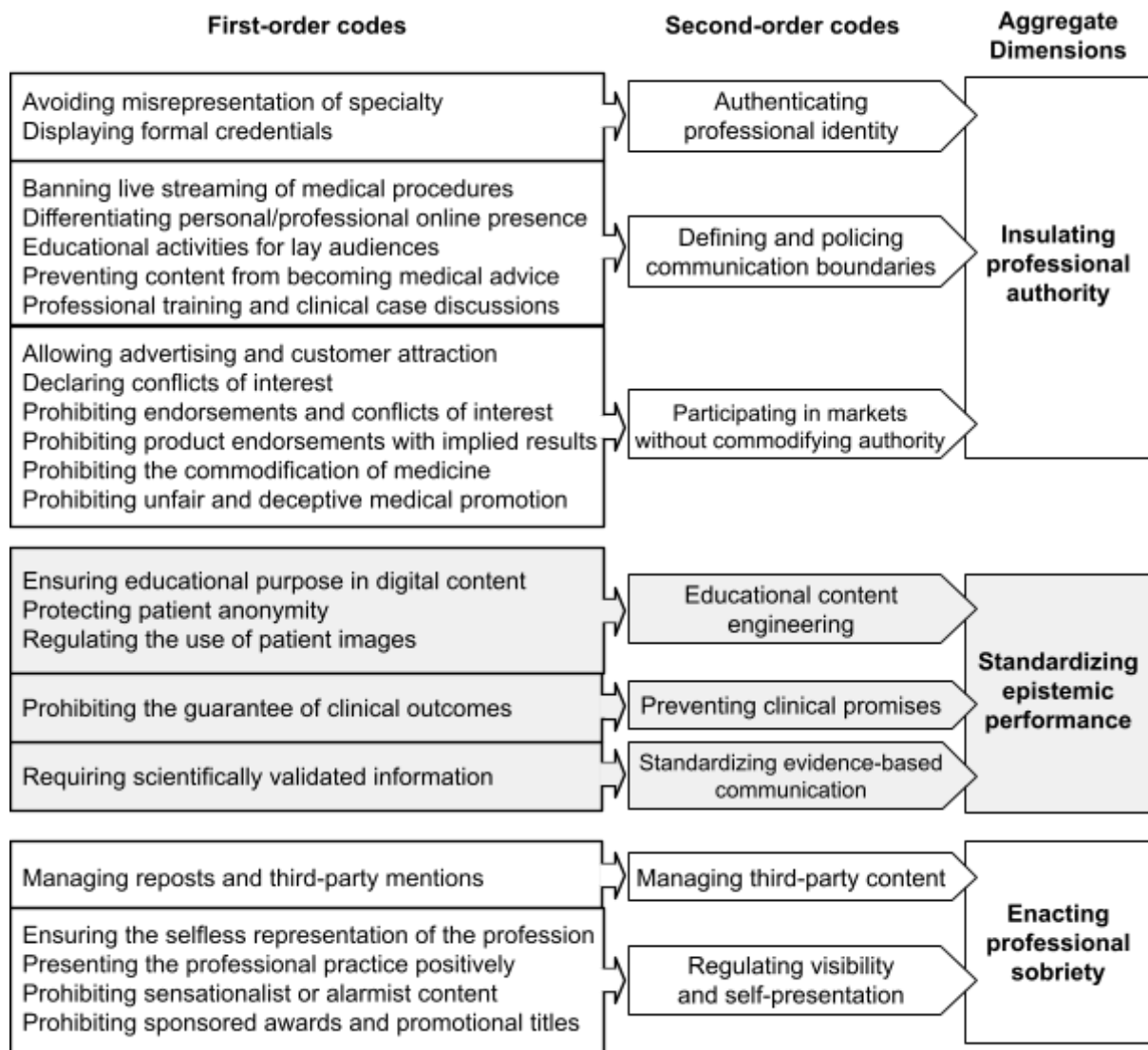
The documents function as the prescriptive corpus of occupational professionalism, according to Evetts' (2013) ideal type, playing a role analogous to the instructional units of the platforms. While the platforms prescribe competencies to respond to economic logic (organizational professionalism), the CFM guides behaviors, attitudes, and conduct to meet medical ethics and patient safety (occupational/collegial professionalism). If data are socially structured fictions (Glaser, 2007), the CFM creates a fiction of how medicine 'should be'. Thus, the first and second phases compared two sets of regulations of how content creators and physicians 'should behave', specifically those of the platforms and those of the CFM, which allowed the next phase to study how medical influencers deal with them.

Rather than attempting to capture the full, performative complexity of professionalism, the analysis focused on the regulatory prescriptions and institutional ideals projected by the Council. These documents represent a formal effort to negotiate medical identity within a digital reality, providing a discursive representation of the occupation that could be apprehended and contrasted with platform prescriptions to reveal structural tensions.

Theoretical sampling guided the data construction and analysis, in line with constructivist grounded theory. The results of the previous phase (POP) served as sensitizing concepts. The coding was performed using ATLAS.ti software entirely manually, without activating the tool for code suggestions using artificial intelligence. The first document analyzed was the Medical Code of Ethics (CEM), establishing the ethical and abstract basis of the profession (9 codes). The analysis followed a progression of complexity. The second round examined CFM Resolution No. 2336/2023 (10 new codes identified), of an operational nature, and the third document, the Medical Advertising Manual (5 new codes identified), focused on the tactics of conduct. The low rate of new codes in the final round indicated saturation at the descriptive level. The 24 initial codes, treated as 1st-order concepts (e.g., prohibiting the commodification of medicine, requiring scientifically validated information, protecting patient anonymity), were subjected to focused coding, generating 2nd-order themes (e.g., authenticating professional identity, participating in markets without commodifying authority, standardizing evidence-based communication).

Through constant comparison, these themes were distilled into three aggregate dimensions (as shown in the data structure in Figure 7).

Figure 7 — Second phase data structure



Reflecting the constructivist emphasis on action and process, the dimensions were named using gerunds to capture what the regulations do rather than merely what they are. The aggregate dimensions of Medical Occupational Professionalism are Insulating professional authority (related to the definition and protection of professional territory), Standardizing epistemic performance (related to the regulation of content structure), and Enacting professional sobriety (related to the regulation of public self-presentation). The integration of these three dimensions into a mutually reinforcing regulatory system revealed the core category for this phase of analysis, namely Fiduciary Authority Protection. This category designates the central organizing logic through which the CFM seeks to shield professional status against the commercial logic of platforms while reaffirming the character of medicine as a ‘means-activity’. Similar to the Performative Credential identified in the previous phase,

Fiduciary Authority Protection serves as the central finding of this analytical stage, transitioning to the final theory as one of the two structurally incompatible regimes that influence the actions of MMIs.

5.4.2.1 Cross-regime theoretical comparison

This analytical stage required no additional data construction. The comparison proceeded from the categories already constructed in the two preceding stages, treating them as the comparative material for mapping structural incompatibilities between regimes.

The constant comparison between Platform Organizational Professionalism (POP), established in the first phase of this study, and Medical Occupational Professionalism (MOP), examined in the first stage of this phase, advanced to the theoretical level. The comparison involved contrasting the platform's competency categories with the categories of occupational medical norms. Instead of comparing raw incidents, this stage focused on comparing aggregate dimensions across the two regimes to identify structural tensions. Three axes of incompatibility emerged from this comparison: commodification versus the anti-commodification firewall, visibility-driven content production versus evidence-based communication, and personal branding versus professional sobriety. These axes defined the spaces where the imperative of one regime directly violates the other, as when Rationalizing Performance (from POP) conflicts with Standardizing Epistemic Performance (from MOP), and established the relational structure that grounded the theoretical argument of the thesis.

From this comparison, I constructed a chart of structural incompatibilities and mutually exclusive professional identities, which synthesizes the ontological incompatibility between the two regimes and maps the relationships between categories at an abstract theoretical level (Charmaz, 2014; Gioia; Corley; Hamilton, 2013). This result, presented and discussed in section 6.3, revealed that incompatibility is not absolute and that there are non-confrontational spaces through which medical influencers can move.

The section below explains the data construction for the third phase of this thesis.

5.4.3 Data to examine the practices of medical influencers⁶

To investigate how medical influencers deal in practice with the incompatibility between POP and MOP, it is necessary to select individuals whose ‘professionalization’ as

⁶ Note on prior publication: An initial stage of this analysis (focused on YouTube) was presented at the XXVIII SemeAd seminar, co-authored with Lisiane Quadrado Closs (Pithan; Closs, 2025). Phrases from the description of the data construction method from the specific platform were used in writing this chapter.

content creators is unquestionable. Thus, I decided to identify individuals who met three criteria. The first two requirements were that the individual participated in the three main platforms most relevant to the Brazilian creator economy, Instagram, TikTok, and YouTube (Navarro, 2024), and maintained simultaneous activity on all three, since cross-platform production is a characteristic trait of creators (Abidin, 2018; Meisner, 2023) to optimize interactions and revenue (Ma; Gui; Kou, 2023). The third selection criterion was that the individuals must be physicians with more than 1 million followers on at least one of the chosen platforms, which is a characteristic of mega-influencers (Hawley, Ismail, 2023). Although platforms require lower numbers for monetization (see Table 5), having over 1 million followers leaves no doubt about the ‘professional’ character of the activity, indicating that individuals follow highly effective practices in boosting metric performance and have the material conditions to live exclusively from digital production, even if they do not.

5.4.3.1 Selection criteria and identification of medical mega-influencers

The search for these medical mega-influencers (hereinafter MMI) began with rankings available on HypeAuditor⁷, an influencer marketing analytics platform that provides analysis reports, research tools, and evaluation of the global influencer sector. This platform stands out from other alternatives, such as Modash and Social Blade, by offering rankings by country and niche for Instagram and YouTube content creators. There are no TikTok creator rankings for specific niches, only by country. However, the absence of a TikTok profile was an exclusion criterion in the selection of creators for this research.

I created a temporary trial subscription to access data unavailable through open access. The selection began with capturing the Top Health & Self Help YouTube Channels in Brazil ranking on June 20, 2025. HypeAuditor classifies YouTube channels based on the average number of views, comments, and reactions (likes and dislikes) from all videos published in the previous month. There were three rankings with different ordering criteria, including 50 most followed channels, 50 most viewed, and 50 most commented. After detecting channels that appeared in more than one list, I identified a total of 108 YouTube channels. The Instagram ranking was based on the number of followers and the level of engagement (measured by likes and comments) of non-private Instagram accounts with over 10K followers who had posted in the last 30 days and contained 100 profiles.

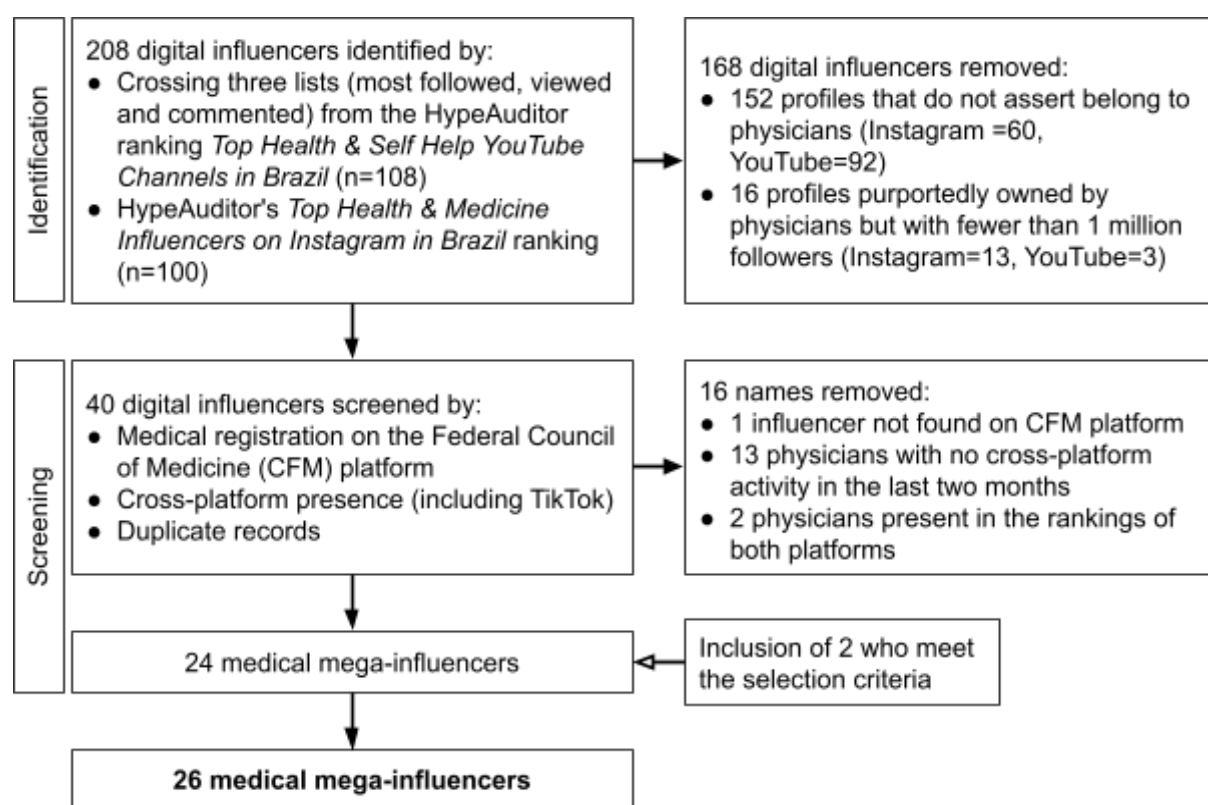
Identification continued with individual verification of profiles on YouTube and Instagram (N=208) and the removal of 152 organizations and individuals who did not identify

⁷ <https://hypeauditor.com/>

as physicians and 16 physician profiles with fewer than 1 million followers on the data construction date.

I searched for the remaining 40 names on the Federal Council of Medicine (CFM, 2025) website, and one was removed because, although the influencer reported having recently completed medical school, they were not registered on the council's platform. I searched for the remaining 39 physicians on the three selected social media platforms, excluding 13 creators that were either missing from one of the platforms or had not published on one of them in over 60 days as of the date (June 20, 2025). Duplicate records for two physicians who appeared on both HypeAuditor lists were eliminated. Two more physicians were then added because they met all the criteria. Ultimately, 26 creators were included in the selection (see Figure 8).

Figure 8 — Selection of research subjects



Using the information available on CFM (portal.cfm.org.br/busca-medicos), I created a spreadsheet that includes the names, registration numbers with the Regional Council of Medicine of the Brazilian state of practice (CRM), Specialist Qualification Registration numbers (RQE), specialties, training institutions, and years of graduation. A compact version of this information can be seen in Table 5.

Table 5 — Selected medical mega-influencers: specialty and graduation year

ID ⁸	Specialty and genre	Graduation
SURG1	General surgeon - Male	2002
PSYCH1	Psychiatrist - Female	1989
PED1	Pediatrician - Female	1983
GYN	Gynecologist - Male	2012
CARD1	Cardiologist and Internal Medicine specialist - Male	2002
GP1	Without a specialty / General practitioner - Female	2013
SURG2	General surgeon - Male	2012
DRM	Dermatologist - Female	2012
PED2	Pediatrician - Male	1982
F/CM	Family and Community Medicine specialist - Male	2007
ANG	Angiologist and Vascular surgeon - Male	1998
GP2	Without a specialty / General practitioner - Male	1967
CLS	Colorectal Surgeon - Male	2001
ORTH	Orthopedist and Traumatologist - Male	2003
GIS	Gastrointestinal surgeon - Male	1997
GP3	Without a specialty / General practitioner - Female	2023
PS	Plastic surgeon - Female	1992
CARD/NUT	Cardiologist, Nutrology physician - Male	1972
GP4	Without a specialty / General practitioner - Male	2019
GP5	Without a specialty / General Practitioner - Male	2012
PSYCH2	Psychiatrist - Male	2007
GP6	Without a specialty / General practitioner - Female	2017
IDS	Infectious Diseases specialist - Male	2015
CARD2	Cardiologist, Internal Medicine specialist - Male	1985
CARD3	Cardiologist and Clinical cardiac electrophysiologist - Male	2008
CARD4	Cardiologist - Male	1974

Although the university's ethics committee approved the identification of individuals whose data were drawn from content they have made publicly available on social media, full names have been omitted as a precaution. During the course of this research, one of the physicians in the sample was the target of a lawsuit initiated by the Brazilian Attorney General's Office (Advocacia-Geral da União) and convicted by the courts for publicly

⁸ Although the university's ethics committee approved the identification of individuals whose data were drawn from content they have made publicly available on social media, full names have been omitted as a precaution. During the course of this research, one of the physicians in the sample was the target of a lawsuit initiated by the Brazilian Attorney General's Office (Advocacia-Geral da União) and convicted by the courts for publicly contradicting the scientific consensus on mammographic screening. He, in turn, initiated legal proceedings against the YouTube channel of two science communicators who reported his judicial conviction. Researchers who require the complete list of participants may contact the author directly (liana.pithan@gmail.com).

contradicting the scientific consensus on mammographic screening. He, in turn, initiated legal proceedings against the YouTube channel of two science communicators who reported his judicial conviction. Furthermore, this decision aligns with the grounded theory imperative of maintaining analytical abstraction, ensuring that the focus remains on the emergent social processes and conceptual patterns rather than on the individuals themselves (Charmaz, 2014; Glaser, 2019).

5.4.3.2 Data construction across platforms

Once the 26 MMI were selected, data construction began. The first stage focused on YouTube, as it is the platform that provides the most space for text insertion in the channel bio (description) and the most evidence of participation in reward programs. The data from this phase were constructed by capturing the following information, namely (1) YouTube channel's name; (2) full channel description; (3) profile picture; (4) channel banner image and text; (5) external links; (6) tags; (7) playlist titles; (8) description text of the twenty most recent videos; (9) the presence of evidence of participation in YouTube's partnership program in the form of the 'Join' (Channel Memberships) and 'Super Thanks' features, respectively, a monthly subscription to access symbolic benefits or exclusive content and a button to donate single amounts to the creator; (10) the presence of the 'From a registered physician in Brazil' feature, granted by YouTube upon request from creators who prove their profession through professional associations; (11) whether videos are boosted as ads via Google Ads Transparency Center; and (12) whether videos received paid promotions.

To capture information amenable to automated extraction, I wrote two scripts in Python using the official YouTube API (Application Programming Interface) library (`googleapiclient`), with the initial code generated by Google Gemini (Pro Plan) and subsequently debugged with ChatGPT (Free Plan) assistance. Starting from a list of 26 unique identifiers (channel IDs) for the selected channels, two complementary extractions were performed in June and July 2025. In the first extraction, the script accessed the API's *channels().list()* endpoint to obtain public information from each channel, such as name (title), institutional description (description), creation date (*publishedAt*), number of subscribers (*subscriberCount*), cumulative views (*viewCount*), and total number of published videos (*videoCount*). Following initial analysis, a second extraction was conducted using the *search().list()* endpoint followed by *videos().list()* to retrieve additional metadata from the twenty most recent videos of each channel (id, title, description, publication date, number of views, likes, and comments), consistent with grounded theory's iterative approach to data

construction. The data were organized in CSV (comma-separated values) format files and uploaded to Google Sheets.

Information beyond the reach of automated copying was manually copied (external links after channel descriptions and the features ‘Join’, ‘Super Thanks’, and ‘From a registered physician in Brazil’) and entered into the spreadsheet. I also analyzed profile pictures and banner images directly on YouTube, recording observations in analytic memos, and manually obtained information from the Google Ads Transparency Center on whether the influencers received paid product placements, sponsorships, and endorsements that have been declared on YouTube and paid for traffic boosting. However, the search for paid traffic must be done using the name of the entity that paid for the boosting, not the creator’s name, which may have caused false negatives. When publicly available, I also used the names or National Registry of Legal Entities (CNPJs) of the influencers’ companies to conduct the searches. After a preliminary analysis of the corpus, I converted the table into individual text files for each influencer and uploaded them to atlas.ti software for coding.

While the first two analytical phases constructed the structural architecture of POP and MOP as ontologically distinct professional regimes, the third phase pursued the central theoretical question of this thesis by investigating how do medical influencers navigate the impossibility of dual adherence. This shift from structural mapping to processual explanation justifies more detailed methodological exposition of Phase 3’s coding procedures, as the mechanisms and core category emerging here constitute the thesis’s substantive theoretical contribution.

5.4.3.3 Coding process

Initial coding remained open to what emerged from the data, without attempting to force them into categories established in earlier phases of the research. I first coded data from channels with over 1 million YouTube subscribers (N=14), then data from the remaining 12 channels. The emergence of new codes decreased as the process advanced but did not cease entirely, yielding 43 initial codes in this phase. Throughout data construction and analysis (June through August 2025), I maintained a comparative spreadsheet to identify cross-cutting patterns and exceptions across all influencers while using ATLAS.ti for granular coding of individual strategies. Across all three platforms, I investigated the use of paid promotion tools, such as Meta ads (Facebook/Instagram), TikTok’s Promote tool, and YouTube’s in-feed ads via Google Ads, which creators use to extend the reach of their content beyond what is possible organically.

The next phase consisted of copying from the physicians' Instagram profiles: (1) username; (2) full name; (3) biography; (4) profile picture; (5) business account status; (6) business category name; (7) external URLs; (8) hashtags from the twenty most recent posts; (9) captions for the twenty most recent posts; (10) whether posts are boosted as ads; and (11) whether the profile offers a monthly subscription option for exclusive content. I obtained the first nine types of information using the automated web scraping tools provided by the Apify platform `instagram-profile-scraper-task`. Next, I manually checked whether influencers offered paid subscriptions and searched the Meta Ad Library, the public repository of ads served on Facebook and Instagram, to verify the presence of promoted content. The web scraper delivers data in CSV format, which I added to the comparative spreadsheet and converted into individual text files for each influencer for coding in ATLAS.ti. Three new initial codes emerged from the data in this phase.

The third round of data construction addressed TikTok. Using the `clockworks/tiktok-scraper` tool available on the Apify platform, I copied (1) profile picture; (2) channel description; (3) nickname; (4) business category; (5) business account status; (6) external URL; (7) whether the creator sells products through TikTok; (8) hashtags from the twenty most recent videos; (9) captions for the twenty most recent videos; (10) whether posts are boosted as ads; and (11) whether posts are sponsored. The CSV data were added to the comparative spreadsheet and transformed into individual text files for each influencer for coding in ATLAS.ti. This phase consolidated the identification of patterns and recurrences but did not yield new initial codes. Profile images were analyzed, and observations were recorded in memos. Meta ads, TikTok's Promote tool, and YouTube's in-feed ads (via Google Ads) are all paid promotion tools that creators use to extend the reach of their content beyond what is possible organically.

The initial analysis of TikTok data yielded no new codes compared to the previous rounds (YouTube: 43; Instagram: 5; TikTok: 0). While this repetition might suggest saturation in a descriptive sense, the comparative analysis revealed that these data lacked the necessary conceptual density to explain the dynamics of self-presentation mechanisms. As Charmaz (2014) warns, researchers often mistake the repetition of events or stories for theoretical saturation. Confronted with what Dey terms 'theoretical insufficiency' (1999, as cited in Charmaz, 2014, p. 257), I decided to discard the premature categorization derived from these 'thin' data. Instead, I opted for a strategic redirection of data construction to ensure depth. Using a custom Python script, I captured transcripts from 20 YouTube videos per medical influencer, resulting in a corpus of 520 transcripts. This return to the field was to secure a

reservoir of rich data (Charmaz, 2014) robust enough to allow for internal theoretical sampling and to capture the full variation of the observed mechanisms.

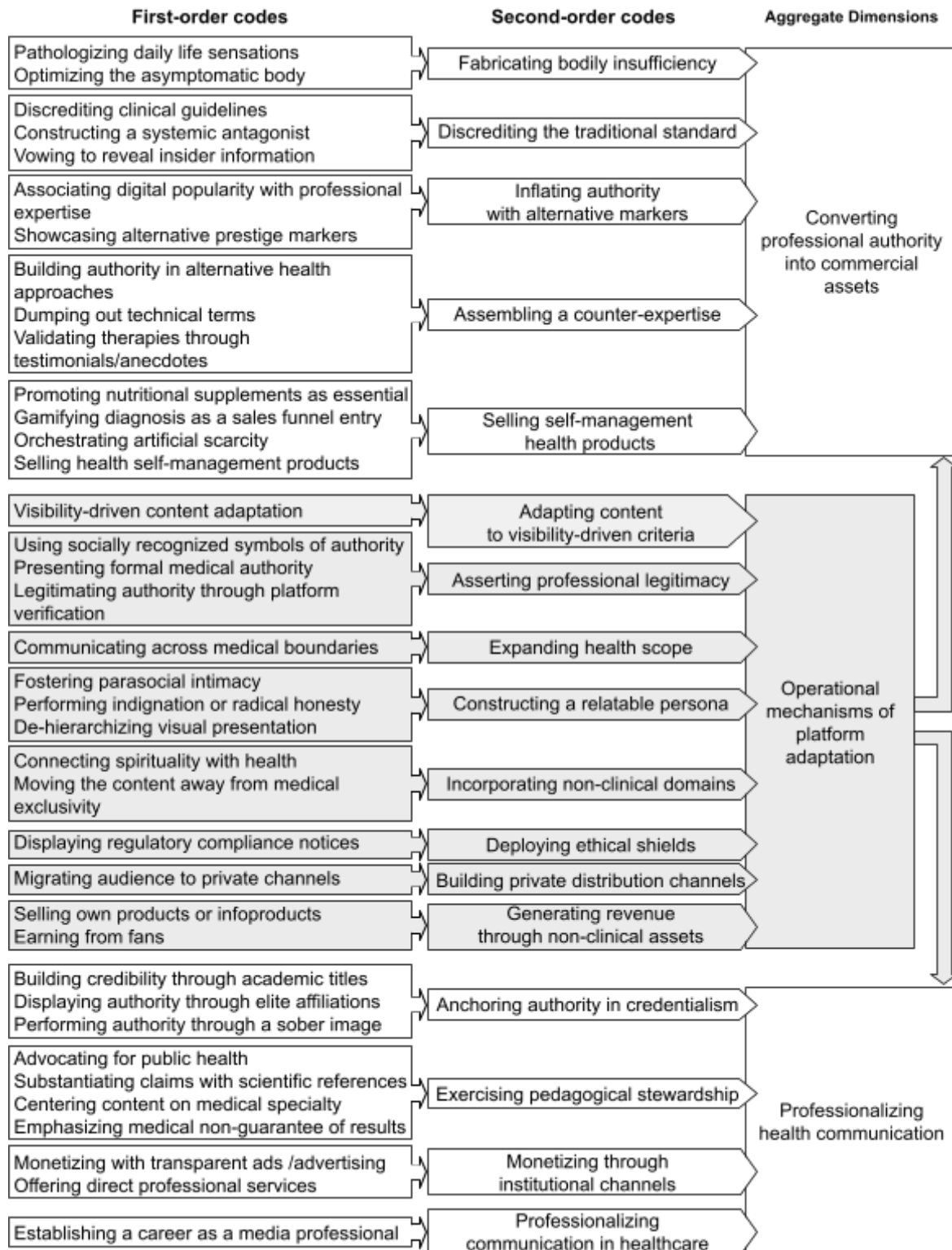
I selected YouTube as the primary source for deep analysis for three strategic reasons, starting with its dominant reach, as it is the leading platform for audience reach among the 26 medical influencers, totaling over 53 million subscribers compared to Instagram's 41 million followers and TikTok's 10 million (as of June 26, 2025). Analysis of content distribution patterns indicates YouTube functions as the central hub for long-form informational content, while Instagram serves as a visual interaction space and TikTok remains an experimental frontier for most physicians in this sample. The second reason involves data richness and accessibility, since YouTube enables automated transcript extraction via API, providing dense textual data essential for analyzing discourse nuances. In contrast, Instagram does not provide native transcriptions, and TikTok, despite offering this feature, represents the platform with the least reach and depth of argumentation. Finally, the platform offers monetization maturity, whereas TikTok's monetization features are still relatively nascent in this sample. Analyzing YouTube content, therefore, allows for a more robust understanding of the dynamics influencers develop to navigate the tensions between POP and MOP.

The selection of transcripts for in-depth analysis was conducted manually through purposive sampling, guided by three sensitizing questions designed to capture the structural tensions between the regimes, namely whether the video is exclusively informational or also selling a product, whether the MMI performs sober medical authority or personal branding, and whether the MMI grounds the content in scientific evidence or in alternative/vague epistemic claims. This theoretical selection resulted in 58 video transcripts analyzed in depth. To ensure representativeness without redundancy, MMIs with highly consistent or monothematic conduct had two videos analyzed, while those exhibiting greater variation in their strategies had three videos included.

I documented these dynamics in memos, moving from descriptive observations to relational propositions. The initial open coding generated a set of 48 provisional codes. To ensure analytic precision, I subjected this list to a refinement process involving: (1) merging hyper-granular codes that described identical phenomena; (2) segmenting codes that concealed subtle but theoretically significant variations; and (3) discarding codes that lacked analytic power to address the research problem. This rigorous filtering resulted in 38 refined initial codes (e.g., discrediting clinical guidelines, selling health self-management products, substantiating claims with scientific references), documented in the codebook and visualized in the data structure presented as Figure 9.

The lower rate of new codes in later transcripts, combined with the theoretical coherence among emerging categories, indicated that the analysis had achieved saturation, in the sense that categories possessed sufficient “theoretical completeness” (Charmaz, 2014, p. 213) to explain the phenomenon and relationships among major categories were clear.

Figure 9 — Third phase data structure



The logic in this phase was not purely inductive. When confronting the transcripts, the analysis sought to infer the best possible explanation (Kempster; Parry, 2014) for the observed patterns. When a medical influencer simultaneously employs formal credentialing indicators alongside platform-native entertainment formats, the analytical question shifts from ‘what is happening?’ to ‘why is this necessary?’, as recommended by Bhaskar (2008). The answer, that neither regime in isolation is sufficient to sustain the medical influencer’s position, constituted a retroductive inference regarding the causal mechanisms at play (Kempster; Parry, 2014), which progressively consolidated into the theoretical argument of this thesis.

5.4.3.4 Theoretical coding and core category construction

After initial coding was completed, I advanced to focused coding, selecting the most analytically significant among the 38 refined initial codes and testing them against all 58 transcripts, generating 17 second-order codes (e.g., fabricating bodily insufficiency, anchoring authority in credentialism, exercising pedagogical stewardship).

The second-order codes were then grouped into three aggregate dimensions, theoretical categories that capture the relational patterns among codes and connect empirical data to the conceptual level (Gioia et al., 2013). These dimensions represent the strategies MMI employ to manage the structural incompatibility between POP and MOP: Converting Professional Authority into Commercial Assets, Professionalizing Health Communication, and Operational Mechanisms of Platform Adaptation (OMPA).

In theoretical coding, I examined how these three dimensions relate to each other as components of a single explanatory framework (Glaser; Holton, 2023). The analysis showed that the two opposing vectors and the transversal mechanisms they share are not independent strategies but dialectical responses to a single irreconcilable structural condition.

Performatively Reconfiguring Professional Authority emerged from these three dimensions as the core category. This category designates the social process by which MMI responds to the structurally incompatible coexistence of MOP and POP through the continuous, performative reconfiguration of professional authority. The authority is reconfigured performatively because it must be staged responsively to audience consumption data and platform recommendation systems to sustain legitimacy. This core category sustains **Performative Reconfiguration of Professional Authority**, the substantive theory proposed in this thesis.

The process operates through two opposing vectors (represented by the first two dimensions). Each initial code is analytically exclusive to one vector, in that its empirical

occurrence is invariably embedded in that vector's teleology; however, no single initial code is a necessary condition for the vector's instantiation, since individual units of analysis activate distinct subsets of the available codes depending on the specific performative strategy enacted. The third dimension (OMPA) does not represent a distinct identity strategy, but rather the synchronous and transversal instruments MMI employs to secure the visibility and engagement metrics that guarantee relevance in the digital ecosystem. Consequently, these transversal codes were positioned as a mediating category between the two dimensions in the data structure (Figure 9), whereas the discussion chapter will explicate how they are operationally integrated into the two distinct vectors.

OMPA is present in both vectors, but its function and teleology diverge. In Vector 1, these mechanisms serve the conversion of professional authority into commercial assets; in Vector 2, the same mechanisms serve the professionalization of health communication. This teleological divergence (identical tools, opposing purposes) renders OMPA analytically distinct from the two vectors, yet constitutively linked to both. It represents the practical translation of POP competencies performed by MMI in the face of MOP demands.

5.4.4 Ontological grounding of the analytic structure

The two vectors identified in this analysis operate within the domain of the Actual, within critical realist ontology (Bhaskar, 2008; Archer, 2020). The structural conditioning (*T1*) in Archer's (2020) morphogenetic cycle comprises the platform's generative mechanisms and the preexisting institutional structure of the medical profession, whose incompatibility is activated in the domain of the Actual when physicians operate simultaneously under both. The prescribed disciplinary systems identified in Phases 1 and 2, POP and MOP, map the institutional prescriptions through which this structural conditioning reaches agents.

Consequently, the two identified vectors represent the domain of Social Interaction (*T2–T3*), encompassing the agentic responses through which medical influencers manage structural conditions they did not create. Crucially, structure conditions but does not determine, as physicians retain the reflexive capacity to orient their practice toward either the commercial logic of POP or the institutional logic of MOP, even as both orientations require adaptation to platform demands. This capacity, constrained but real, is what makes the reconfiguration performative rather than mechanically determined.

The Operational Mechanisms of Platform Adaptation (OMPA), described in section 6.4, represent the synchronic tools and competencies employed across both orientations. They

constitute the practical mediators through which the structural imperatives of the platform are instantiated at the level of individual performance.

The following chapter presents the findings organized according to the three phases of this investigation. Sections 6.1 and 6.2 detail the conflicting structural conditions (*T1*) governing the field through the visibility imperatives of Platform Organizational Professionalism (POP) and the regulatory boundaries of Medical Occupational Professionalism (MOP). Section 6.3 establishes the structural incompatibility between the two regimes through cross-regime comparison. Finally, Section 6.4 analyzes the domain of social interaction (*T2–T3*), presenting the substantive theory of Performative Reconfiguration of Professional Authority and the specific mechanisms by which physicians manage the incompatibility between these two regimes, and identifies the structural elaboration (*T4*) this process produces.

6 RESULTS PRESENTATION AND DISCUSSION

This chapter presents and discusses the findings of the three phases of this research. Section 6.1 examines the discourse of professionalism produced by Instagram, TikTok, and YouTube in their instructional material for content creators, identifying the competencies and disciplinary logic that constitute Platform Organizational Professionalism (POP). Section 6.2 reconstructs the normative-regulatory framework established by the CFM governing physician engagement in digital media, delineating the boundaries and imperatives of Medical Occupational Professionalism (MOP). Section 6.3 establishes the structural incompatibility between the two regimes through cross-regime comparison of their respective prescriptions.

Sections 6.1, 6.2, and 6.3 operate at the stage of structural conditioning (*T1*), mapping the institutional regimes whose incompatibility defines the field of action available to physician-influencers. Section 6.4 shifts to the stage of social interaction (*T2–T3*), presenting the substantive theory of **Performative Reconfiguration of Professional Authority**, which explains how the 26 medical mega-influencers examined in this study navigate, exploit, and selectively comply with these competing regimes in their digital practice, and identifies the structural elaboration (*T4*) produced by this process, namely the datafication of professional authority and the emergence of a substitute validation system.

6.1 Platform Organizational Professionalism (POP)⁹

This section presents and discusses the first phase of the thesis's research, which investigated the discourse of professionalism on Instagram, TikTok, and YouTube in their sessions aimed at instructing those who wish to become 'professional' content creators. This research was conducted with Araujo and Dalmoro with the aim of understanding the behaviors and practices that the platforms expect from a professional influencer and reward (Pithan; Araujo; Dalmoro, 2025). The results of this work were used for constant comparison in the following phases of the thesis, with one investigating the CFM's recommendations for physicians and the other analyzing the profiles and content of 26 medical influencers on the three aforementioned platforms.

⁹ Note on prior publication: The empirical research that gave rise to this chapter was developed in co-authorship with Wilian Fernandes Araujo and Mateus Dalmoro and presented at #AoIR2025: The 26th Annual Conference of the Association of Internet Researchers (Pithan; Araujo; Dalmoro, 2025). The final version published in the proceedings incorporated valuable suggestions from the three reviewers, one of whom identified herself as such: Issaaf Karhawi, from the University of São Paulo. Therefore, although it has a unique structure and formulations, this chapter may contain passages already used in the previous version.

From the 400 instructional units of Instagram, TikTok, and YouTube, three interdependent dimensions of competencies emerged that comprise Platform Organizational Professionalism (POP). These dimensions form an articulated disciplinary regime (Fournier, 1999). While Complying with platform governance (Dimension 1) defines the conditions for platform participation, Rationalizing performance (Dimension 2) determines the imperative under which the worker must act, and Sustaining emotional resilience (Dimension 3) assigns individuals the responsibility for dealing with structural contradictions. They form a recursive system that keeps workers disciplined by inciting fear of invisibility. Failure in one dimension causes compensation in the other.

In this articulation, the dimensions overlap to ensure the efficacy of the disciplinary regime. As Fournier (1999) argued, disciplinary power lies in the systematic connection between discursive elements and practices. On platforms, this interdependence means one dimension establishes the conditions for the others to operate. Complying with platform governance, for instance, serves as a provisional license to access the system, a prerequisite for pursuing the performance metrics required by the platform. Simultaneously, Sustaining emotional resilience absorbs the structural contradictions between these demands. They create a recursive system where failure in one dimension leads to increased intensity in others, while success requires constant adaptation to changing rules and automated curations and moderation systems.

POP's primary mechanism is the Performative Credential. Unlike traditional professional credentials, which are institutionally validated and relatively stable (Larson, 2017), the Performative Credential is validated by automated systems, requires the creator to demonstrate competence through performance metrics, and thus must be reaffirmed on a continuous basis.

6.1.1 Dimension 1: Complying with platform governance

The first dimension (D-1) refers to adherence to norms when participating on platforms. This condition applies to all actors, but it has a determining weight for the viability of creators' careers because not knowing or disobeying the rules can lead to loss of visibility, demonetization, and banishment. Table 6 shows the second-order codes that compose this dimension and the expected results of competencies for creators and platforms.

Table 6 — Analytical structure and outcomes of Complying with platform governance

Complying with platform governance • Conforming to platform rules • Platform-compliant monetization	Results for Creators Prevent penalties and demonetization to maintain and maximize revenue.
	Benefits to Platforms Govern labor indirectly through regulation, threatening violators with invisibility.

Source: Adapted from Pithan, Araujo, Dalmoro (2025)

These competencies are operationalized through platform-specific monetization eligibility criteria (Table 7), which function as gatekeeping mechanisms that condition professional viability on metric performance.

Table 7 — Platform monetization eligibility criteria and gatekeeping thresholds

	Monetization resource	Followers or subscribers / Consumption / Production volume / Other requirements
IG	Gifts on Reels	500 followers / Professional account, 18+ years old
	Badges on Live Streams	10K followers / 18+ years old
	Subscriptions	10K followers / 18+ years old
	Seasonal Bonuses / Reels	Minimum of 5 million bonus-eligible Reels views per month (last 3 months) / Account that publishes original content
	Breakthrough Bonus*	After acceptance into Meta's program, publish 10 Reels on Instagram within 30 days
TT	Creator Rewards Program	10K followers / 100K video views in the last 30 days / Videos longer than 1 minute / 1K views on the 'For You' feed to monetize each video
	Subscription Bonus	10K followers / 100K video views in the last month / Publish exclusive videos for subscribers
	TikTok LIVE	18+ years old
YT	Tier 1: Fan Funding (Memberships, Super Chat, Shopping for own products)	500 subscribers / 3K viewing hours (long videos/year) OR 3 million Shorts views (90 days) / 3 valid public uploads in the last 90 days
	Tier 2: Ad and Premium Revenue (watch page and Shorts feed)	10K subscribers / 4K viewing hours (long videos/year) OR 10 million Shorts views (90 days) / Comply with advertiser-friendly content guidelines
	Shopping (affiliate / third-party brand products)	10K subscribers / same hours/views requirements as Level 2 / Not an official music/artist channel
* USA only		

The creator acts as a 'regulated agent,' free to operate within the platform's rules, such as terms of service, guidelines, and monetization policies, which they must understand, interpret, and strategically follow to avoid penalties. YouTube (2024), for instance, advises

creators, “Don’t try to look for loopholes or try to lawyer your way around the guidelines—just understand them and try to respect the spirit in which they were created.” This way, creators are constantly reminded that compliance conditions their revenue streams.

This requires adapting production to platform compliance criteria, diversifying revenue streams, and interpreting performance metrics to predict financial returns. Instagram (2024) explicitly links monetization to audience engagement, stating that creators can “develop deeper connections with your most engaged followers and grow your recurring monthly income by giving subscribers access to exclusive content and benefits.” Such messages reveal how governance operates through both incentives and threats, in which creators who comply with the rules expect enhanced visibility and monetization, while those who violate them face penalties such as demonetization or invisibility. However, empirical research reveals the failure of this promise.

For platforms, the contractual layer that requires creators to be competent in compliance with rules is structural. It enables ecosystem governance by defining permitted behaviors, incentivizing desirable conduct (Siciliano, 2021), and ensuring content production, circulation, and visibility (Poell; Nieborg; Duffy, 2022). Supported by both infrastructural limitations and contracts, this model of governance at a distance transfers to participants the responsibility of self-management (Bucher, 2018). Creators’ self-regulation includes the responsibility of detecting and contesting moderation errors, as exemplified by TikTok (2024), which, after stating the removal of policy-violating content, encourages “everyone to appeal moderation decisions they believe were made in error.”

The expected benefit of these competencies is the guarantee of maintaining permission to use the platform, preserving its visibility and monetizing it. However, following the rules does not always ensure this because the rules change frequently (Niederauer; Maggi, 2022), and automated moderation is inconsistent and opaque (Caplan; Gillespie, 2020). Even individuals who have not violated the rules report experiencing reduced content visibility due to shadowbans or being flagged as unsuitable for advertisers (Caplan; Gillespie, 2020; West, 2018). Inequity in moderation and flagging disproportionately affect creators with small followings, marginalized groups, and minorities (Caplan; Gillespie, 2020; Duffy; Meisner, 2023; Haimson *et al.*, 2021). Appeals against punishment are often slow, limited to review requests, and rarely succeed, since platforms retain unilateral authority over the verdict (Kopf, 2020; Ma; Kou, 2022). Even when a demonetization appeal is successful, it does not recover the initial loss of revenue. Moreover, the platforms’ responses often depend on the

performance metrics of creators and their compliance with platform norms, favoring already privileged creators (Caplan; Gillespie, 2020).

The gap between prescribed work and actual work (Dejours, 2012) is evident in the discrepancy between the platform's security promises and the reality of unpredictable application. This reveals that compliance functions as discipline, not as protection. For creators, this situation amounts to unfair and unequal treatment.

Due to unreliable moderation, creators adopt preventative self-censorship strategies, such as avoiding controversial topics, adjusting metadata (Ma; Kou, 2021), altering words sensitive to automated moderation triggers (Kojah *et al.*, 2025), and using intertextual references and coded language (Shi, 2024; Shen; Haimson, 2025). This structural insecurity reveals that Complying with platform governance serves as a disciplinary tool that intensifies work and keeps creators in perpetual vulnerability, rather than a professional standard that offers protection or stability.

6.1.2 Dimension 2: Rationalizing performance

While the first dimension defines the conditions imposed on those who participate in the platforms, the second dimension (D-2) prescribes how to behave. To the creators, it refers to the strategic and practical tasks of creating, optimizing, and distributing content, as well as interacting with the audience, to maximize measurable performance and reach. This group of competencies elevates metric visibility as a central value of the occupation, replacing the discretionary judgment of the creative professional. Table 8 depicts second-order codes and the anticipated outcomes for creators and platforms.

Table 8 — Analytical structure and outcomes of Rationalizing performance

Rationalizing performance • Creating and managing content to attract viewers • Cultivating audience engagement and recurrence • Optimizing content based on audience data • Optimizing content for platform algorithms • Optimizing workflow management • Producing with technical quality	Results for Creators Increase engagement and visibility to meet significant metrics and attract brands. Benefits to Platforms Increase data production by setting high targets for content creators to be rewarded.
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Source: Adapted from Pithan, Araujo, Dalmoro (2025)

The creator is a full-fledged media and marketing professional who plans, executes, manages, and evaluates all aspects of their digital presence in response to audience preference data and platform automated curation. The platforms' instructions about these competencies are explicit and action-oriented toward visibility. Therefore, in addition to producing content, creators are expected to act strategically to maximize performance metrics, subordinating creative freedom to data-driven optimization.

Creators must first focus on creating and managing content to attract and retain viewers' interest, using practices such as Instagram (2024) instruction to "Get them hooked within the first few seconds of your videos," subordinating narrative structure to retention metrics.

To master this dimension, creators must optimize content based on audience data and automated curation requirements, making it legible to platform systems and, therefore, to their business logic. Instagram suggests analyzing what is "performing well" to "create more of it," while YouTube (2024) recommends "looking at what videos your audience is currently watching" to identify trending topics. This focus on data-driven production privileges measurable repetition as a safer strategy to meet performance metrics. TikTok (2024) further advises monitoring watch time to have "a holistic view of how users are consuming and engaging" with videos to identify areas for improvement, such as "adding a few key details" to make videos "more discoverable." Similarly, YouTube (2024) requires making use of "accurate keywords in your title, description, and file name" to increase discoverability.

Beyond technical optimization, Rationalizing performance requires managing interactions to ensure the continuous generation of engagement data. Content serves as the basis for building audience engagement and return through what platforms frame as authentic connections. Instagram (2024) advises creators to "keep the conversation going. Stay active in comments, engage, and show your community that you care and are listening." The emotional labor that the platform presents as an authentic connection works, for them, to generate engagement data.

Explicit prescriptions suggest creators must optimize their workflows to increase productivity by repurposing content across different formats, using templates and automation tools, and improving output by "batching similar tasks together," as exemplified by YouTube (2024). This operational focus turns the creative process into a managed, scalable workflow.

Highly engaging, visibility-optimized, and audience-centered content drives continuous data production, increases user retention and stickiness, and reinforces network effects (Parker; Van Alstyne; Jiang, 2016). Therefore, these competencies are essential for

creators to produce the value extracted by platforms (Srnicek, 2017) and thus have a structural function. For creators, this dimension is what makes it possible to expand reach, metrics, and visibility, converting them into monetization opportunities.

Audience metrics function as both relevance indicators and revenue predictors. View counts directly impact revenue, while follower counts suggest long-term audience retention (Christin; Lewis, 2021). Content production and follower interactions are planned to generate measurable results (Duffy, 2020b), which shapes creative work around what platforms can measure rather than artistic or reputational concerns (Siciliano, 2021).

Relational work emerges as equally central to content creation in platform instructions. Practices such as thanking followers, responding to concerns, and offering giveaways are used to maintain metric performance (Lou; Zhou, 2024); but many experience this as exhausting and stressful labor (Villegas-Simón *et al.*, 2023), often exacerbated by exposure to harassment and hate speech (Meisner, 2023).

Because automated curation and moderation systems are seen as opaque and unpredictable (Verwiebe; Buder; Osorio-Krauter, 2024), creators develop mental models through trial and error to figure out ‘algorithms’ preferences’ (Bishop, 2019; Bucher, 2018; Cotter, 2019). Permanent experimentation reflects being “out of face,” in Goffman's (1982, p. 8) terms, which means participating in interactions “without having ready a line of the kind participants in such situations are expected to take.” Creators must continually improvise their professional performances without stable scripts, running the risk of using the “wrong face” whenever changes in affordances and automated systems suddenly render practices illegitimate.

This includes adjusting posting frequency, building a self-brand (Arriagada; Ibáñez, 2020), and incorporating attention-capture elements, such as impactful titles, attractive visual aesthetics (Roivainen, 2024), storytelling narrative (Pertiwi; Sanusi, 2023), hashtags, and keywords (Huang; Ye, 2024; Ma; Kou, 2021). However, given constant changes in platforms’ features, the knowledge is perishable, requiring continuous relearning (Arriagada; Ibáñez, 2020).

Platform prescriptions suggest that posting frequently and regularly satisfies audience expectations and keeps work relevant. In practice, creators perceive that recommendation systems favor publication consistency, so they prioritize productivity over creative autonomy and well-being (Arriagada; Ibáñez, 2020). Rationalizing performance replaces discretionary occupational judgment with a standardized, metrics-based logic. This subordination of creative discretion to metric optimization creates structural tension with platform

prescriptions for authenticity (Dimension 3) and experimentation within compliance boundaries (Dimension 1). Platform professionalism, therefore, aligns human labor with business imperatives.

6.1.3 Dimension 3: Sustaining emotional resilience

This dimension (D-3) includes the necessary emotional and subjective competencies mobilized to sustain a long-term career. By prescribing these competencies, platforms recognize the risks inherent to the nature and conditions of the work but treat them as career adversities to be overcome. Thus, they transfer to creators the responsibility of dealing with the harmful consequences of the disciplinary regime, transforming structural burnout into personal failure due to lack of resilience¹⁰. Table 9 presents the second-order codes and the expected results.

Table 9 — Analytical structure and outcomes of Sustaining emotional resilience

Sustaining emotional resilience • Cultivating creativity, authenticity and connection • Maintaining well-being and work-life balance • Managing uncertainty and setbacks	Results for Creators Overcome work-related risks to well-being, health, and creativity to sustain a career. Benefits to Platforms Promote resilience to the negative effects of working conditions, preserving career myths.
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Source: Adapted from Pithan, Araujo, Dalmoro (2025)

The creator acts as a personal entrepreneur, responsible for cultivating resilience and health while attempting to reconcile contradictory demands between normative compliance (D-1) and maximization of metric visibility (D-2). Faced with the insolvency of this contradiction, the prescriptions serve as rhetorical safeguards, allowing platforms to recognize structural challenges without assuming direct responsibility.

Expressing concern for well-being acknowledges that creative work is connected with suffering and exhaustion. Instagram (2024) exemplifies this by stating that “it’s hard to stay inspired when you’re not in a good place,” recommending that creators recognize and prevent burnout, besides using the “block button” for peace of mind. TikTok (2024) advises balancing life and work to ensure “not allowing your work to overpower your personal needs,” while

¹⁰ It bears noting that this analysis addresses what platforms prescribe to creators as a category; it does not assert that medical mega-influencers, as a structurally privileged group with a pre-existing primary income source, uniformly experience the burnout and precarity these prescriptions presuppose.

YouTube (2024) minimizes the impact of low-performing videos, recommending “don’t worry too much, it’s not going to totally impact the performance of your future uploads or kill your channel.”

These efforts require self-reflexive competencies to sustain creativity, experiment with aesthetics, build a personal brand perceived as authentic, and cultivate emotional bonds with followers. TikTok (2024) emphasizes that users seek to “learn, get inspired, laugh and cry with you through engaging, raw, authentic content” rather than polished versions, and in a video posted by @creators profile (Instagram, 2024), an influencer claims that focusing on “vanity metrics” can hinder the creative process and make creators “lose sight of what you really love to do.” A YouTube (2024) quote addresses the challenge of balancing these contradictory demands, suggesting that creators should “slowly pivot, as opposed to doing a drastic change.” By acknowledging suffering, platforms appear to break with managerial cultures that deny psychological harm and disregard workers’ subjectivity (Dejours, 2012). However, they are operating a symbolic management of harm, framing it as an inherent risk of the occupation. Thus, management absolves organizations of responsibility for work-related harm (De Gaulejac, 2007), instrumentalizing the subjectivity of workers to lead them to voluntarily adhere to business objectives.

This dimension serves a structural function for platforms by ensuring the continuous flow of data production; therefore, the prescription of well-being does not aim to protect creators but to preserve their productive capacity. Emotional management is thus reconfigured as self-monitoring of emotional integrity. Creators must manage their own exhaustion to sustain productivity, internalizing personal deficits (lack of resilience, failure to balance work and life) which are, in fact, structural consequences of contradictory demands. Platforms do not care for creators; they instruct creators to fix themselves. Additionally, this workforce spreads career myths such as freedom, authenticity, and self-fulfillment, which are critical for attracting new entrants (Duffy; Wissinger, 2017).

In practice, platform demands systematically undermine well-being. Unpredictable curation and moderation systems trigger insecurity (Duffy, 2020a), overwork (Arriagada; Ibáñez, 2020), anxiety, and burnout (Niederauer; Maggi, 2022), with many creators reporting guilt over engagement fluctuations and loss of spontaneity (Huang; Ye, 2024). Creators are expected to normalize this “psychological toll” of the occupation (Lou; Zhou, 2024, p. 6) as a necessary competence for sustaining a career under organizational platform professionalism.

6.1.4 Conceptual synthesis: the Performative Credential

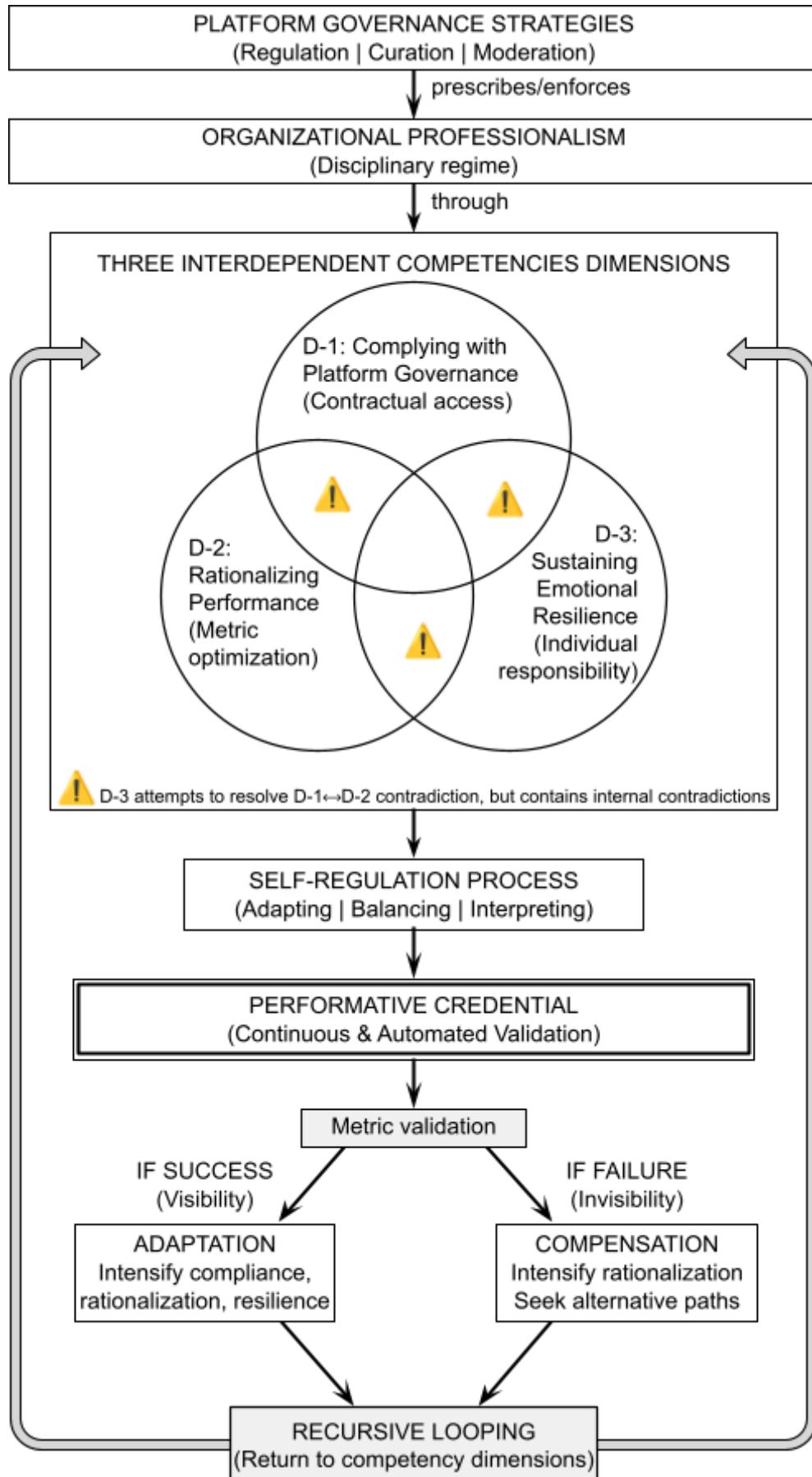
POP operates through a disciplinary regime composed of three interdependent dimensions of competencies. Complying with platform governance (D-1) sets the conditions for joining, Rationalizing performance (D-2) outlines how creators must perform to prove their value, and Sustaining emotional resilience (D-3) means they have to handle and cope with structural contradictions. These form a recursive system in which failure in one dimension triggers compensations in the others, while the reach of desired metrics demands constant adaptation to changes. For example, for those who violate the rules (D-1), invisibility is the punishment, but strictly adhering to the norms means either restricting visibility or assuming the risks of transgression. The creator then needs to intensify the work and the permitted practices of content optimization (D-2). This effort, in turn, involves overwork, causes exhaustion and insecurity, and inhibits authenticity and creativity. Therefore, it requires emotional self-regulation (D-3). Since there is no stable endpoint and only permanent adjustments, the cycle restarts, as illustrated in the theoretical model (Figure 10).

When creators do not meet conflicting demands, the structural contradiction is depoliticized. Failure is viewed as a deficit of competence (Thorne, 2022). Thus, platforms operate what we named symbolic management of suffering¹¹ (drawing on Pagès *et al.*, 2006; De Gaulejac, 2007) by recognizing structural risks but framing them as adversities to be managed individually. This mechanism absolves organizations of responsibility for work-related harm, instrumentalizing the subjectivity of workers to lead them to voluntarily adhere to business goals. Such individualization is the central disciplinary mechanism of the regime (Fournier, 1999), transforming structural impossibility into personal failure.

This recursive regime constitutes what we call Performative Credential, defined as the central mechanism of professionalism in the organizational platform (Pithan; Araujo, Dalmomro, 2025). Unlike professional credentials, which are based on training, are institutionally validated, and are relatively permanent (Larson, 2017), Performative Credential is based on demonstrations of contradictory competencies, is validated by performance metrics, and has a short lifespan, requiring continuous renewal. The management that justifies the behavioral demands of organizational professionalism (Evetts, 2013) by invoking the interests of abstract entities like the “sovereign client” (Fournier, 1999, p. 288) is rendered invisible on platforms and takes the material form of regulations, algorithmic codes, curation and moderation systems.

¹¹ This mechanism is identified here as a structural feature of the platform’s disciplinary regime, not as a claim about the subjective experience of MMIs in particular.

Figure 10 — Platform Organizational Professionalism: continuous adaptive process



While the Performative Credential emerges as the core category of this analytical phase, it transitions into the final theoretical model as the central organizing logic of Platform Organizational Professionalism, one of the two structurally incompatible regimes whose coexistence conditions the actions of MMIs.

But the contradictions force workers to devise inventive and subversive tactics to bridge the gap between prescriptions and actual possibilities, mobilizing intelligence and affectivity to complete the work (Dejours, 1993). The content creator then embodies De Gaulejac's (2007, p. 243-244) "flexible man," whose precarious, adaptable identity is shaped by market demands, contrasting with the "professional man," whose identity was linked to a job and peer recognition. The inability to simultaneously meet incompatible demands forces the creator into permanent adaptation, which guarantees the platform a permanently responsive workforce.

Professional autonomy, defined by Freidson (2007) as the capacity to exercise discretionary judgment protected from managerial or market control, is eroded when the logic of optimizing metrics becomes more important than the relevance or social value of the content. Thus, discretionary judgment is replaced by Rationalizing performance. This regime is challenging for medical influencers, who must reconfigure professional authority into this imperative. In this scenario, occupational authority becomes mobilized as a visibility resource rather than a basis for discretionary judgment, enabling platform careers. The normative-regulatory framework that Medical Occupational Professionalism (MOP) opposes to this commercial logic is the focus of the following section.

6.2 Medical Occupational Professionalism (MOP)

After mapping the organizational professionalism defined by platforms (POP) through the recommendation of competencies for content creators, this chapter addresses the regime defined by CFM for social media use. This section was constructed by identifying the disciplinary regime of Medical Occupational Professionalism (MOP) in the same way as done with POP in the previous chapter, followed by a comparison of both models.

First, the systematic analysis of three normative documents from the CFM (code of ethics, regulation, and advertising manual) allowed identification of how the council proposes to regulate the identity, content, and performance of physicians on social media. In the context of this research, these documents represent occupational professionalism, a disciplinary regime in Fournier's (1999) sense, operating through discourse constructed internally by the professional group to guide and self-regulate conduct. The difference is that occupational professionalism is a discourse constructed internally by the group, based on shared beliefs and values, to be incorporated by each professional with the objective of guiding and self-regulating their conduct (Evetts, 2013).

From the analysis emerged three interdependent aggregated dimensions, in which Insulating professional authority (D-I) establishes who can legitimately invoke medical authority and under which economic conditions, Standardizing epistemic performance (D-II) controls what can be said and how communication must be structured, and Enacting professional sobriety (D-III) regulates how visibility and professional self-presentation should be performed. These dimensions form a coherent, mutually reinforcing system of rules to defend the fiduciary nature of the activity. D-I establishes social closure via credentials; D-II designs epistemic control via scientific validation; and D-III defines institutional dignity via sobriety. Violating any dimension undermines the others. Credentials without scientific rigor erode trust, sobriety without credentials lacks legitimacy, and scientific authority without institutional dignity degrades collective professional status.

The mutually reinforcing regulatory system comprising these three dimensions forms the core category Fiduciary Authority Protection, through which the CFM simultaneously seeks to protect professional status against the commercial logic of platforms and reaffirm the character of medicine as a 'means-activity' (rather than end-activity). The process is operationalized by the imposition of conformity rituals, under the premise that following the correct procedures guarantees the ethics of the content and the integrity of the image associated with the ideal of service. However, the causal powers of the regulatory mechanism remain largely unexercised. CFM's judgment and inspection structure (composed of a fixed

number of 28 titular councilors) has remained static (Salmeron, 2022) while the medical profession experienced exponential growth, jumping from 113.495 professionals in 1980 to nearly 600.000 in 2024 (Scheffer, 2025). This structural disproportion creates an impunity loophole, allowing violations to persist undetected. Similarly to the findings in the previous phase, Fiduciary Authority Protection serves as the core category for this stage of analysis, but it transitions into the final substantive theory as the central organizing logic of MOP, one of the two structurally incompatible regimes whose coexistence conditions the actions of MMIs.

Unlike POP, which treats emotional labor as a productive competency (D-3), MOP does not regulate physicians' subjective experience. D-III governs public self-presentation, treating physicians as institutional representatives whose observable conduct, rather than inner states, is subject to professional discipline.

This analysis did not find a fully parallel between MOP and the emotional dimension of POP (D-3). Rather than an analytical gap, this asymmetry reflects a fundamental difference in regulatory logic. Platforms prescribe emotional and resilience competencies, such as recognizing burnout, managing well-being, and cultivating creativity under pressure. In contrast, MOP does not address subjectivity or emotional harm, whether caused by clinical work or physicians use of social media. This divergence reveals distinct governance principles, as platforms attempt to prepare creators to remain functional in their service, demonstrating an instrumental concern with worker productivity, while CFM regulates peers as autonomous professionals whose well-being is a private matter. It demonstrates the difference between how organizational management instrumentalizes subjectivity for productivity and how occupational governance regulates observable conduct rather than subjective experience.

This section reconstructs the disciplinary regime of Medical Occupational Professionalism (MOP) through the systematic analysis of CFM normative documents; the cross-regime comparison with POP is conducted in the following section.

6.2.1 Dimension I: Insulating professional authority

The first dimension (D-I) articulates the Weberian concepts of social closure and Larson's (2017) notion of professions as authorized discourse communities. The CFM regulates the exclusive right to speak with specialist authority about medicine in digital environments. Insulating professional authority operates through three mechanisms, encompassing the authentication of professional identity (who can invoke authority),

definition and policing of communicative boundaries (where, when, and how authority can be exercised), and protective market firewall (separation between medical practice and commercial activity). By requiring display of credentials, such as CRM and RQE numbers, the CFM activates the social closure that distinguishes authorized members from laypeople; by regulating content and self-presentation, it protects the collective right of the profession to construct meanings about health for the public while preventing medicine from being practiced as commerce.

Table 10 details the three mechanisms through which this dimension operates, derived from the focused codes of the analysis.

Table 10 — Insulating professional authority: mechanisms and functions

Insulating professional authority Establishes who can legitimately invoke medical authority and under which conditions.		
Authenticating professional identity → Who can invoke authority	Defining and policing communication boundaries → Where, when, how authority can be exercised	Participating in markets without commodifying authority → Protective market firewall separating practice / commercial activity

When creating professional profiles on social media, physicians must display the registration numbers in the state council where they work (CRM) and, if they possess it, the specialist registration (RQE). The CFM exempts from following this and other rules those with strictly personal profiles on social media, but making a post of medical character activates professional status and makes the regulation compulsory. As platforms dissolve the work-life boundary, the CFM emerges as an inescapable regulator; to be exempt from the rules, individuals are prohibited from posting content about their occupation.

The regulation recommends that professionals conduct themselves as “representative of medicine” (CFM, 2023, chapter V, art. 10°) and prohibits them from participating in product advertising inducing a guarantee of results, offering consultancy as a substitution for consultation, and publishing content that could be confused with “diagnostic judgment, procedures, and prognosis” (CFM, 2023, chapter IV, art. 9°). By distinguishing medical advice (prohibited) from information (permitted) and consultation (regulated) from education (allowed), CFM graduates regulatory space, while platforms merge these territories. MOP explicitly prohibits the endorsement of medications, but any mentioned product can be interpreted as if the physician were saying that it works, even without an explicit guarantee.

Therefore, the platform weakens the barrier between authority and commerce. The attempt to demarcate boundaries demonstrates that the CFM perceives the crisis of limits in the digital context, where platforms merge traditionally segregated spaces, such as office and classroom, and mix audiences that medical practice historically separated, such as patients, peers, and the lay public.

CFM recognizes that anti-commodification is impossible without limited commodification and elaborate distinctions to make it workable. The council allows presence on platforms to serve as advertising to attract patients but ignores remuneration via content creation, silencing or underestimating the visibility logic underlying the monetization of digital work. MOP differentiates licit economic interest and commodification of medicine by describing practices. Disclosing the clinic address and consultation price and informing about the equipment used and techniques offered are permitted actions. While participating in sponsored awards, consortia, tied sales, omitting conflict of interest, and endorsement of medications are prohibited. The specificity of the prohibitions, as shown in Table 11, suggests that the CFM perceives commercial influence as an existential risk to professional legitimacy and medical identity. The prohibition on participating in advertisements for commercial companies (CFM, Art. 115) and in advertising for medications and products with guaranteed results (CFM Resolution No. 2336/2023, item IV) does not mention ‘nutritional supplements.’

According to Anvisa (Brasil, 2018), nutritional supplements are classified as food. The regulation governs participation in advertisements, but does not address the use of a professional title to mention product benefits in one's own media outlets. Thus, the CFM's silence keeps the issue open, since a supplement brand sponsoring a medical influencer does not constitute formal advertising, and the endorsement may not be explicit. This gap is critical to the regulatory instrument and the empirical analysis of Phase 3.

Identifying this gap requires cross-referencing two distinct regulatory bodies. The CFM prohibits physicians from participating in advertising for medications and food products when such advertising implies guaranteed results, but does not specify whether nutritional supplements fall within the food category. Anvisa's classification framework resolves that ambiguity, yet neither body explicitly requires the cross-institutional reading on which enforcement depends. A physician seeking to determine whether supplement endorsement is permitted under professional ethics cannot answer that question from within the CFM's own regulatory documents alone. The gap is therefore a structural feature of a bipartite regulatory architecture rather than a simple legislative omission.

Table 11 — Anti-commodification firewall: prohibited vs. permitted economic activities

Prohibited (Commodification)	Permitted (Market Participation)
Guaranteeing results	Advertising competencies
Endorsing products for profit	Disclosing equipment used
Converting reputation to commercial asset	Building professional reputation
Offering discounts as a promotional gimmick	Announcing fees transparently
Tied-selling schemes	Accepting multiple payment forms

With this, the CFM creates an anti-commodification firewall that allows competition for patients but prohibits medicine from being practiced as commerce “in no circumstance or form” (CFM, 2019, p. 16). Like computer firewalls, it operates through packet inspection (evaluating each activity) and port control (some transactions permitted, others blocked). The underlying principle is medicine as a means to patient health, with financial exchange as incidental. Physicians are fiduciaries responsible for employing available means, not for results, protecting the trust relationship from transforming into a commercial transaction.

This dimension is fundamentally normative, establishing what is permitted/prohibited. Differently from the normative dimension of POP, in which norms establish compliance for visualization and monetization, here norms are supported by the ideological discourse of values historically shared by the group. Also in contrast with platforms, which individualize competencies and responsibilities, MOP addresses collective interest. While MOP sees physicians as fiduciaries, who serve patient interests, POP sees them as entrepreneurs, who maximize personal economic value. Medical influencers must then present themselves as disinterested health educators and monetizable entrepreneurs.

6.2.2 Dimension II: Standardizing epistemic performance

While D-I establishes who can communicate, this dimension (D-II) regulates the substance and form of medical knowledge in public communication. The concept articulates Larson’s (2017) notion about the standardization of knowledge as the basis for creating a uniform currency of professional competition, with Freidson’s (2007) emphasis on formal, abstract, and theory-based knowledge as the foundation of professional authority. By requiring a scientific basis and vetoing promises of a cure, the CFM attempts to impose

scientific (epistemic) rationality over the commercial and entertainment logic of platforms, preventing medicine from being perceived as charlatanism or a mere consumer product. Standardizing epistemic performance operates through three mechanisms, namely epistemic control via scientific validation, defining what can be said; procedural control of educational structure and privacy protection, describing how it should be structured; and prohibition of guarantees of results. Table 12 details the three mechanisms through which this dimension operates, derived from the focused codes of the analysis.

Table 12 — Standardizing epistemic performance: mechanisms and functions

Standardizing epistemic performance Regulates what counts as valid medical knowledge and how content must be structured to be legitimately educational.		
Standardizing evidence-based communication → Epistemic control through scientific validation (only consensus from literature)	Educational content engineering → Procedural control of educational structure and privacy protection	Preventing clinical promises → Control of promises through prohibition of guarantees (medicine as means to an end-activity)

MOP determines that physicians limit “comments to the clinical context, always supported by current medical literature” and prohibits references to treatments without scientific support, as well as non-validated experimental methods (CFM, 2024, p. 19). Thus, the CFM attributes to professionals the role of guardians of evidence-based medicine, acting as “special communities of discourse, endowed with the authority of speaking about and for their field and, in so doing, constructing its meaning for the lay public,” as Larson (2018, p. 33) describes professions. The requirement of scientificity and prohibition of cure promises repeats the rhetorical strategy used by the medical elite of the 19th and early 20th centuries to disqualify healers and homeopaths as charlatans (Coelho, 1999; Pereira Neto, 2000). Whoever claims to offer exclusive treatments and guaranteed results becomes the modern equivalent of the practitioner who sold secret elixirs, and the CFM reacts by wielding science as a tool for exclusion of competitors to maintain the monopoly of authority.

This dimension functions as content engineering with specific instructions for the formal structure of posts to have a pedagogical character and protect patient privacy. For example, when using before/after type images, the content must include at minimum four patients of different body types and be accompanied by text about “therapeutic indications, possible satisfactory or unsatisfactory evolutions, factors that may negatively influence the

result, and possible complications” (CFM, 2024, p. 47). The patients portrayed must be anonymized and have granted explicit consent.

The council prohibits “guaranteeing, promising, or insinuating good results” of treatments and products and calls upon professionals to “ardently defend” the ‘means-activity principle’ “to protect medicine from judgments in Justice that demand high compensation should the promised results not be achieved” (CFM, 2024, p. 31).

This dimension is parallel to Performance Rationalization (D-2 of POP). But, instead of describing practices to increase visibility and engagement, D-II controls the form of content to preserve the image of the profession and protect the public from misleading information. The two regimes encourage profoundly distinct narratives. While POP recommends narrative impact (hooks, storytelling) and optimizing content to adapt it to metric indicators, MOP prescribes practices that may reduce content performance, such as mentioning complications and showing multiple patients in before/after photos, and directs adherence to a rigid structure and respect for scientific evidence.

The regulation governs the form of scientific citation, requiring anchoring in recognized literature, prohibiting references to unvalidated methods, prohibiting guarantees of results. But it lacks a mechanism to prevent biased evidence selection or the interpretation of studies beyond their original scope and context. The standard assumes that using scientific literature ensures alignment with medical consensus. This assumption is structurally vulnerable because formal compliance with the requirement (the presence of bibliographic references) can coexist with a violation of its substantive function, which is to anchor to the consolidated state of knowledge. This structural gap in the regulatory instrument will have an empirical impact, as documented in the Phase 3 analysis.

After establishing what physicians can say (D-I) and how the content should be structured (D-II), the third dimension addresses how they should present themselves publicly.

6.2.3 Dimension III: Enacting professional sobriety

D-III is fundamentally behavioral / performative because it governs public self-presentation. Rather than instrumentalizing subjectivity like platforms do to maintain creator productivity, the CFM regulates physicians’ observable conduct as institutional representatives. This contrasts with D-3 of the POP, which prescribes emotional management as a competency, as the MOP does not regulate the subjective experience of physicians.

D-III prescribes sobriety in the public self-presentation of physicians. Three theoretical foundations explain the recommendation. The first is the Weberian notion of status

groups that depend on social honor and a distinct lifestyle to differentiate themselves from ordinary merchants (Weber, 2004). The second is Larson’s (2017) argument that inappropriate individual behavior threatens the collective image of the profession. The third is Freidson’s (2007) emphasis that public trust depends on the perception that the professional acts out of an ideal of service, not out of economic interest or personal vanity. The CFM has a goal to keep medicine from being confused with entertainment or vulgar commerce and losing its moral authority. Enacting ‘Professional sobriety’ operates through two mechanisms. ‘Monitored sobriety’ establishes conditional permissions and ‘Categorical prohibitions’ in direct self-presentation, and extended responsibility holds physicians accountable for third-party content with praise, considered indirect self-presentation. Table 13 details the two mechanisms through which this dimension operates, derived from the focused codes of the analysis.

MOP allows “issuing generic comments about pleasure with work, joy in receiving patients and companions, motivations with the daily challenges of their profession, and generating positive current for the good image of medicine” as long as the physician keeps patients anonymous (CFM, 2023, chapter IV, art. 9º). The council encourages altruistic representation of the profession, emphasis on service to the public, and loyalty to the collective ethos of medicine. It is prohibited to practice unfair competition, disseminate untruthful content, or engage in sensationalist communication and self-promotion, such as participation in sponsored awards that convert authority into promotional assets.

Table 13 — Enacting professional sobriety: mechanisms and functions

Enacting professional sobriety	
Regulates how physicians must present themselves publicly, balancing visibility for client attraction with imperatives of sobriety, dignity, and institutional representation.	
Regulating visibility and self-presentation → Monitored sobriety: defines conditional permissions and categorical prohibitions in direct self-presentation	Managing reposts and third-party mentions → Extended responsibility: physicians accountable for third-party content praising them, treated as indirect self-presentation

The council understands as sensationalism the procedure to exalt one’s own performance; therapy not recognized by the CFM; adulterated statistical and scientific data; the execution of clinical or surgical procedures; information that may cause unrest, insecurity, panic, or fear in the public; and visual representations and information that induce the perception of a guarantee of results (CFM, 2024). The definitions of unfair competition and

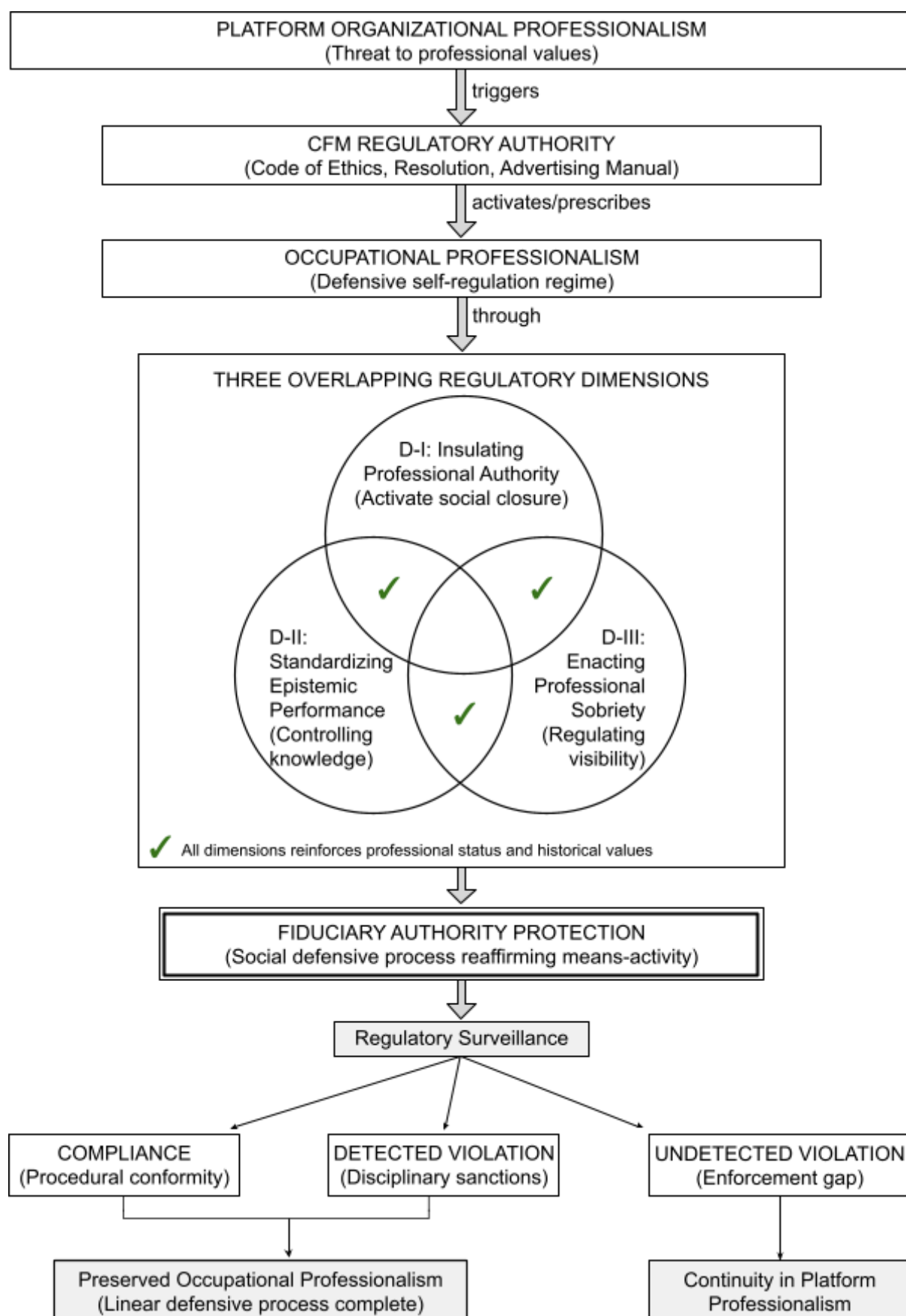
untruthful content have points of overlap. Condemned practices include offensive criticisms of peers, announcements of free services, and insinuating exclusive or revolutionary practices (CFM, 2023).

Physicians have extended responsibility for content they did not create, but that increases their visibility. Third-party posts shared or reposted by the physician come to be considered as their publications for purposes of applying the rules. Sharing praise is permitted, as long as it is not sensationalist and the frequency does not exceed “two per semester” (CFM, 2024, p. 77). Additionally, the Commission for Disclosure of Medical Matters (Codame) can investigate repeated or systematic posts by third parties with praise for technique or procedure results even if they have not been shared by the physician.

This regulation shows a sophisticated understanding by the CFM of platform mechanics by recognizing that testimonials, selfies, and praise function as advertising in fact and that, in the attention economy, reputation is co-constructed by multiple agents. Thus, the council associates reputational gain with regulatory responsibility, in which physicians cannot exempt themselves from responsibility for content that benefits them commercially, even if produced by others. This rule may inhibit, for example, the hiring of click farms that sell artificial engagement services to inflate metrics (Grohmann *et al.*, 2022), but it imposes an additional challenge to physicians because it is impossible to control what patients spontaneously post or how the recommendation system amplifies the content.

The regulatory model of MOP faces a contradiction between normative prescription and enforcement capacity. Although the CFM establishes detailed rules for digital medical practice, the inspection structure remains outdated. The number of councilors has remained the same since the council’s foundation (Salmeron, 2022), while the category has experienced a recent and unprecedented explosion, nearly doubling in less than fifteen years, jumping from 304.406 physicians in 2010 to 597.428 in 2024 (Scheffer, 2025). This disproportion generates delays in proceedings and limits the capacity to detect infractions. In practice, an impunity loophole is created whereby medical influencers who violate CFM norms can continue operating according to platform professionalism demands without facing disciplinary sanctions, not because the CFM approves such practices, but because the regulatory system cannot process all violations.

Figure 11 — Medical Occupational Professionalism: defensive response



Regarding image, this dimension is inverse to D-3 of POP. Platforms prescribe that content creators be themselves, treating authenticity as market differentiation, while CFM calls upon professionals to be representatives of the category, establishing institutional dignity as collective protection.

Figure 11 summarizes the disciplinary regime of Medical Occupational Professionalism in response to the threats posed by the demands and incentives of platforms. This regulatory structure, anchored in the protection of authority and the defense of medicine as a ‘fiduciary obligation’, will be contrasted in the next section with the regime of Platform Organizational Professionalism to reveal the ontological incompatibilities between the two models.

6.3 Cross-regime comparison

Having established the disciplinary regimes of POP and MOP in the preceding sections, this section presents the comparative analysis that closes the second research phase. The comparison required no additional data construction; it proceeded from the categories already produced in the two preceding sections, treating them as comparative material. The systematic confrontation of POP and MOP at the level of aggregate dimensions revealed that the two regimes are not merely different in orientation but structurally incompatible. Their prescriptions are causally antagonistic, such that full adherence to one requires the violation of the other.

Section 6.3.1 establishes the ontological basis of this incompatibility, Section 6.3.2 examines how it manifests across the three primary axes of dimensional tension, and Section 6.3.3 advances to two analytical implications that the dimensional comparison raises but does not resolve.

6.3.1 Ontological incompatibility between POP and MOP

The two regimes are ontologically incompatible because they presuppose distinct natures of professional existence, pitting the performative/commercial (POP) on one side and ethical/institutional (MOP) on the other. There is no course of action that allows complete adherence to both regimes; their prescriptions are causally incompatible, making dual

professionalism a structural impossibility activated in the domain of the Actual¹² when physicians operate simultaneously under both.

Professional existence as digital influencer on the platform is ephemeral and contingent on validation by automated systems that measure audience responses. Legitimacy is not built by something influencers possess (such as abstract knowledge or a diploma), but something they must continuously reiterate by achieving performance goals. On the other hand, medical professional existence is stable and institutional, based on formal credentials that guarantee exclusive access to a labor market and status. The fundamental tension associated with professional nature and its validation of authority occurs between a performed and dynamic status (becoming) versus an acquired and stable status (being).

At the teleological level, there is a clash between the purpose of professional action. Organizations program their platforms to extract value from all interactions. The platform logic, especially the Performance Rationalization dimension (D-2), drives creative work toward ends (metric results), subordinating discretionary judgment to the logic of metric optimization. In turn, MOP's core principle (medicine as fiduciary obligation rather than commercial transaction) directly contradicts this. The CFM prohibits unfair competition, the offer of exclusive therapies, and sensationalism, because this corrodes the trust relationship and transforms professional performance into commercial transactions. The principle of medicine as 'means-activity' becomes a normative fiction in the space that converts everything that circulates into economic value.

In the relationship with the market, the platform structures the professional as a self-entrepreneur, responsible for their visibility and career management, converting attention into monetization. In turn, Fiduciary Authority Protection operates through a market firewall that allows participation to attract clients but prohibits the commoditization of medicine. For POP, medical authority is an asset to be leveraged and monetized. For MOP, authority is a moral obligation that fades when used for commercial purposes.

Table 14 illustrates the basic distinctions between the two regimes in terms of ontological, teleological, and market characteristics, which result in mutually divergent professional identities.

¹² Following Bhaskar (2008), the domain of the Real comprises structures and mechanisms that exist independently of whether they are activated or observed; the domain of the Actual comprises the events that are realized when mechanisms are activated; the domain of the Empirical comprises what is observed. The application of this framework to the morphogenetic cycle was developed in the methods chapter.

Table 14 — Structural incompatibilities and mutually exclusive professional identities

	Platform Organizational Professionalism	Medical Occupational Professionalism
ONTOLOGICAL DIMENSION	Professional existence: Performative and ephemeral Status validated by automated systems measuring audience responses; must be continuously reiterated through performance	Professional existence: Stable and institutional Status validated by formal credentials (CRM, RQE) guaranteeing exclusive market access goals
	Professional identity: Physician as entrepreneur/individual brand Maximizes personal economic value	Professional identity: Physician as fiduciary/autonomous institutional representative Serves patient interests above own
	Authority validation: Dynamic status (becoming) through metric performance Distinction through metrics	Authority validation: Acquired status (being) through permanent credentials Distinction through peer recognition
TELEOLOGICAL DIMENSION	Purpose of professional action: Driven toward ends (metric results) Subordinates discretionary judgment to metric optimization logic	Purpose of professional action: Medicine as 'means-to-an-end activity' (fiduciary obligation) Prohibits sensationalism, exclusive therapies, unfair competition
	Professional tasks redefined Diagnosis: Observe audience data/metrics and identify performance problems Inference: Connect metrics to platform knowledge Treatment: Prescribe actions to optimize visibility and maximize engagement	Professional tasks protected: Diagnosis: Observe patient symptoms and identify disease/problem/need Inference: Connect diagnosis to medical knowledge Treatment: Prescribe interventions to resolve symptoms/disease
MARKET DIMENSION	Relationship with market: Expansion of authority through commodification Medical expertise as valuable asset generating revenue from fans/brands	Relationship with market: Containment through market firewall Allows participation to attract clients but prohibits commoditization of medicine
	Professional existence: Constitutive (creates value, generates income, produces professional status) Visibility as goal	Professional existence: Instrumental (attracts patients, defends boundaries) Sobriety as virtue
	Trust: Can be leveraged	Trust: Must be protected

The incompatibility between causally antagonistic mechanisms manifests in four transversal contradictions that permeate all dimensions of both regimes, including (a) Autonomy vs. External Governance, where MOP reaffirms professional autonomy based on discretionary judgment, while POP governs through automated systems that subordinate the creator's judgment to metric-driven optimization. (b) Normative Ethics vs. Communicational Performativity, in which MOP privileges conduct, decorum, and intention, while POP

transforms metric performance into the central object of work. (c) Ex Ante Regulation vs. Continuous Modulation, contrasting MOP controls using previously defined norms and prohibitions, while POP requires constant changes to achieve visible incentives with opaque standards. (d) Permanent credentialism vs. Performative Credential, where MOP requires authentication by formal credentials (CRM, RQE) that provide stable professional status, while POP validates authority through perishable metrics that establish a concurrent system of validation.

The analysis makes explicit the difference, in Abbott's (1988) terms, between professional jurisdiction (permanent claim over territory of expertise) and settlement (contingent position continuously negotiated). Medical influencers operate under both regimes simultaneously, creating bifurcated authority split between the permanent in the institutional terms of CFM and the perishable in the constantly adjusted terms of platforms.

This transformation represents, in Freidson's (2001) terms, the fundamental conflict between professional logic and market logic. While the quality of professional work is defined by peers through specialized knowledge, platforms operate in market logic, where the consumer (audience) and performance metrics define value and control work. Abbott (1988) describes the commodification of specialized knowledge as the process by which professional expertise is embodied in products or routines that can be bought and sold without the involvement of the jurisdictional profession.

In the phenomenon analyzed by this research, instead of an external technological threat displacing professionals, they themselves voluntarily translate their knowledge into scalable products, inserting it into the organizational logic of the platforms. The context redefines the three central tasks of professions (Abbott, 1988), in a scenario where diagnosis addresses audience data rather than patient need, treatment aims for metric performance rather than cure, and inference is reoriented to serve data extraction. The organizational logic of platforms corrodes and redefines the key dimensions of professionalism, expertise, autonomy, and authority (Noordegraaf, 2020). Expertise becomes an asset to attract and engage the public, autonomy is domesticated by the affordances and preferences of the recommendation and ranking system, and authority is converted into a commodity to attract advertisers, paying subscribers, and buyers of infoproducts¹³.

¹³ The term infoproduct circulates among content creators and digital marketing practitioners to designate knowledge-based digital products (such as online courses and e-books) commercialized directly to consumers. The academic literature offers no consolidated equivalent in English; the phenomenon appears described as "info-products or digital information products" presented "with a formative purpose" outside formal educational circuits (Escandell-Poveda; Papí-Gálvez, 2023, p. 2). The unhyphenated form is adopted here following the morphological pattern of established English compounds with the prefix info-.

6.3.2 Dimensional tensions between regimes

This section examines how the ontological incompatibility established above manifests across the three primary axes of dimensional tension produced by the cross-regime comparison. Each axis confronts one or more dimensions of POP, namely Complying with platform governance (D-1), Rationalizing performance (D-2), and Sustaining emotional resilience (D-3), with the dimensions of MOP, namely Insulating professional authority (D-I), Standardizing epistemic performance (D-II), and Enacting professional sobriety (D-III), identifying where the imperative of one regime directly violates the prescription of the other.

Despite the fundamental incompatibility, the comparison also identified non-confrontational interstitial spaces between dimensions, either due to convergence or because one regime addresses what the other ignores. For example, D-2 recommends “informative, in-depth content to show your expertise” (TikTok, 2024), whereas CFM advises physicians to provide educational content.

6.3.2.1 Commodification vs. anti-commodification firewall

The first tension refers to the confrontation between the monetization logic of the platforms (D-1) and the anti-commodification firewall of MOP (D-I), in which the platform policies recommend the conversion of authority into a commercial asset and revenue diversification, and the CFM (2019) prohibits medicine from being practiced as commerce. Table 15 depicts the fundamental tension between the logic of monetization and the ethics of anti-commodification.

Irresolvable contradiction: Platforms position medical content as commodified labor, monetizable through ads, subscriptions, and brand partnerships, which CFM defines as unethical commodification. Physicians who monetize their digital work through subscriptions to provide access to exclusive content and through partnerships with brands are acting as commercial agents, even when the content remains educational. The very act of professionalizing as a platform creator commodifies medical authority, yet medical influencers digital efforts are unsustainable without financial rewards. Medical authority is datafied; that is, legitimacy via performance metrics takes precedence over traditional medical legitimacy via credentials.

Table 15 — Tension between Dimension 1 (POP) and Dimension I (MOP)

Platform Organizational Professionalism	Medical Occupational Professionalism
Dimension 1: Complying with platform governance	Dimension I: Insulating professional Authority
Logic: Platforms demand diversified revenue streams, adapt to monetization criteria, develop deeper connections to grow recurring monthly income, and promote prices/discounts	Logic: CFM prohibits the practice of medicine as commerce. Medical work is a 'means-activity' and not an end in itself to maximize profit
Exemplary quote: "With Instagram subscriptions, you can develop deeper connections with your most engaged followers and grow your recurring monthly income by giving subscribers access to exclusive content" (Instagram, 2024)	Exemplary quote: Physicians "are prohibited from offering their services through consortia and similar schemes, raffles of any kind... and other methods that suggest the idea of reward or advantage" (CFM, 2024, p. 33)
Meaning of medicine: Platforms treat medical expertise as valuable knowledge for audiences and brands, capable of generating revenue from fans and advertising revenue	Meaning of medicine: CFM treats medical expertise as fiduciary knowledge serving public health, not personal enrichment
Status as a professional: Validated by metrics and monetization	Status as a professional: Validated by credentials and professional registration

6.3.2.2 Visibility-driven content production vs. evidence-based communication

The second tension arises from the conflict between rationalizing performance to achieve the visibility and engagement metrics necessary to advance their career (D-2 of POP) and standardizing the epistemic performance recommended by CFM. Where POP requires the creation and optimization of content based on observable data, adopting practices such as content adaptation and experimentation and workflow management such as routinization and scalable production, MOP, on the other hand, demands a formal educational structure and communication of medical uncertainty. Table 16 summarizes the contrast between visibility-driven creation of content and scientific evidence-based communication.

Irresolvable contradiction: Medicine involves uncertainty and requires communication contextualized by scientific evidence. However, the rapid consumption of content on platforms forces simplification of complexities. Platforms recommend linguistic and visual resources to capture the audience's attention (such as appealing to humor, emotional storytelling, using impactful images and titles, and inserting a hook in the first few seconds of the video) that both represent relatively easy shortcuts to reach the public and dangerously approach sensationalism or alarmism. Choosing to optimize content for visibility, in addition to deviating from the MOP, involves the risk of violating the professional entity's norm.

Table 16 — Tension between the Dimensions 2 and 3 (POP) and Dimension II (MOP)

Platform Organizational Professionalism	Medical Occupational Professionalism
Dimensions 2 & 3: Rationalizing performance & Sustaining emotional resilience	Dimension II: Standardizing epistemic performance
Logic: Maintaining metric performance requires content updates, adaptation, and continuous creative experimentation based on audience feedback metrics	Logic: Communication must be based on scientific evidence and established medical literature consensus and conform to a structured pedagogical format, reinforcing medicine's 'obligation of means' by prohibiting guaranteed success
Exemplary quote: "See what's working (and what's not), and understand who's watching your content so you can plan how you post in the future" (YouTube, 2024) "You'll be able to dive into your video, follower, and account performance to learn what's working and what's not, then adjust accordingly!" (TikTok, 2024)	Exemplary quote: Physicians are prohibited from publicizing treatment "whose value has not yet been expressly recognized scientifically by a competent body" (CFM, 2019, p. 43) "The use of patient images (...) will have to follow four steps outlined for advertising. Otherwise, it will be treated as sensationalism and self-promotion" (CFM, 2024, p. 46)
Nature of the content: Platforms treat content as an interactive social process, driven by creative experimentation, popularity, and participant reactions	Nature of the content: CFM considers communication on social media as medical advertising or publicity that must be scientifically based, ethical, and educational
Content validation: What audiences engage with is what the creator should produce more of	Content validation: What established scientific literature and collective professional consensus support is what physicians must communicate

6.3.2.3 Personal branding vs. professional sobriety

The third tension is between the platforms' recommendation to build a personal brand to gain a 'monetizable' reputation on platforms (D-2 and D-3 of the POP) and the CFM mandate of professional sobriety and collective identity (D-III of the MOP). The MOP requires physicians to maintain institutional dignity and self-presentation as representatives of the profession and prohibits self-aggrandizement and self-promotion by claiming exclusive qualities as physicians or employing exclusive treatments. In turn, the process of self-branding involves crafting and adjusting one's self-presentation based on market analysis and audience feedback to position an image as a professional differentiator and competitive advantage. Table 17 depicts the conflict between developing a personal brand and adhering to professional sobriety standards.

Irresolvable contradiction: Platforms encourage content creators to build a distinct personal brand, which is viewed as a resource for producing long-term revenue. However, CFM defines as self-promotion what platforms consider essential personal brand development. Within the 'context collapse,' the structural flattening of multiple audiences into

one (Marwick; boyd, 2010) prevents physicians from separating their peers from the audience or from the platform's metric systems. Platforms eliminate the distinction between "frontstage" and "backstage" (Goffman, 2002), forcing simultaneous performance for incompatible evaluative regimes.

Table 17 — Tension between Dimensions 2 and 3 (POP) and Dimension III (MOP)

Platform Organizational Professionalism	Medical Occupational Professionalism
Dimensions 2 & 3: Rationalizing performance & Sustaining emotional resilience	Dimension III: Enacting professional sobriety
Logic: Platforms encourage creators to engage in personal brand building to convert their subjectivity into a measurable asset, resulting in a consistent content flow and a distinct personal brand that maximizes audience retention and revenue generation	Logic: CFM prohibits self-promotion and individualized professional distinction, requiring physicians to present themselves as institutional representatives, whose authority derives from collective credentials and values. Sobriety also involves making physicians accountable for third-party content that amplifies their personal reputation as indirect self-promotion
Exemplary quotes: "For your personal brand, start by thinking of all the things you're passionate about. What is your area of expertise?" (TikTok, 2024) "Earn money by doing what you love. Get paid for the content you create on Instagram while staying true to yourself and your personal brand" (Instagram, 2024)	Exemplary quotes: Physicians are prohibited from "behaving in a sensationalist or self-promotional manner, engaging in unfair competition, or disseminating untruthful content" (CFM, 2024, p. 36) "The sobriety of comments must also be safeguarded, a point of balance so that publications do not slip into sensationalism, self-promotion, and unfair competition" (CFM, 2024, p. 28)
Professional identity: Physician as distinctive individual brand; entrepreneur monetizing own expertise	Professional identity: Physician as institutional representative; collective embodiment of medical profession

Under these structural conditions, to succeed as a digital influencer, some degree of commercialization of authority (not medicine itself) becomes inevitable, which directly conflicts with CFM's mandate for institutional representation and collective identity. When building this personal brand involves claiming exclusive or privileged capabilities, it becomes even more problematic.

6.3.3 Proceduralization and the education-commerce inseparability

The three axes of dimensional tension examined above establish where the regimes collide. This section examines two analytical implications that emerge from that collision and that the dimensional comparison alone does not exhaust, namely how procedural compliance with CFM norms legitimizes self-promotion, and why the separation between education and

commerce becomes an object of active and continuous work rather than a condition guaranteed by formal compliance.

The two models of professionalism compel medical influencers to pursue a normative and ethical balance that is, at best, unstable. This reveals two central theoretical questions. The first question concerns how formal compliance legitimizes self-promotion. The CFM's regulatory strategies, centered on procedural content engineering, operate on the assumption that form is capable of containing the commodification character of medicine. By shifting focus from intention to formal compliance, regulation transforms self-promotion into an ethically defensible practice, allowing medical authority to be integrated into circuits of visibility, reputation, and monetization without apparent loss of professional legitimacy. Proceduralization, thus, does not eliminate market logic; it makes it institutionally palatable. The second question concerns the improbability of separating education and commerce in content circulating on social media. All content circulation generates economic, reputational, or symbolic value. The more effective a physician's educational communication is (in terms of reach, engagement, and public recognition), the greater their integration into commodification circuits. Such an arrangement does not mean that separation is impossible, but that it becomes an object of active and continuous work, not a given guaranteed by procedural compliance.

How medical influencers maneuver across these contradictions, whether they adhere to the procedural fiction, subvert it, or invent hybrid tactics, and whether they perform the separation of education and commerce are questions that the subsequent empirical analysis will investigate.

6.4 The theory of the Performative Reconfiguration of Professional Authority

As previously established in the preceding chapters, the medical mega-influencers (MMI) found themselves caught between two incompatible causal mechanisms. On one hand, the imposition of Performative Credential (POP) by platforms conditions their existence on visibility. On the other hand, the imperative of the CFM's Fiduciary Authority Protection (MOP) prohibits them from commodifying the profession.

To address this structural fissure, Performatively Reconfiguring Professional Authority emerges as the core category of this thesis and underpins the substantive theory termed **Performative Reconfiguration of Professional Authority**. The category refers to the ongoing process of the agents; the theory names the phenomenon that this process produces and that this thesis explains. This performative dynamic is what characterizes the substantive theory presented here, in which medical authority ceases to be a fixed attribute and becomes a responsive performance. Content creators must adapt continuously because platform updates and evolving recommendation models (Duffy, 2020a; Bucher, 2018) make stable strategies impossible. Structural instability results from recalibrations of the programming and unilateral changes in the rules (Duffy *et al.*, 2021), making visibility a temporary reward that deteriorates as soon as the system is updated.

The incompatibility between the regimes is activated in the domain of the Actual through the concrete practices of agents, generated by the causal powers of the platform as an organizational form of production, management, and work operating upon a preexisting institutional structure. The physician's response process operates through two vectors that represent opposing dialectical variations to a single phenomenon, diverging in the priority assigned to each professional regime and, consequently, in the direction of the conversion of authority. Converting Professional Authority into Commercial Assets (Vector 1) and Professionalizing Health Communication (Vector 2) emerged as the aggregate dimensions of this phase of analysis. In the first, the commercial logic of POP subordinates MOP, converting authority and credibility into attention. In this context, the physician's competence as an influencer is validated by performance metrics (organizational demand). In the second aggregate dimension, MOP subordinates the logic of POP, converting attention into credibility to educate and inform, reinforcing collegial logic (occupational demand) and defending medicine and health institutions.

The analysis begins with the description of the transversal operational mechanisms that secure the agents' relevance on social media. The chapter then presents each vector separately to examine the structural and economic conditions that render reconfiguration

unavoidable for the agent. The text concludes with the synthesis of the theory and the examination of its structural implications.

6.4.1 Operational Mechanisms of Platform Adaptation (OMPA)

Unlike the Vector 1 (Converting Professional Authority into Commercial Assets) and Vector 2 (Professionalizing Health Communication), which follow a diachronic logic, the category termed Operational Mechanisms of Platform Adaptation (OMPA) comprises a set of synchronic instruments and strategies required for a physician to sustain a career as a digital influencer, regardless of their content orientation and monetization model. The universal necessity of these mechanisms confirms that platforms are organizational forms of production, management, and labor (Pithan; Closs, 2024).

Table 18 — Teleological divergence of Operational Mechanisms of Platform Adaptation

Mechanism	Description	Vector 1 function	Vector 2 function
Adapting content to visibility-driven criteria	Adjustment of content agenda and format to platform relevance criteria, SEO, and trending topics	Traffic capture for commercial conversion funnel	Reach optimization for public health information
Asserting professional legitimacy	Display of formal and visual markers of medical authority (CRM, RQE, white coat, verification badge)	Converting authority markers into product credibility	Authenticating the medical source and, consequently, the information provided
Building private distribution channels	Migration of the audience to proprietary channels such as WhatsApp and Telegram	Sales funnel outside public scrutiny and platform mediation	Direct distribution of educational content without platform mediation
Constructing a relatable persona	Use of the internet's emotional grammar, controlled exposure of private life, and informal language	Fabrication of intimacy to convert audiences into consumers	Building trust to communicate science without hierarchical distance
Deploying ethical shields	Use of disclaimers, legal notices, and liability waivers	Legal protection through framing content as informational	Reinforcement of the need for in-person consultation and individualized treatment
Expanding health scope	Communication across medical boundaries, irrespective of specialty registration	Broadening the consumer market through mass demand	Exercise of pedagogical stewardship in public health
Generating revenue through nonclinical assets	Revenue obtained through infoproducts, fan features, and courses	Primary revenue source: sale of self-management health products and supplements	Supplementary revenue: editorial books and academic courses
Incorporating nonclinical domains	Inclusion of topics not exclusive to medicine, such as weight loss, spirituality, and lifestyle	Conquest of niches with varied interests; sale of nonclinical products	Addressing the audience's broader questions and interests

To attain the level of celebrity characteristic of a mega-influencer, the physician must submit to the structural principle of popularity, making visibility the primary object of their work. These instruments share the same nature but assume opposing functions depending on the teleological orientation of the vector in which they are deployed. Table 18 details this divergence.

6.4.2 Vector 1: Converting Professional Authority into Commercial Assets

Vector 1 (V1) content dissociates the absence of disease from the concept of full health, presents official clinical parameters as insufficient, and mobilizes professional authority to validate the sale of partner-brand supplements or proprietary infoproducts. V1 represents the collapse of the ‘ethical firewall’ described in MOP, adopting market practices that the CFM has sought to combat since 1929 (the announcement of cures and self-promotion), and violates the principle historically defended by the institution of classifying medicine as a means-activity rather than an end-activity (CFM, 2019).

The process by which professional expertise and judgment exit the exclusive control of the profession and are transformed into saleable products or automated systems is what Abbott (1988) describes as the commodification of specialized knowledge. In this vector, the physician resolves the structural collision by surrendering to the ‘causal powers’ of the Performative Credential and converting authority and knowledge into scalable products, whether in the form of paid subscriptions granting access to exclusive content and interactions or as self-management health products. In this movement, the agent that arbitrates authority changes from the state and professional peers to the curation systems of the platforms and the public.

The need for visibility outweighs the institutional mandate for Fiduciary Authority Protection, leading the agent to embrace the platform's commercial logic at the expense of traditional professional boundaries. The V1 trajectory shows a progression across three phases. In Phase A (setup), the agent establishes authority and engineers demand, for instance by fabricating a clinical insufficiency or invalidating the traditional scientific consensus. In Phase B (the task), the agent builds the affective and epistemic infrastructure that bridges authority and sale, constructing parasocial intimacy, manufacturing substitute expertise, and colonizing non-clinical domains. In Phase C (outcome), the accumulated trust is converted into revenue, while compliance displays manage regulatory exposure.

Informative content builds the follower base that makes advertising content viable, while direct advertising generates immediate financial returns only for creators with

accumulated audience trust, according to Liu *et al.* (2024). Although their sample includes creators without medical registration and cannot be attributed exclusively to physicians, the economic logic the study documents corresponds structurally to the sequence observed in V1.

These steps and phases are synthesized in Table 19 and detailed subsequently. Steps and phases are not analytical categories in the grounded theory sense; they are a storytelling device adopted to render the sequential logic of the vector intelligible as a process, a choice compatible with Charmaz's (2014) emphasis on capturing social processes in motion rather than as static states.

Table 19 — Vector 1 dynamic mechanism structure

Phase A: Establishing authority and demand (setup)	
Steps	Focal codes
1. Fabricating a clinical insufficiency	Asserting professional legitimacy* Adapting content to visibility-driven criteria* Expanding health scope* Fabricating bodily insufficiency
2. Invalidating the traditional scientific consensus	Discrediting the traditional knowledge base
Phase B: Bridging the connection and resolution (the task)	
3. Engineering parasocial intimacy	Constructing a relatable persona*
4. Manufacturing substitute expertise	Inflating authority with alternative markers Assembling a counter-expertise
5. Colonizing non-clinical domains	Incorporating non-clinical domains*
Phase C: Converting digital labor into sales (outcome)	
6. Executing the commercial conversion	Building private distribution channels * Generating revenue through nonclinical assets* Selling self-management health products
7. Displaying compliance	Deploying ethical shields*

*Operational Mechanisms of Platform Adaptation

6.4.2.1 Phase A: establishing authority and demand (setup)

The initial phase of V1 comprises the steps Fabricating a clinical insufficiency and Invalidating traditional medical authority, which involve creating the conditions for the sale.

6.4.2.1.1 Step 1: Fabricating a clinical insufficiency

Three of the five practices in this phase are transversal. Asserting professional legitimacy through the display of formal and visual markers serves to confer credibility on proprietary and partner-brand products, while adapting content to visibility-driven criteria maximizes traffic capture for the commercial conversion funnel. In turn, expanding the thematic scope to whole-body health topics broadens the consumer market by replacing a

restricted niche with mass demands. For instance, ANG¹⁴ is an angiologist and vascular surgeon, and SURG2 is a general surgeon, yet both produce content and self-management health products on weight loss, while GP1 and GP4 are generalists who address hormonal modulation. In these cases, the digital market creates incentives for the physician to produce content beyond the scope of their registered specialty, given that the platform neither recognizes nor rewards the boundaries that the credential defines. The economic rationale of thematic colonization is not specific to the Brazilian corpus. Research by Du *et al.* (2025) on Chinese physicians operating on a health platform demonstrated that expanding the scope of topics beyond their medical specialty was associated with higher conversion rates to paid consultations. Breadth of content topics predicted commercial performance independently of clinical reputation.

Fabricating bodily insufficiency is specific to this vector, operating through the pathologization of everyday sensations. The physician reframes vague states of fatigue, tingling, sadness, or pain as symptoms of treatable imbalances. The audience is informed that walking on pins and needles is the “first sign of something much deeper and more serious” (GP1); that homocysteine levels reveal a “biological age” that may not correspond to the chronological one (ORTH); that “you can fall ill spiritually” and that “what does a person die of? Of a broken heart” (CARD/NUT) indicates that a person can die from emotional distress.

The spectrum ranges from the organic to the spiritual, but the rhetorical operation is the same, aimed at redefining the normal as pathological and the pathological as treatable by the speaking physician or by the method they advocate.

The sequence is documentable in multiple corpus units. CLS condenses common fatigue, tingling, and memory lapses into a question: “Have you noticed tingling in your hands or feet? Has your memory been failing more frequently? Do you feel tired even after a good night's sleep?”, before pointing out that “the silent signs of something much more serious: vitamin B12 deficiency” and directing the viewer to an affiliated supplement store link. GP1 applies the same logic, attributing hair loss, depression, anxiety, fatigue, and dry skin to a lack of B12, while GP4 says that ferritin levels below 100 cause a person to “vegetate,” then promoting their paid health community¹⁵.

¹⁴ The medical mega-influencers are identified strictly by the IDs assigned to them in section 5.4.3.1. Consistent with constructivist grounded theory (Charmaz, 2014), the goal of this analysis is to conceptualize social processes and patterns rather than to provide individual descriptions. Therefore, participants' actions and statements are invoked solely to illustrate the properties of the theoretical categories.

¹⁵ Additional corpus instances include GP6, who transforms the anatomical trait of joint hypermobility combined with spontaneous bruising into a diagnosis of “inflammatory vascular disease” and redirects the viewer to her infoproduct community; PLST, who reframes nocturnal awakening at 3 a.m. as a marker of melatonin deficiency

This step further involves optimizing content for visibility by following health topics that are performing well on the platform, including those from other specialties, and deploying markers of professional legitimacy to confer clinical weight on what is, functionally, the construction of a market.

6.4.2.1.2 Step 2: Invalidating the traditional scientific consensus

The second step is the delegitimization of medicine's institutional authority through the contestation of its traditional and consensus-based scientific foundations. Institutional medicine is presented as an obsolete, insufficient system captured by industry, and the MMI positions themselves as the holder of knowledge that the system conceals. The critique may be technical, as when GP4 questions the standard protocol for treating hypothyroidism: "How are you going to treat hypothyroidism with T4 alone? Where is the T3?" (GP4). It may be systemic: "The system does not want you healthy, the system wants you sick" (GP4); or it may operate through the implicit dismissal of other physicians' authority, using content titles such as "The secret no physician reveals" (GP1), "They never told you this" (GP6), or "What nobody told you" (F/CM). This type of title is the expression of a platform grammar in which the promise of revelation is an attention capture mechanism, not an occasional byproduct.

Whereas professions must continuously invoke occupational ideology to defend their legitimacy before the state and the public (Larson, 2017), Step 2 of Vector 1 (S2/V1) inverts this logic, given that the MMI uses professional prestige to question the scientific consensus that underpins the profession. The physician who performs this movement acts against the collective professional project, mobilizing individual status for personal ends. This constitutes an 'unintentional emergent effect,' in Archer's (2020) terms, because the grammar of revelation and secrecy implies casting doubt on the integrity of the medical profession as a whole, undermining *esprit de corps* regardless of each agent's orientation.

The pursuit of public trust as a substitute for peer validation is not new to Brazilian medicine. Since the nineteenth century, different segments of the profession had recourse to public communication as a space of legitimation when the internal mechanisms of the profession proved insufficient (Coelho, 1999; Pereira Neto, 2000). V1 radicalizes this movement by making the audience, rather than peers, the permanent arbiter of authority.

and elevated cortisol; and GP3, who attributes a diffuse symptom cluster to iron insufficiency and offers a paid event and community subscription as resolution.

6.4.2.2 Phase B: bridging the connection and resolution (the task)

The intermediate phase of the vector includes the steps Engineering parasocial intimacy, Manufacturing a substitute expertise, and Colonizing non-clinical domains.

6.4.2.2.1 Step 3: engineering parasocial intimacy

The first step of this phase consists of establishing proximity and accessibility, operating in the register of affect and belonging. Engineering parasocial intimacy begins by breaking the hierarchy of institutional medicine. The physician uses informal language when addressing the audience, exposes aspects of their private life, and expresses indignation or complicity.

Language use is illustrated in terms of form by verbalizations of affection such as “My dears” (GP3) and the use of identity-based nicknames for followers, such as “Ginemusas” (GYN). In terms of content, the mechanism operates when GP6 mentions her mother’s and grandmother’s legs to discuss lipedema and how she “broke” the genetic inheritance through her “anti-inflammation” method; when ORTH interrupts a live medical explanation to receive a delivery from Mercado Livre, a Latin American marketplace platform; and when GP4 exposes the cure of his employee’s depression and pain to validate his parasite cleansing and metal detox method after “traditional medicine failed”.

As a transversal mechanism, ‘Fostering parasocial intimacy’ (Initial code 1–Constructing a relatable persona’s) is rarely distinguishable in itself and requires contextual observation. Whereas in Vector 2 (V2) trust contributes to educating the audience and attracting patients to the clinic, in V1 parasocial intimacy serves to build confidence that will subsequently be converted into sales. Baym (2018) defines the effort to lead the audience to participate actively through authenticity as relational labor, since measurable engagement is the main indicator of credibility and value in the platform economy (Duffy *et al.*, 2021; Oliva, 2022). In medicine, this practice replaces professional institutional validation with platform logic, establishing audience metrics as the operative standard of authority.

The construction of the relatable persona here involves the acceptance of the “algorithm fallacy” (Araujo, 2021, p. 30), where the physician shapes their professional identity through metric feedback (what sells/engages). They become a self-enterprise, defined by Arriagada and Ibáñez (2020) as the continuous execution of work upon oneself (autolabor) for personal promotion. Guided by metrics, the physician assumes the identity of an entrepreneur, and the “performance” becomes their professional reality, a state where the performer is “taken in by his own act” (Goffman, 2002, p. 11).

This step's communicative informality is in part due to format constraints imposed by the platform. Research on gynecologists and obstetricians on TikTok discovered that the platform's native tools force physicians to produce content that meets entertainment standards as a structural requirement for reach, regardless of the individual practitioner's stylistic preferences (Stein; Yao; Aitamurto, 2022). The corpus's de-hierarchization of visual and linguistic presentation is therefore not reducible to a strategic choice by the agent; it also reflects the communicative grammar that the platform requires for visibility.

The use of attire as a mechanism of de-hierarchizing visual presentation (Initial code 3 – Constructing a relatable persona) to break the physician-patient visual hierarchy is also a transversal mechanism of Engineering parasocial intimacy. However, V1 is distinguishable here by the intensity of informality. Beyond dispensing with the white coat and formal clothing, practitioners in this vector appear shirtless in beach, pool, gym, or bedroom settings (SURG1, SURG2, ANG, and GP4). This breach of protocol is frequently defended as a mark of authenticity, as illustrated by ANG when responding to a user who called him “ridiculous” for appearing shirtless. “Despite being shirtless, I am here as a physician to help you”. Bodily informality extends to the intimacy of domestic everyday life, with GP3 washing her hair in the shower and producing “Get ready with me for Christmas” tutorials or ANG wearing a wig and a women's dressing gown to embody the opposite gender in a video. This amounts to constructing a personal brand defined by the physicality of the performance.

Step 3 of V1 is completed by the emotional register of language, in which the MMI is ‘performing indignation or radical honesty’ (Initial code 2), whether for confrontation, denunciation, or scolding the audience. As a transversal mechanism, its function varies dramatically according to the vector. In V1, the mechanism assumes traits of anti-establishment histrionics. GP4 criticizes conventional treatments by exclaiming, “For the love of God!” and labels physicians who rely solely on reference values as “by the book dude”. Along the same lines, GP6 adopts a mocking tone toward peers who criticize her approach to “chronic inflammation”. “That's something stupid people say (...) What kind of medical school did these people attend, dear God?”, creating an emotional bond with followers against traditional approaches. Radical honesty, in turn, operates in the register of moral shock to reprimand the audience for their habits, as illustrated by PLST when referring to cheese consumed by followers as “putrefied mammary excrement from a cow”, deploying repulsive language to generate disgust toward common dietary habits and converting aggressiveness into moral authority to sell alternative solutions.

This amounts to constructing distinction through the deliberate transgression of the epistemic, ethical, and performative boundaries of institutional medicine. Conspiratorial approaches (e.g. saying that ‘they’ are against the revelation of a truth), natural alternatives promotion (home remedies, herbal treatments, radical dietary restrictions, and supplements), and challenging the consensus produce personal brands grounded in claims of superior authority and violations of MOP. This transgressive differentiation inverts the ‘professional project’ (Larson, 2017) through which medicine secured its monopoly by claiming scientific superiority over ‘ignorant’ quacks and positioning itself as a defender of public welfare rather than profit (Coelho, 1999; Pereira Neto, 2000). What the Imperial Academy of Medicine once classified as quackery (not following consensus-based scientific foundations, exploiting public fear) now produces Performative Credential (in the terms discussed in Chapter 6.1).

As explained in Chapter 2, platform recommendation systems privilege content with emotional intensity because it generates engagement, despite its conspiratorial nature, its controversiality, or even its veracity. Consequently, claims of transgressive differentiation tend to perform highly in views, comments, and shares precisely because they violate professional consensus. This tactic demonstrates that under certain platform conditions, abandoning occupational professionalism’s standards becomes more economically rational than maintaining them. The rhetoric of fighting ‘big pharma’ or ‘the medical industrial complex’ enables direct-to-consumer commerce that bypasses institutional gatekeeping, so commercial motivation is laundered as altruistic truth-telling.

Audience segregation, which is a basic condition of impression management (Goffman, 2002), is eliminated by the context collapse of platforms (Marwick; boyd, 2010). Peers, potential patients, sponsors, and the state receive the same content simultaneously. As seen in Chapter 2, healthcare professionals respond to this condition in two ways. Some engage in contextual collusion, merging accessible, knowledgeable, and popular facets of identity (Atef; Fleerackers; Alperin, 2023). Others attempt, but fail, to restore the definition of spheres because the blurring of boundaries on platforms is independent of the agent’s intentions (Maggio *et al.*, 2023). In V1, the fused identity is the product being offered, and context collapse is the platform condition that makes its reach possible.

The performance of intimacy and expertise that converts spectators into consumers is the same that exposes the physician to institutional scrutiny. An empirical example involves the lawsuit filed by the Brazilian Attorney General’s Office (*Advocacia-Geral da União*) against GP4, which culminated in a judicial order to remove content associating mammography with increased breast cancer incidence (Brasil, 2025a). GP4 could not limit his

conspiratorial rhetoric to just his target audience. However, the public reaction to the case evidences the rupture of authority generated by context collapse. Whereas the judiciary sanctioned him, comments on media outlet posts reporting the ruling were predominantly in defense of the physician. The episode demonstrates the collision of regimes in which the same transgressive performance that is punished under MOP's institutional metrics proves highly successful in retaining audience trust and engagement under POP's logic.

6.4.2.2.2 Step 4: Manufacturing substitute expertise

This step involves manufacturing substitute expertise through the central and exclusive behaviors of Vector 1. After discrediting institutional authority, the influencer steps in by suggesting a parallel hierarchy of prestige that boosts their authority through alternative markers, and second by proposing medical approaches different from those of conventional evidence-based medicine. This amounts to creating a parallel 'expertise' that legitimizes unconventional health approaches and products (such as supplements and integrative/alternative therapies).

The citation of scientific literature, which in the MOP serves for epistemic compliance, is reconfigured in V1 as an instrument for legitimizing positions that contradict the consensus. GP6 uses references from PubMed and meta-analyses to address chronic inflammation and supplementation protocols; GP5 uses specific scientific articles to support the use of lard in the treatment of alopecia, relying on scientific evidence to promote a non-consensus therapy. In both cases, the form of the CFM standard is met, but without following its central validation criterion, which is peer consensus on the evidence. Thus, the MMI uses selective bibliographic citations to build alternative expertise and to position itself above conventional medicine, maintaining the appearance of compliance.

Kaul *et al.* (2024) documented that self-promotional content scored lower on indicators of scientific quality than clinical and educational posts by physicians on Instagram. The research compared post categories from the same individuals, demonstrating that the self-promotional format depresses epistemic standards regardless of the physician's overall orientation. Beyond the selective use of scientific literature, authority in V1 is also constructed through non-epistemic markers that bypass peer validation entirely.

Building authority in alternative health approaches (Initial code 1 – Assembling a counter-expertise) includes mentioning the number of followers and digital popularity awards, self-proclaiming themselves as a reference in the field without credentials to do so, using technical jargon as epistemic dumping, and healing testimonials as testimonial validation.

These practices prepare for the recommendation of alternative approaches, such as integrative, functional, or holistic medicine, ozone therapy, and detoxification.

Both generalists and specialists present themselves as practitioners of integrative medicine, a specialty not recognized by the CFM that claims to treat the ‘root of the problems,’ while traditional medicine merely ‘masks’ symptoms with industrialized medications. To defend the approach, some MMI criticize medical superspecialization as a ‘reductionist subdivision’ of the human body (CARD/NUT, GP4), proposing in its place a systemic view that often merges biological, mental, and spiritual processes.

To validate the approach that escapes traditional scientific scrutiny, the validation criterion is also reconfigured for testimonies and anecdotal cases (Initial code 3 – Assembling a counter-expertise). While the MOP requires evidence-based medicine, the POP values social proof. Thus, a WhatsApp message screenshot, before-and-after photos of clients (GP6), testimonials from family members or employees who were treated (GP4), or interviews with satisfied patients (ORTH) are considered valid as proof of healing, replacing the clinical trial with witness testimony.

The ‘dumping out technical terms’ (Initial code 2) functions as a rhetorical defense of the methods advocated by the MMI and consists of the use of medical jargon, complex biochemistry, or names of metabolic pathways, often out of context or in excess, to perform hype-competence. It is not proof of knowledge but a performance of competence for the layperson, typical of the managerial/organizational validation described by Fournier (1999). It serves to disqualify standard medicine, classified as simplistic, and to silence the lay questioning, creating an illusion of deep science and a knowledge gap between the MMI and the ‘traditional physician.’ GP4 criticizes conventional treatments for hypothyroidism, saying that “the only function of 100% T4 through deiodinase enzymes 1 and 2 is that it turns into 80% of total T3”, without explaining what deiodinases are and the specific percentages and suggesting that conventional physicians do not investigate the cause. In this movement, the MMI subverts the educational premise of medical professionalism (MOP), as technical language becomes a tool for hierarchization and self-aggrandizement, rather than for translation or health education.

In turn, GP1 uses linguistic resources to refine the sales pitch. In a video about magnesium, she quickly lists terms like “dimagnesium malate... improves mitochondrial function... magnesium taurate... magnesium threonate”. The use of specific biochemical terms validates the product and transforms the supplement into an advanced medical technology, which would not happen if she simply said ‘Take magnesium, it’s good.’ CARD/NUT also

adopts the strategy, but with a focus on peers who share his approach. He uses complex terms originating from physics and systems theory, such as “open system,” “emergent property,” and “trim tab”, to explain his method and promote his postgraduate course to other physicians, remaining cryptic to the lay public.

6.4.2.2.2 Step 5: Colonizing non-clinical domains

The fifth and final step of phase B involves the incorporation of non-exclusive or non-medical domains, such as spirituality and religion, beauty, well-being, lifestyle, and weight loss, the latter of which can also be addressed by nutritionists and physical educators. The expansion both broadens the addressable market and creates regulatory distance from the CFM. The religious mention functions as a mechanism of tribalization and worldview. By proclaiming “less medicine, more Jesus Christ”, ANG proposes faith as an alternative to pharmacology, incorporating spirituality as a personal merit and creating a bond with the Christian audience. This mechanism reaches its peak in V1 with GP6, who merges theology and therapy by offering “Saint Hildegard’s psychotherapy”, reading Psalms, and suggesting the use of amethyst stones to treat “vices of the soul”. In contrast, V2 addresses spirituality strictly as an adjunct to the patient's well-being, as illustrated by CARD4 suggesting that “reciting the rosary at the right pace” can slow down breathing and decrease heart rate and blood pressure, “providing peace and overall well-being”.

Finally, addressing topics that are not the exclusive domain of medicine distances the content from CFM oversight and paves the way for the commercialization of products in the subsequent phase of V1, such as GP3’s proprietary hair cosmetics and CARD/NUT’s courses and DVDs teaching the public how to achieve “prosperity” and “success,” “get rich”, and prepare for civil service examinations and the Brazilian Bar Association exam (Ordem dos Advogados do Brasil). This pattern is consistent with Cunningham and Craig’s (2021) description of the commodification of personality traits, in which the attention and reputation gained in one area allow for autonomous revenue streams. In V1, medical authority establishes initial audience trust, but commercial operations gradually move into domains where CFM jurisdiction is ambiguous or absent. Medicine provides credibility and the platform provides the reach, while the products sold belong to neither register.

6.4.2.3 Phase C: converting digital labor into sales (outcome)

The commercial conversion is the final stage of the vector and includes the steps Executing the commercial conversion and Displaying compliance. The first activates the sales

structure, and the second seeks to mitigate legal and ethical risks related to social media production.

6.4.2.3.1 Step 6: Executing the commercial conversion

The goal of this step is to convert the audience into consumers of supplements from partner brands, proprietary digital products, and self-management health products. The health advertising ecosystem on platforms is the environment in which this conversion operates. A recent report from NetLab-UFRJ found that 76% of the health ads they looked at were fraudulent, frequently deploying white coats and physician imagery to sell ‘miraculous’ solutions (Santini et al., 2025), a pattern that the commercial infrastructure of V1 both exploits and reproduces.

The methods of generating revenue through non-clinical assets include direct monetization of fans, such as YouTube’s ‘Join’ and ‘Super Thanks’ features and paid subscriptions/memberships on the three social media platforms analyzed, as well as payment through YouTube’s advertising revenue-sharing programs and TikTok’s performance-based bonuses. Indirectly, digital presence drives the sale of proprietary books directed at laypeople and educational services for other physicians, as well as attracting patients to the medical clinic. These monetization tools are transversal and are not distinguishing markers in isolation, with minor differences between the two vectors, for example, whether the books are self-produced ebooks (V1) or books published by publishers and distributed by bookstores/marketplaces (V2). The defining differentiators include partnerships with nutritional supplement brands and the sale of self-management health products, which are exclusive to V1, as well as remuneration through sponsorship and individual or institutional advertising in MMI content, with a clear separation between editorial and commercial aspects, exclusive to V2.

Supplements and self-management health products often coexist in their promotion. The rhetorical construction of need, which we analyzed in the previous phase, precedes both. Advertising is often explicit; for example, a video about a vitamin is accompanied by an affiliate link from the partner brand in the description, the product is displayed on screen, or the influencer offers discount coupons. CLS, for example, links lack of energy to the absence of coenzyme Q10 and inserts in the video description the link to “Antioxidant that I recommend”. The sales mechanism takes on the format of a talk show with PSYCH1, who interrupts the interviews on his podcast to display the supplement containers on the table and

announce that “we have our QR code here on the screen, coupon... over on the [brand’s name] website”.

In addition to the defense of encapsulated nutrition, justified by GP1 under the motto that “supplementing is not a fad. It’s prevention”, there is the encouragement of the consumption of high and supraphysiological doses, disregarding traditional reference values. This discourse culminates in the creation of standardized consumption protocols, as ANG does when presenting “the seven most promising supplements” for joint pain, transforming health recommendations into a direct sales funnel. This contrasts with V2’s transparent institutional sponsorships, where paid promotion is explicitly indicated.

Engagement mechanisms reinforce the sales structure by simulating the diagnostic process. The MMI creates questionnaires or ‘quizzes,’ on inflammation, thyroid, and parasitic infections that simulate an anamnesis (GP1, GP6) and whose results point to ‘dysfunction’ or ‘risk,’ followed by the offer of an infoproduct. The launch ritual complements this structure through the orchestration of artificial scarcity, such as weeks of free classes (CLS, GP6, F/CM, SURG2), ‘challenges’ (ANG), reduced prices for a limited time (PSYCH2, SURG2), waiting lists (PLST, PSYCH2), and limited availability, always with an end date, creating urgency to purchase.

The promotion of supplements documented in the corpus violates Resolution No. 243/2018 of Anvisa (Brazil, 2018). Even when they include disclaimers about the need for clinical monitoring before use, some MMI engage in risky conduct. The standard prohibits attributing therapeutic purposes to supplements, but ANG recommends type 2 collagen and omega-3 to combat joint pain, and GP1 recommends lysine and green propolis to cure herpes. Anvisa determines that nutrient amounts must comply with the limits established for specific population groups, but CLS disputes the laboratory reference range for vitamin B12 (190 to 560) and recommends levels around 800, while ORTH reports applying 600K units of injectable vitamin D3 in regenerative patients. The Anvisa’s standard prohibits claims that conventional food is unable to provide the components necessary for health, but GP1 states that soil depletion makes full health through food alone unlikely, and PLST declares that contemporary nutritional imbalance makes it impossible to obtain omega-3 through diet.

The second critical regulatory aspect is that of self-management health products¹⁶, the quintessence of V1. While professional practice is fundamentally characterized by inference,

¹⁶ In contrast to supplement advertisements, these products extend beyond the observational limits of this study. To understand the components of these infoproducts, I analyzed their descriptions on specific websites where they are sold, as well as participated in WhatsApp and Telegram groups for the MMIs that offer them.

that is, the process that connects abstract knowledge to diagnosis and treatment in specific cases (Abbott, 1988), the digital environment requires a translation of medicine that is industrial, asynchronous, and transferable. Thus, the logic of scalability and standardization of platforms challenges the individualized, fiduciary, and tutelary nature of the medical act, and this conflict materializes in the form of the infoproduct.

This monetization strategy aligns with the structural mechanisms of the platformized health market. Captured by fear or sensationalism and led from social media to private messaging apps like Telegram and WhatsApp, people find health content that is a “conversion funnel” to the selling of supplements, courses, and alternative therapies, according to the study by Silva *et al.* (2025, p. 39) on anti-vaccine and conspiracy-minded Latin American and Caribbean Telegram communities. This lucrative business model begins through social media and generates profit for the platforms, which earn from health-related advertising, regardless of its scientific accuracy (Santini *et al.*, 2025).

The designations vary; among them are method, protocol, code, guide, and course. What is proposed also varies, from guides to promote self-awareness and provide actionable instructions to reduce symptoms (ANG, GP6, PSYCH1) to specific guidelines, such as “natural deworming” (GP1, GP4). Some promote the expectation of results by presenting customer testimonials reporting symptom relief (CLS, ORTH, PSYCH2) or significant weight loss, including statements such as “You can lose 10, 15, 20 kilos in weeks” (SURG2) or “Want to lose up to 10 kg in the next month? Join my WhatsApp group” (ANG).

This content operates in an area that the CFM has not yet defined. The collegiate body differentiates health information (permitted) from medical consultation (regulated) based on the assumption that the recipient understands the communication as general, not as specific treatment. This assumption is acceptable for books, articles, or media appearances, as these are unidirectional formats with limited specificity and no interactive medium with the physician. The infoproducts sold by V1 MMI are courses built around self-management health protocols designed for direct application by the consumer. This characteristic weakens the assumption, because even if the physician does not explicitly compare their product to the medical act, it is impossible to prevent the user from expecting this equivalence. The CFM prohibits consulting from replacing in-person care (Resolution No. 2336/2023), but the rule regulates the intention of the sender and ignores the effective appropriation by the receiver, since the meaning of the message is actively constituted in reception and interaction (Martín-Barbero, 1997). By standardizing instructions in scalable products, the MMI

eliminates the case-by-case discretion that is inherent in occupational logic (Freidson, 2007) and transfers the risk of conduct without in-person diagnosis to the consumer.

The pricing structure documented in the corpus reveals the commercial architecture of this conversion. PSYCH2 sells a training program for depression management at BRL 1,497 (or twelve installments of BRL 154.82), explicitly comparing its cost to annual in-person treatment (which he prices at BRL 12,000 for his consultation rate) and claiming therapeutic equivalence with in-person therapy while offering greater contact hours with the professional. ORTH offers a course in regenerative medicine protocols at BRL 597, framed explicitly as an accessible substitute for patients who “do not have money to pay for a consultation.” GP4 sells community access at BRL 2,447 (or twelve installments of BRL 249), while simultaneously offering Instagram subscriptions at BRL 16.99 per month and TikTok subscriptions at BRL 29.90 per month. In this stratified access structure, free content constitutes the top of the funnel, infoproducts represent the intermediate tier, and clinical consultations occupy the bottom of the funnel, accessed by higher-ticket clients.

What this structure obscures is that free platform content functions as a facade of democratization. Consultation fees ranging from BRL 3,000 to BRL 5,000¹⁷ place individualized access out of reach for most of the audience these physicians cultivate online. My field observation during immersion documented audience members explicitly stating their inability to afford private consultation. The infoproduct partially simulates the consultation through features such as WhatsApp messaging and email access to the physician’s team, without the regulatory protections that govern in-person care. Infoproducts are distributed behind paywalls and in members-only areas of proprietary platforms. Therefore, they are outside the scope of ordinary CFM monitoring. The lack of anonymity in the CRM reporting system discourages the use of the mechanism in situations where the violation is more opaque.

6.4.2.3.2 Step 7: Displaying compliance

The seventh and final step of V1 is to demonstrate compliance through legal notices and disclaimers, exempting the MMI from liability for what the audience does with that information. Therefore, the disclaimer of purely informative content functions as impression management (Goffman, 1988) that projects a facade of ethical compliance to avoid

¹⁷ Consultation fees for the MMI who still practice were verified through public scheduling platforms, clinic WhatsApp message interactions, and audience comments on the physicians’ content between June and August 2025. These values were part of the netnographic immersion, not the systematized corpus in section 5.4.3.

punishment. According to healthcare professionals interviewed by Ruan *et al.* (2020), legal disclaimers offer insufficient protection because platform use merges professional and personal networks, exposing content to evaluative regimes for which it was not designed. Thus, the differentiation between information and medical advice becomes a rhetoric of protection, corroborating Larson's (2017) analysis that ethical discourse can be instrumentalized to defend financial gains.

In this vector, the MMI operates as an 'entrepreneur of the self' who converts digital fame into revenue, as described in Chapter 2. The CRM¹⁸ number, lab coat, stethoscope, and the pronoun Dr. [*dr./dra.*] in the channel name make up the facade of authority that converts visibility into transactions, as the same markers that the MOP requires as a guarantee of competence are mobilized here as inputs for commercial operation.

Abbott (1981) warned that the dispersion of professional charisma via the media tends to erode jurisdictional supremacy in the long term. The 'platformized' version of this process, in which visibility is measured by engagement metrics and authority is validated by followers rather than peers, represents the contemporary and amplified realization of that warning. By repeating itself on a large scale, the process produces what Archer (2020, p. 144) calls "structural elaboration", that is, the condition of possibility that sustains it is transformed in the course of its own operation. What emerges goes beyond the modification of a system of authority, amounting to the constitution of a substitute system. It is precisely because it represents the scenario of greatest tension with the regulatory apparatus, culminating in the creation of this parallel system, that the presentation of V1 preceded V2 in this chapter. The radical nature of its reconfiguration maneuvers required a level of detail that V2, due to its more restrictive nature and alignment with occupational liturgy, does not demand.

¹⁸ The requirement to display the professional registration number (CRM) and specialty number (RQE), established by CFM Resolution No. 2336/2023, is not consistently met throughout the corpus. 14 MMIs omit the CRM on one or two platforms, with a focus on YouTube and TikTok; 7 MMIs omit it from all platforms, while only five mention it on all platforms. The distribution of omissions by platform is consistent with practice modifications in response to structural differences in each environment, such as platform-specific self-branding (Scolere; Pruchniewska; Duffy, 2018). According to Arriagada and Ibáñez (2020), Instagram prioritizes a fixed biography for creators, while YouTube and TikTok prioritize content description per video (Shi, 2024; Simpson; Semaan, 2023). Displaying the registration depends on the creator's active and recurring decision for each publication. Because the CFM standard is relatively new, it is difficult to distinguish between deliberate noncompliance and ongoing institutional adaptation. This fact is documented as empirical data, demonstrating the difference between the regulatory requirement and the practice observed in the corpus, and it is consistent with the structural regulatory gap discussed in section 6.3.6.

6.4.3 Vector 2: Professionalizing Health Communication

The structural incompatibility between POP and MOP does not imply that every action violates one of the regimes. There are areas of convergence, complementarity, and interstitiality that V2 systematically exploits, as discussed in the methodology chapter. Conversely, this vector represents the agent's exhaustive effort to sustain Fiduciary Authority Protection (MOP) while inhabiting an infrastructure governed by the Performative Credential (POP). Here, the physician strategically employs platform tools to safeguard occupational legitimacy and defend public health, not to commodify their practice.

The platform seems to be an arena in which the profession can claim authority before the lay public. Here, the MMI uses the infrastructure that threatens them to defend their territory, to inform and validate public health institutions and current official guidelines, and to combat narratives that undermine trust in established treatments. Historically, Brazilian medicine served a similar purpose when the First Republic's hygienist elite intervened in public problems and established collective authority through mass communication (Pereira Neto, 2000). The structural difference is that now each act of communication generates performance data that the platform uses to decide its distribution. This model of public health converted into broadcasting operates under the 'invisible management' of platforms (Pithan; Closs, 2024), following prescriptions for professional creators, such as SEO, visibility-driven creation, and persona building (Pithan; Araujo; Dalmoro, 2025), mobilized here for the purposes of defending the profession and individual reputation.

Table 20 — Vector 2 dynamic mechanism structure

Phase A: Anchoring rational-legal authority (setup)	
Steps	Focal codes
1. Anchoring authority in credentialism	Anchoring authority in credentialism Asserting professional legitimacy*
Phase B: Delivering evidence-based health information	
2. Producing platform-mediated medical broadcasting	Constructing a relatable persona* Professionalizing communication in healthcare Adapting content to visibility-driven criteria*
3. Exercising pedagogical stewardship	Exercising pedagogical stewardship Expanding health scope* Incorporating non-clinical domains* Deploying ethical shields*
Phase C: Converting digital labor into revenue (outcome)	
4. Monetizing authority without commodifying medicine	Building private distribution channels* Generating revenue through nonclinical assets * Monetizing through institutional channels

*Operational Mechanisms of Platform Adaptation

V2 unfolds across three phases. In Phase A (setup), the agent anchors authority in credentialism, transferring to the digital environment the rational-legal legitimacy accumulated through formal credentials and institutional affiliations. In Phase B (the task), the agent produces platform-mediated medical broadcasting and exercises pedagogical stewardship, deploying visibility tools in service of evidence-based health communication. In Phase C (outcome), authority is monetized without commodifying medicine. These steps are synthesized in Table 20 and detailed subsequently.

6.4.3.1 Phase A: Anchoring rational-legal authority (setup)

The first phase has only one step and represents the transfer to the digital environment of offline rational-legal authority, based on belief in formal norms and competence validated by rules and statutes (Weber, 2011). Unlike V1, where authority needs to be manufactured because the physician lacks either specialized credentials or a reputation based on long practical experience, here authority preexists and is carried over from doctoral degrees and teaching positions at elite institutions, affiliations with leading hospitals, and specialty registration (RQE).

For example, CARD2 and PED1 have doctoral degrees and are professors at the University of São Paulo School of Medicine (FMUSP). CARD4 highlights his doctorate in cardiology from the same university and displays his affiliation with the clinical staff of the Sírio-Libanês Hospital, Albert Einstein Hospital, and as an assistant physician at InCor (Heart Institute of the FMUSP Hospital das Clínicas). CARD2 reports his position as director of the Cardiology Center at the Sírio-Libanês Hospital and president of InCor.

In V2, credentials attest to the authority and experience of professionals, giving them access to the mass audience in a manner similar to that of traditional media. This comparison is reinforced by the fact that some of the physicians investigated are frequent interviewees or columnists on open radio and TV channels, news portals, daily newspapers, and magazines (GP2, CARD2, CARD4, PED1, PED2, PSYCH1). Length of practice is a characteristic of this cohort, as the ‘least’ experienced member has been a physician for 37 years.

6.4.3.2 Phase B: Delivering evidence-based health information (the task)

The intermediate phase of V2 consists of the steps Producing platform-mediated medical broadcasting and Exercising pedagogical stewardship. The MMI operate as media

companies or professional communicators, professionalizing their communication by adopting the language of the media without losing decorum and without making academic authority ‘boring’ or invisible to the platforms’ curation system.

This performance brings physicians closer to what Abbott (1981) and Douglas (1966) describe as professional impurity. Any medical influencer illustrates Douglas’s (1966, p. 36) definition of impurity as that which is “out of place”, things that cross boundaries and violate cultural categories. In turn, medical specialists who create health content generally embody professional impurity as described by Abbott (1981), characterized by the front line of a profession that is in direct contact with the complex, undefined, and disordered totality of clients, such as general practitioners and emergency room physicians. These “impure” professionals are the most prestigious among the public, in opposition to the intraprofessional hierarchy, which values hyper-specialists (Abbott, 1981, p. 134–135). As consultations with the professional elite are inaccessible to the masses, performing professional impurity is a resource for bonding and credibility with the public. Examples include PED1, a pediatrician, talking about adult health with a family preventive bias, and CARD4 integrating faith and prayer as a complementary form of comfort for cardiac patients.

6.4.3.2.1 Step 2: Producing platform-mediated medical broadcasting

The physician acts as a presenter or broadcaster, producing evidence-based content adapted to the format and language required by the platform, without compromising epistemic rigor. The POP’s requirement for linguistic flexibility to ensure digital reach challenges the sobriety and epistemic rigor that the council (MOP) advocates. Therefore, V2 requires sophisticated communication competency, such as adapting content to audience interests without resorting to sensationalism, simplifying the explanation of complex topics without distorting them, and managing audience engagement without manipulative emotional appeals. The work requires maintaining professional dignity in an environment that rewards histrionics and conflict.

The focus on visibility involves addressing seasonal and appealing topics, like “Sleeping with socks in winter” or “Whooping cough” (PED1), as well as contradicting popular themes addressed by V1 representatives. This is what GP2, CARD1, CARD4, and PSYCH2 do by explicitly condemning unsupervised nutritional supplement use (which is indirectly encouraged by those who promote nutritional supplements) and attributing the trend towards these products to commercial rather than clinical needs. Among these, CARD4’s condemnation of unsupervised supplement consumption is explicit at the level of corpus

language. He instructs his audience to replace “the search for shortcuts with a routine that genuinely nourishes,” arguing that “true immunity is not on the shelves,” and redirects viewers to seek clinical evaluation, stating that “professional guidance is fundamental” for the consumption of these compounds. The phrasing targets the commercial supplement market directly, framing it as a substitute for clinical mediation rather than a complement to it.

Some young MMI have internalized the platform’s logic and are skilled in the linguistic plasticity of translating MOP into the grammar of attention. For example, IDS uses the language of entertainment and humor to ensure the retention of messages about infectious diseases, attracting institutional advertising (Ministry of Health and Disney Plus) that seeks this reach. He asks, “Don’t like sucking but put the ‘lollipops’ out to fight?” and “Did you get a load of ‘milk’ and panic?” referring to the male genitalia and sexual intercourse, to introduce the technical explanation about Post-Exposure Prophylaxis and HIV prevention. He adheres to ethical guidelines, like recommending seeking medical help within 72 hours, uses the audience’s slang and, at the same time, circumvents the platforms’ ban on graphic language about sexual organs and acts.

Another example of balancing form and content is CARD1, who uses resources bordering on alarmism, such as clickbait structure (capitalized titles, sense of urgency), to deliver conservative content. He warns about the side effects of medications and considers that, even with discomfort, patients cannot stop taking medication without professional supervision. “The supplement you take every day to protect your heart may actually be clogging your arteries”, he warns, and includes in the description links to the three academic studies mentioned in the video.

Part of the corpus declares the relationship with professional production or marketing companies (CARD2, GP2, IDS, PED2, PLST, PSYCH1). This indicates that the precariousness of content creators caused by task overload and uncertainty regarding financial return (Arriagada; Ibáñez, 2020; Burgess; Green, 2018; Cunningham; Craig, 2021) is not reproduced among medical mega-influencers. They already start from a privileged position, as they have a primary financial source, with social media work being a means of increasing revenue.

6.4.3.2.2 Step 3: Exercising pedagogical stewardship

The third step is the exercise of pedagogical curation. The physicians position themselves as judges of health information, correcting misinformation, contextualizing studies, and defending scientific consensus and public health. For example, CARD2 addresses

vaccine-related misinformation in cardiovascular health, and PED2 and GP2 also fight disinformation about vaccines.

PED2 makes this pedagogical function explicit in his channel's framing, positioning his content as a direct response to "the main fake news about vaccines" with the declared goal of demonstrating "the importance of vaccinating our children." He also signals institutional allegiance through posts carrying the message "long live the SUS¹⁹," positioning his platform presence as a defense of the public health system, not only of a clinical practice. PED1, on the other hand, divides common beliefs into myths and truths. For example, she denies that lemon water thins the blood or that cold water speeds up metabolism. The myth-busting format is structurally embedded in PED1's content across platforms: her posts include a standing directive in capital letters ("NEVER SELF-MEDICATE. ALWAYS SEE YOUR DOCTOR") appended to all publications, converting the corrective move into a routine ethical marker.

This ethical framing also prepares the ground for thematic expansion. The social role of the educator strategically transcends the limits of RQE, incorporating general health topics and non-clinical ones (like behavior, spirituality, and well-being) to broaden reach and strengthen ties. As mentioned in the V1 discussion, expanding the thematic scope beyond medical specialties is associated with larger and more engaged audiences among healthcare professionals on the platforms (Du *et al.*, 2025). In V2, however, this expansion serves audience consolidation rather than commercial conversion, which is consistent with the finding that informational breadth predicts audience growth regardless of advertising performance.

The intermediate phase of the 'task' is completed with legal notices and warnings that the content is informative. The notices are complemented by explicit guidance for the audience to seek their physician (PED1) and warnings such as "I do not provide care or clarification of symptoms via WhatsApp" (CARD3), separating content creation from clinical practice. CARD2 exemplifies how this boundary is enforced through epistemic redirections embedded in content rather than only in formal disclaimers. Alongside correcting misinformation, stating that "there is no scientific evidence linking vaccines to cases of heart attack or sudden death", he formulates explicit prescriptive limits: "Any medication must be prescribed by a physician" and "Self-medication does not work; one must not self-medicate." The dual move, debunking a false claim and immediately anchoring resolution in the clinical encounter, structures the audience's position as one of informed deference rather than self-management.

¹⁹ Brazilian Unified Health System (SUS, for its acronym in Portuguese)

6.4.3.3 Phase C: Converting digital labor into revenue (outcome)

Financial conversion in V2 is a single step and is structurally different from V1, because monetization does not organize content, although it supports its production. Influencers ‘sell’ technical expertise and reputation, directly or indirectly monetizing their authority but without commercializing medicine. The reward may be direct or indirect, but it is always institutional. Direct monetization occurs through the platforms’ own remuneration programs, through platform features that allow fans to pay for subscriptions or one-off gifts (GP2, CARD1, CARD3, CARD4, PSYCH1), or through advertising and institutional and transparent partnerships, separating editorial and commercial content (GP2, IDS).

GP2’s channel makes this separation operational at the level of metadata. Commercial arrangements are disclosed through standardized statements such as “Content produced in partnership with RD Saúde, Drogasil and Raia” and “Content produced in partnership with Tena Brasil,” while the channel’s self-description positions it as journalistic and documentary production (“Brazil’s largest health channel [...] Speaking to everyone, with credibility”) realized through an external content production company. The descriptions also link to podcasts, content about the SUS, and documentaries on domestic violence, with no offer of self-management protocols, structurally separating the commercial slot from the editorial mission.

Indirect revenue is obtained by attracting patients to the office without promises of results (CARD3, CARD4, DRM, IDS, PED2); selling editorial books (GP2, GYN, PED1, PED2, PSYCH1); and the reverberation of participation in traditional media (CARD2, CARD4, GP2, PED1, PED2). Closed WhatsApp or Telegram groups serve to distribute educational content and build audience loyalty, bypassing platform curation, not for sales launches.

The OMPA mechanism that best illustrates teleological divergence is Generating revenue through non-clinical assets, as the difference between the vectors is of causal position, not degree or intensity. In the other mechanisms, V1 and V2 do the same thing with opposite intentions. In monetization, the logical structure of the process is reversed. In V1, revenue organizes content because its entire sequence (from the manufacture of insufficiency to substitute expertise) is designed to reach the commercial transaction. Content is the funnel, not the product. What is sold is the self-management protocol, the supplement, and the infoproduct. Medical content is the means of access to the consumer, not the end of the work.

In Vector 2 (V2), the logic reverses, allowing revenue to sustain content. Physicians produce health communication as an end goal, deploying revenue-generating mechanisms

such as editorial sales, transparent institutional advertising, and patient attraction to sustain that production. Revenue is the condition for continuity, not the criterion for content. This inversion is structural because it determines the patient's position in the process. In V1, the viewer is a potential consumer from the first contact; in V2, they are a recipient of information whose eventual conversion into a patient is a consequence. The medical authority that V1 consumes is the same that V2 needs to preserve.

6.4.4 Comparative synthesis: dialectical variations of authority in the substantive theory

The following synthesis advances from the empirical documentation of Vectors 1 and 2 to the theoretical propositions they sustain.

Viewed through the lens of critical realism (Archer, 2020), the substantive theory of Performative Reconfiguration of Professional Authority explains the social interaction ($T2-T3$) through which human agents manage incompatible structural conditionings ($T1$). Whether mercantilist (V1) or institutional (V2), the MMI's practices are dialectical agentic responses to being caught between two incompatible structural conditionings, namely the platform's prescriptions, operationalized through the Performative Credential, demanding metric visibility for digital survival, and the medical council's prescriptions, operationalized through Fiduciary Authority Protection, demanding anti-commodification for occupational legitimacy. The substantive theory demonstrates that when these two incompatible structural conditionings bear simultaneously upon the same agent, professional authority cannot remain a static attribute; it must be continuously and performatively reconfigured to satisfy both the algorithm and the institution.

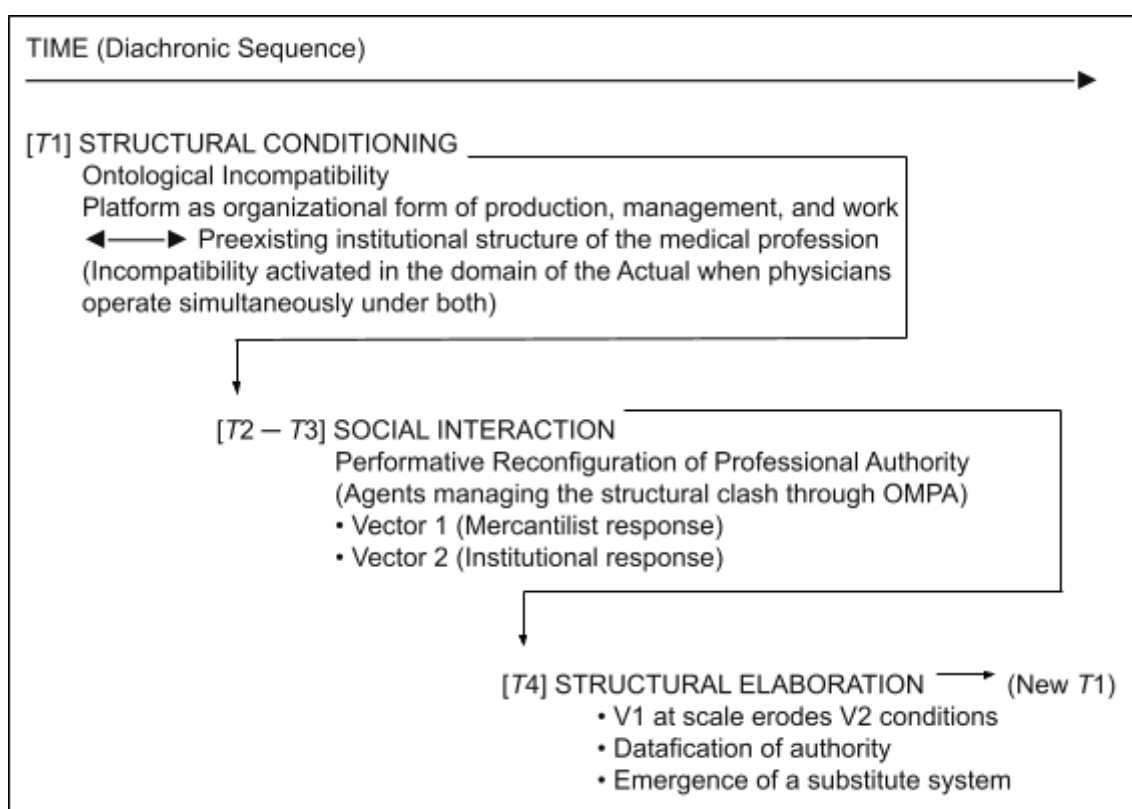
The classification of MMI into V1 and V2 is teleological rather than scalar. The vectors differ in the causal relationship between monetization and content production; V2 represents an inversion of the logical structure found in V1, regardless of intensity or degree. Instances where a practitioner exhibits specific V1 or V2 characteristics clearly are examples of each vector's defining mechanism at work, each confirming the classification rather than complicating it. The absence of hybrid cases in the corpus stems from structural incompatibility: a single mechanism cannot organize content around commercial conversion while simultaneously subordinating monetization to a communicative purpose.

The theory employs a binary structure, yet individual practitioners remain mobile. For example, two physicians who operated consistently within V1 reoriented their content production toward V2 during the four years of netnographic observation for this thesis. While this study was not designed as a longitudinal panel and therefore does not reconstruct their

reorientation, the observation confirms that vectors describe reconfigurable teleological orientations instead of stable personal attributes.

Credential saturation does not explain the content that physicians produce on platforms. What it does is condition who has structural incentives to produce it and why. Figure 12 situates the substantive theory of Performative Reconfiguration of Professional Authority within the morphogenetic cycle of Archer (2020).

Figure 12 — Ontological framework of the substantive theory



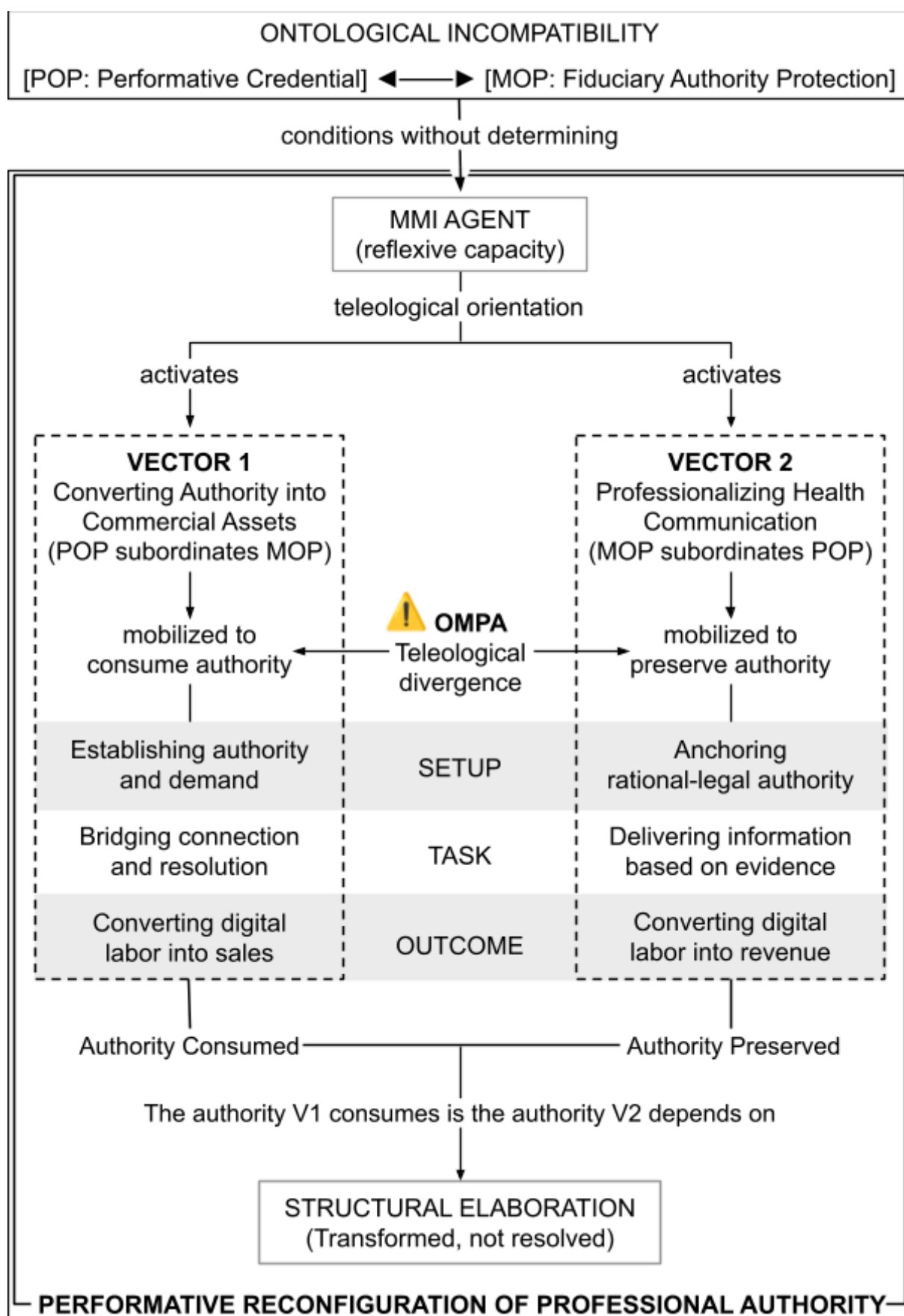
At *T1*, the platform's generative mechanisms and the preexisting institutional order of the medical profession create ontologically incompatible conditionings that shape (but do not determine) the actions of MMI. At *T2–T3*, agents respond to the incompatibility by reconfiguring professional authority in a continuous and performative manner, organized into two opposing vectors and enacted through the transversal Operational Mechanisms of Platform Adaptation. At *T4*, this agentic process leads to structural elaboration, as V1 practiced on a large scale erodes the institutional conditions that enable V2, whereas V2 seeks morphostasis within the same infrastructure that structurally favors its opposite.

What new insights does this theory provide that the available frameworks lack? Evetts (2013) identified the tension between occupational and organizational professionalism, but his typology describes two ideal types of control and work organization, while this research

presents them as two vectors available to the same agent and under the same condition. Performative Reconfiguration of Professional Authority empirically addresses a tension that Evetts (2013) identifies but does not exhaust, demonstrating that physicians do not choose one regime and abandon the other, but performatively reconfigure authority in divergent directions according to the teleological orientation they adopt. The corpus documents this simultaneity by showing that the same operational instruments, the same platforms, and the same content formats produce opposite trajectories according to the agent's teleological orientation.

Facing the same structural condition and deploying the same operational instruments, the agent produces dialectically opposed trajectories depending on the teleological orientation adopted. The OMPA is not an external layer to the vectors but a set of synchronic instruments that the agent mobilizes from within each trajectory and activates based on the orientation assumed. In Vector 1, POP subordinates MOP and revenue organizes content, making the audience a potential consumer and consuming the professional authority that Vector 2 relies on to function. In Vector 2, MOP subordinates POP and revenue sustains content, allowing the audience to receive information while preserving the institutional authority that collective medical legitimacy requires. The two vectors are not ideal types describing distinct agents, but simultaneously available responses to a single irreconcilable condition, enacted through the same instruments in three sequential phases. Because the incompatibility is not resolved but reproduced at a greater scale, the process results in structural elaboration rather than closure. Figure 13 illustrates the internal logic of the reconfiguration process. Following Gioia, Corley, and Hamilton (2013), the figure converts the static data structure into a dynamic theoretical model, emphasizing the causal arrows that keep the reconfiguration in motion.

Figure 13 — Performative Reconfiguration of Professional Authority: process model



Beyond the response to Evetts, the substantive theory documents a structural condition that the sociology of professions had not previously named in this configuration. Abbott (1981), drawing on Douglas (1966), established that intraprofessional prestige derives from purity, defined as the capacity to work with problems already abstracted and delimited by colleagues of lower status. Frontline professionals who receive the disordered, untranslated complexity of clients occupy the lowest positions in intraprofessional hierarchies, even as the public venerates them precisely for that proximity. The MMI instantiate a form of professional impurity that exceeds the analytical limits of Abbott's original formulation. To reach mass audiences on platforms, physicians of uncontested specialist standing descend to a frontline without triage, receiving the undifferentiated and algorithmically assembled public whose conditions, motivations, and literacy are unknown in advance. This movement, which intraprofessionally resembles a descent toward impurity, generates on platforms a form of prestige grounded not in abstract knowledge, but in the performance of accessibility itself. What is analytically new is not that frontline professionals draw charisma from contact with social disorder, a pattern Abbott (1981) already described, but that the platform allows this charismatic contact to be codified and scaled as a Performative Credential, decoupled from the institutional structures that previously governed both purity and prestige. The Performative Reconfiguration of Professional Authority names the social process through which this impurity is not merely endured but actively managed, in opposite directions, by agents who deploy the same structural condition for incompatible ends.

V1 empirically documents a condition outside the scope of available frameworks, namely the case of members of a profession under intact formal jurisdiction and without external challenge voluntarily translating their knowledge into scalable products embedded in organizational systems. Abbott (1988) had identified technology and organizations as the two main external forces that historically generated new domains for professions and destroyed existing ones. Social media platforms occupy both positions simultaneously, functioning as technical infrastructures that govern content distribution through algorithmic recommendation, curation, and moderation systems and as organizational actors that define the performance criteria that condition access to monetization, effectively determining what counts as a viable professional practice within their ecosystem. The Performative Reconfiguration of Professional Authority documents what Abbott (1988) analyzed as separate categories, presenting it as an unprecedented combination that maps the response of a jurisdictionally protected profession to an external force merging technology and organization into a single control device.

6.4.5 Market saturation and the platformization of medical careers

Understanding why this performative reconfiguration gains traction requires examining the current state of the medical profession in Brazil as a field, and not just the choices of individual agents. The MMI were investigated in this thesis as an empirical expression of the broader tendency toward the platformization of all work activities, that is, the growing dependence of the most varied forms of labor on digital infrastructures and their logics (Grohmann, 2021), from which not even the archetype of the professions escapes. Beyond the impact of the organizational context of platforms on the empirical field, however, the analysis revealed that the current state of the occupation itself contributes to the ‘platformization’ of medicine.

The saturation of the Brazilian medical market, documented in Chapter 4, has produced a surplus of credentialed professionals without differentiation. The accelerated expansion of medical schools between 2014 and 2024, with the private sector absorbing 91.5% of new places, alongside the absence of proportional growth in residency positions, tends to generate an increasing number of physicians without a registered specialty (RQE). While not statistically significant, it is noteworthy that some of the group’s most recent graduates received their degrees after 2017.

The physician with the most recent professional registrations in the corpus (2023) has maintained a career as a digital influencer since 2013 and, in addition to producing content focused on hair care, runs her hair product brand. Another physician from the same cohort has closed her practice to new patients, referring clients to colleagues on her team, while the other two most recently graduated MMI without RQE see patients exclusively through private consultations. These are markers of professional success and consolidated careers. When the digital performance metrics of these young physicians are observed to exceed those of specialists with decades of practice and doctoral degrees, it becomes evident that the Performative Credential outperforms traditional markers of authority on platforms and enables financial rewards substantial enough to reduce clinical practice or make it selective.

Credential saturation does not explain the content that physicians produce on platforms, but it does condition who has structural incentives to produce it and why.

The opposition between V1 and V2 is the contemporary manifestation of an intraprofessional conflict that Brazilian medicine has carried since the First Republic, when generalists, specialists, and hygienists disputed the legitimate mandate of the profession (Pereira Neto, 2000). According to Abbott (1988), such conflicts arise from internal differentiation within a profession, including variations in status, client types, work

organization, and career trajectories, and that vertical internal divisions emerge as a mobility strategy by groups seeking to ascend within the profession through prestige distinctions rather than task differences. The MMI introduces a new stratification whose differentiating criterion is the Performative Credential, based on platform metrics and visibility logic rather than clinical specialty or academic titles. Part of the corpus has specialist registrations and advanced academic degrees, but these credentials give way to platform-validated performance as the primary measure of professional standing in the digital space.

6.4.6 Structural implications: what reconfiguration produces

The analysis of practices in Vectors 1 and 2 documented the social interaction phase ($T2$ – $T3$) of the morphogenetic cycle. Now, Archer's (2020) cycle requires that the analysis advance to stage $T4$, where we verify how this agency produces the elaboration or reproduction of the structural and cultural properties of the medical field. The analysis of the structural implications of reconfiguration focuses on the medical profession as an institution, examining the effects of V1 practices on the collective authority of medicine and the regulatory apparatus that sustains it, before placing this process within the larger context of specialized work's platformization.

V1 practiced on a large scale erodes the conditions that make V2 possible. As the audience becomes accustomed to expecting medical influencers to provide self-management solutions, alternative protocols, and findings that defy consensus, the rigorous content of V2 finds itself at a growing disadvantage within the same distribution system. The datafication of medical authority, if generalized, undoes what the historical professional project has built over decades, which is credentials as a guarantee of competence, peer consensus as an arbiter of quality, and the fiduciary relationship as the foundation of public trust.

In Archer's scheme, this process constitutes structural elaboration ($T4$). The emerging properties of the medical field are being transformed, not reproduced. Beyond the datafication of medical authority described above, this elaboration is further documented by the erosion of the regulatory apparatus, whose enforcement capacity cannot match the scale and speed of the digital environment; and by the constitution of a parallel credentialing market, in which the platform functions as an alternative route to professional differentiation for physicians excluded from the traditional residency pathway. V2 operates in the opposite direction, seeking institutional morphostasis, but the conditions that sustain it depend on the same infrastructure that structurally favors V1.

The regulatory apparatus that sustains this collective authority is not, however, a neutral guardian of the MOP. The council has demonstrated, in episodes of supporting the use of amphetamines for weight loss and defending drugs with unproven efficacy during the Covid-19 pandemic, that it subordinates scientific evidence to corporate interests when convenient (Paumgartten, 2022). The regulatory gap reveals the temporal mismatch between the structural conditioning of $T1$ and the speed of social interaction in $T2-T3$, aggravated by the lack of internal regulatory consistency within the council itself. Resolution 2336/2023 identified the problems without creating enforcement mechanisms compatible with the scale and speed of the digital environment, and did so in a context in which the council itself is not a neutral agent in defining what counts as legitimate medical practice.

The structural conditioning (Archer, 2020) that underpins V1 will not be reversed by piecemeal regulatory adjustments as long as the gap between the expansion of medical training and the supply of residency positions persists, continuously creating a population of physicians who are not differentiated by the traditional route, for whom the platform remains a parallel market for credentials.

What the corpus data reveals goes beyond the behavior of individual physicians. The platformization of medical authority is an example of a larger trend where digital infrastructures reorganize specialized work, affecting not only precarious ones but also professions that have historically been protected by credentials and exclusive control. Platforms are organizations that simultaneously extract value from physicians' work, use medical authority as an input to monetize audience engagement, and produce behavioral data as an independent saleable product. In this context, Brazilian medicine presents itself as a paradigmatic example of the platformization of work. If a profession endowed with extraordinary social prestige, formalized corporate regulation, and legally protected jurisdiction cannot escape the logic of platforms, this exposes the profound capacity of this business model to capture, manage, and reshape the entire world of work.

7 CONCLUSION

This chapter draws together the analytical threads developed across the preceding six chapters to assess what the research produced, what it did not reach, and where the boundaries of its contribution lie. Section 7.1 restates the core problem and synthesizes the substantive theory of Performative Reconfiguration of Professional Authority. Sections 7.2 and 7.3 address the theoretical, methodological, and quality dimensions of the contribution, situating it within the relevant scholarly conversations. Section 7.4 traces the social, political, and regulatory implications of the findings for the medical profession and its oversight apparatus, while Sections 7.5 and 7.6 map the empirical and theoretical limits of the research and set out a forward-looking agenda. The chapter closes, in Section 7.7, with a reflection on what the evidence, taken together, reveals about the structural position of medicine under platform capitalism.

7.1 Restating the problem and synthesising the theory

This thesis demonstrated that, when fiduciary authority-based professions are subject to social media platforms' logic of performative credentialing, professional authority does not decline or transform; rather, it reconfigures itself continuously and performatively in dialectically opposing directions based on the agent's teleological orientation. This conclusion was established by investigating how physicians who simultaneously act as digital mega-influencers manage the tensions between the visibility imperatives of the platforms and the conduct norms of a highly institutionalized and legally protected profession.

The analytical interest lies not in individual practitioners but in the simultaneous subjection of the same agent to two professional regimes whose prescriptions, as Evetts's (2013) ideal types of professionalism establish, are structurally incompatible. Occupational professionalism is structured around collegial authority, discretionary judgment, and fiduciary behavior, whereas organizational professionalism is structured around hierarchical control, metric-driven performance, and commercial conversion of attention.

The investigation was organized around four specific objectives. The first characterized the disciplinary logic and competencies prescribed by Instagram, TikTok, and YouTube, establishing Platform Organizational Professionalism (POP) as a coherent organizational professionalism regime for content creators. The second reconstructed the normative-regulatory framework of the CFM governing physician engagement in digital media, designating it Medical Occupational Professionalism (MOP). The third established the

structural incompatibility between POP/Performative Credential and MOP/Fiduciary Authority Protection through cross-regime comparison of their respective prescriptions, identifying three axes of incompatibility (commodification vs. anti-commodification, visibility-driven vs. evidence-based content, and personal branding vs. sobriety) and producing the analytical condition the theory requires. The fourth constructed, from the data, the substantive grounded theory explaining how medical mega-influencers reconfigure medicine's institutional authority as a responsive performance under the condition of structural incompatibility.

The **Performative Reconfiguration of Professional Authority** explains that, given the structural impossibility of simultaneous adherence to POP and MOP, medical mega-influencers continuously reconfigure their authority as a responsive performance structured into two teleologically opposed vectors, either for commercial conversion or for the institutional preservation of the profession, always mobilizing the same operational instruments. At the stage of social interaction (*T2–T3*) in Archer's (2020) morphogenetic cycle, agents do not resolve the incompatibility between POP and MOP; they manage it.

This reconfiguration operates through two dialectical vectors that diverge in the priority each assigns to the competing regime. In Vector 1 (Converting Professional Authority into Commercial Assets), the commercial logic of POP subordinates MOP. Revenue shapes the content; the audience is seen as potential consumers, and medical authority is turned into a business asset by creating a sense of clinical insufficiency, questioning scientific consensus, fostering a parasocial connection with the audience, manufacturing a substitute expertise, and selling self-management health products. In Vector 2 (Professionalizing Health Communication), MOP subordinates the logic of POP. Revenue sustains content rather than organizing it, the audience is treated as a recipient of information, and platform tools are deployed to defend scientific consensus, combat misinformation, and reinforce public health institutions. Both vectors are available to every agent as responses to a single irreconcilable structural condition, and they are operationalized using the same Operational Mechanisms of Platform Adaptation (OMPA), whose function and teleology diverge depending on the vector in which they are deployed.

Because the incompatibility between the regimes is not resolved but instead reproduced, the process results in structural elaboration (*T4*) rather than closure. Vector 1 practiced at scale erodes the institutional conditions that make Vector 2 possible, while Vector 2 seeks morphostasis within the same infrastructure that structurally favors its opposite. Medical authority, in this process, ceases to be a static institutional attribute and becomes a

responsive performance that must be continuously revalidated before automated recommendation systems and audiences rather than before peers and state-delegated councils.

The empirical grounding of this theory, constructed from 400 platform instructional units, three CFM normative documents, and cross-platform traces of 26 medical mega-influencers, is what allows its contributions to be precisely located within the existing literature.

7.2 Theoretical and methodological contributions

The substantive theory advances the existing literature across four theoretical registers, each grounded in the corpus. The first concerns occupational professionalism and organizational professionalism, described by Evetts (2013) as ideal types of competing discourses that shape professional practice. She did not elaborate on the question of practical resolution when both regimes are simultaneously binding and mutually exclusive. The thesis answers this gap empirically by presenting these regimes as two vectors available to the same agent, operating under the same structural condition. The resolution is not a choice between one and the other, not an intermediate trajectory, nor a hybridization. The documented evidence shows that MMI performatively reconfigure authority in divergent directions according to the teleological orientation they adopt. This phenomenon is demonstrated by the fact that the same content formats and the same operational instruments are available for the production of opposite trajectories, depending on whether the agent subordinates occupational professionalism to organizational professionalism or organizational professionalism to occupational professionalism. The causal inversion between content and monetization is the criterion that empirically distinguishes the two vectors and rules out intermediate trajectories. In V1, content is organized to maximize the monetization of digital activity; in V2, monetization sustains the production of informative and educational content.

The second theoretical contribution concerns Abbott's (1981) analysis of professional impurity and its relationship to public prestige. The MMI instantiate a form of professional impurity that exceeds the analytical limits of Abbott's (1981) original formulation, as physicians of uncontested specialist standing descend to a frontline without triage, receiving an undifferentiated and algorithmically assembled public. What is analytically new is not the charisma derived from contact with social disorder, a pattern Abbott already described, but the platform's codification of that contact as a Performative Credential, decoupled from the institutional structures that previously governed both purity and prestige.

The third contribution addresses the commodification of specialized knowledge. Abbott (1988) identified commodification as an external threat to professional jurisdiction because it embeds expertise in products such as standardized forms or computer systems, eliminating the need for professional mediation and making knowledge available to non-specialists. V1 documents a condition that falls outside this definition, the one where members of a profession with intact formal jurisdiction, without external threat, voluntarily translate their knowledge into scalable products, whether in the form of their own info-products for buyers to self-manage their health or using scientific arguments to endorse nutritional supplements from sponsors. This is a voluntary commodification triggered by the platforms' parallel market for differentiation.

The Performative Credential is a central conceptual contribution. Identified in Phase 1 as the POP mechanism, it marks a structural transformation in how professional legitimacy is constituted. Unlike the traditional professional credential (CRM and RQE numbers and doctoral degree), the Performative Credential is validated by automated metric systems and has a short lifespan. In Abbott's (2005) terms, the traditional credential guarantees a stable jurisdiction while content creation operates as a settlement. The corpus provides concrete evidence that the performative credential surpasses the traditional credential on social media, given that among the most recently graduated MMI in the sample, digital performance metrics exceed those of specialists with decades of practice and doctoral degrees, enabling financial rewards substantial enough to reduce or selectively restrict clinical practice. Credential saturation explains who has structural incentives to enter platforms and why, not what they produce once there.

Methodologically, this thesis demonstrates that constructivist grounded theory (Charmaz, 2014) can be productively combined with critical realist ontology (Bhaskar, 2008; Archer, 2020) to study digitally mediated professional practice without recourse to interviews. The difficulty of obtaining direct access to the 26 MMI through conventional methods prompted a shift toward the analysis of extant digital traces. Examining video transcripts, platform bios, post captions, metadata, and advertising provided access to how physicians manage the tensions between incompatible professional regimes as that management unfolds in practice, capturing what subjects do under structural constraint rather than what they would retrospectively report when asked. This methodological displacement proved better suited to the study's analytical objectives than the originally planned interviews. Physicians managing dual-regime tensions in public would be unlikely to account candidly for that management when directly addressed.

Glaser's (2007) principle that all is data validated this approach. Properline data, strategically presented by participants to appear socially appropriate, reveal fundamental processes of social construction precisely because they are managed performances rather than unguarded admissions. The multi-source, multi-platform corpus, analyzed through iterative coding from 400 platform instructional units (Phase 1), three CFM normative documents (Phase 2), and 58 YouTube video transcripts combined with cross-platform data and metadata from 26 MMI (Phase 3), produced the conceptual density required for retroductive inference from the empirical domain of observable practices to the real domain of generative mechanisms.

7.3 Evaluating the theory: methodological quality criteria

Constructivist grounded theory requires the researcher to evaluate the quality of the generated theory based on the criteria of credibility, originality, resonance, and utility (Charmaz, 2014).

Credibility was built progressively across three dimensions. I developed familiarity with the field through sustained non-systematic observation of the 26 MMI across platforms before and during all analytical phases, following their content, recording patterns in memos, and attending to audience dynamics that systematic data extraction alone would not have captured. Familiarity with the structural conditions governing the field was established through the systematic analysis of phases 1 and 2, which mapped the organizational professionalism prescribed by platforms and the occupational professionalism prescribed by the CFM, respectively, providing the conceptual ground necessary to recognize theoretically relevant patterns in the subsequent phase. Familiarity with the data was achieved through iterative coding of cross-platform profiles, metadata, and video transcripts in phase 3, with theoretical saturation confirmed across successive rounds of analysis.

Originality manifests in three dimensions. Substantively, the thesis investigates a phenomenon (the platformization of a legally protected, high-prestige profession) that had not been systematically theorized in the Brazilian academic literature. Theoretically, no established precedent exists in the literature on professions and platforms for combining critical realism with constructivist grounded theory to generate a substantive theory of digitally mediated professionalism. Methodologically, the use of naturalistic digital traces across three platforms to map structural conditions (*T1*) and their instantiation in professional practice (*T2–T3*), without interview data, represents a demonstrably viable path for grounded theory research on agents who are publicly visible but institutionally inaccessible.

Resonance, in Charmaz's (2014) formulation, requires that the theory illuminate tacit processes and capture aspects of participants' experience that they themselves may not fully articulate. The Performative Reconfiguration of Professional Authority theory achieves this goal in two ways. The first manifests in the analytical work already described in Section 7.2: the Performative Credential makes explicit a dynamic that the corpus evidences but does not name. Second, the vector structure captures a tension that is structurally imposed rather than individually chosen, generated by the causal powers of the platform operating upon the preexisting institutional structure of the medical profession. No agent in the corpus fully escapes the pull of both regimes, and the same OMPA instruments appear across both vectors precisely because the conditioning is shared. The theory renders visible what the agents' practices enact but cannot, within the constraints of digital performance, make explicit.

Utility is assessed by asking whether the theory serves to understand a context beyond the immediate one of the research (Charmaz, 2014). The conceptual vocabulary developed here, the Performative Credential, the teleological divergence of OMPA instruments, and the two-vector structure of reconfiguration, provides an analytical apparatus for investigating how any credentialed profession navigates the simultaneous occupation of organizational and occupational professionalism logics, a condition that is becoming increasingly general as digital platforms extend their governance into professional practice.

7.4 Social, political, and regulatory implications

The structural elaboration documented at T4 of the morphogenetic cycle, the datafication of medical authority, the erosion of the regulatory apparatus, and the constitution of a parallel credentialing market, produces implications that extend beyond the medical profession as an institution to encompass direct public health risk and the broader transformation of specialized work.

The datafication of medical authority constitutes a direct public health risk that the CFM's current regulatory framework addresses primarily as a problem of unfair competition and professional marketing ethics, rather than as a problem of health information quality and patient safety. Vector 1 systematically pathologizes ordinary bodily states, constructs clinical insufficiencies where none exist, and funnels the resulting anxiety toward the purchase of products (supplements, infoproducts, and alternative protocols) that are distributed outside the regulatory perimeter of the CFM and in possible violation of Anvisa's standards for nutritional supplement advertising.

A report by NetLab–UFRJ found that 76% of the health advertisements analyzed were fraudulent, frequently deploying white-coat imagery to sell medically unsubstantiated solutions (Santini *et al.*, 2025). The corpus of this research documents the structural conditions that make this ecosystem possible, platform recommendation systems that amplify emotionally intense content regardless of its scientific accuracy, regulatory gaps that leave supplement endorsement by physicians in an ambiguous space, and self-management protocols designed for asynchronous, scalable consumption that eliminate the case-by-case discretion that is inherent in the medical act.

Sociologically, this arrangement does not produce a premium audience, as Vector 1's commercial logic would suggest. The corpus documents a stratified access structure with three empirically distinct tiers: free platform content at the top of the funnel, infoproducts at the intermediate tier, and direct clinical consultation at the bottom, accessible only by higher-ticket clients (see section 6.4.2). Individuals in the lower tiers receive a product whose scalability is achieved by eliminating the fiduciary relationship and the case-by-case discretion that professional ethics require, and whose distribution behind paywalls and proprietary areas places it outside the ordinary scope of CFM monitoring.

The inadequacy of the current regulatory response is structural, besides operational. CFM Resolution No. 2336/2023, though a significant modernization effort, attempts to regulate intention in an environment whose governance logic regulates behavior through metrics and incentives. The mismatch is threefold.

First, the resolution's enforcement mechanism requires that violations be reported by identified individuals, a design that structurally discourages reporting in a digital environment where violations are public, continuous, and occur at a scale that far exceeds the inspection capacity of regional councils. As documented in this thesis, the number of CFM councilors responsible for judging ethical proceedings (28 titular members) has remained constant, while the profession has undergone a staggering recent expansion, reaching nearly 600K professionals in 2024 (Scheffer, 2025).

Second, the procedural model of compliance (requiring the display of CRM numbers, structured content formats, and disclaimers) creates an impunity loophole, as formal compliance can coexist with substantive violation. Third, the resolution does not address the supplement endorsement gap. The CFM prohibits physician participation in advertising that implies guaranteed results for medications and food products but does not classify nutritional supplements as food, a classification that belongs to Anvisa's regulatory framework. Closing this gap requires a cross-institutional reading that neither body explicitly mandates, leaving a

commercially lucrative and epistemically consequential domain without effective regulatory coverage.

The structural incompatibility documented here points to a reorientation of the CFM's governance model toward what Hofmann, Katzenbach, and Gollatz (2017) describe as reflexive coordination, a posture focused on critical moments when routine activities become problematic and require revision, rather than containing a dynamic environment with static prohibitions. This reorientation implies, at minimum, developing monitoring capacity commensurate with the scale of digital medical communication and closing the supplement endorsement gap through cross-institutional coordination with Anvisa. The empirical foundations of these interventions, however, remain undertheorized and constitute a distinct research agenda.

The third and broadest implication concerns the platformization of specialized work. The evidence assembled in this thesis supports the argument, originally advanced in the introduction, that the platformization of labor tends toward generalization (Grohmann, 2021; Casilli; Posada, 2019), extending to specialized, credentialed work. The theory of the Performative Reconfiguration of Professional Authority is limited to this investigation of Brazilian medical mega-influencers; however, the phenomenon examined is not exclusive to medicine. The presence of other regulated health professionals, such as nutritionists, psychologists, and dentists, in the same HypeAuditor rankings from which the corpus was selected, alongside documented cases in other professions, like judges leaving the magistracy to pursue full-time content creation (Andrade, 2023), suggests that the Performative Reconfiguration of Professional Authority may operate as a transversal process across regulated professions, not as a phenomenon specific to medicine.

7.5 Limitations of the research

This research was designed to maximize the analytical depth of a specific phenomenon, the management of structural incompatibility between POP and MOP by physicians operating at the top of the creator economy in Brazil. This design produced a substantive theory with considerable empirical grounding and theoretical reach, but it also defines the boundaries within which the theory's claims are valid.

The theory is substantive, not formal. It was generated from a specific group, 26 medical mega-influencers in Brazil, at a specific historical moment. The dynamics documented here may differ for physicians who are nano- or micro-influencers, who are just beginning to build a digital presence, and who do not yet have the accumulated audience trust

that makes commercial conversion viable. The structural incentives faced by a physician with 50K followers and no existing specialty credential are not identical to those of a mega-influencer whose Performative Credential exceeds the market value of their RQE, fundamentally clashing with the Fiduciary Authority Protection demanded by Medical Occupational Professionalism. Testing the theory in these adjacent contexts is necessary before extending its claims to them.

The research adopted a design focused on the production of professional identity and authority as it is publicly staged, using naturalistic digital traces such as transcripts, metadata, institutional documents, and cross-platform profiles. This approach strengthened the observation of the performative enactment of authority and provided access to self-presentation as it occurs rather than as it is retrospectively reported. It also delimits the theory's scope, as the Performative Reconfiguration of Professional Authority does not encompass the reception side of the phenomenon. How the audience, patients, potential patients, caregivers, and the lay public actually process, evaluate, and act upon the information and products circulating through these channels remains outside the empirical reach of this research.

The closed WhatsApp and Telegram groups accessed during data construction were unilateral distribution channels, as physicians and their teams posted content that followers received without the possibility of public response. These groups provided access to the supply of products and content, not to the demand side. The question of how the contemporary health consumer weighs the Performative Credential against the traditional credential, whether millions of followers or a hospital affiliation carries greater epistemic authority in a moment of health decision-making, is both theoretically important and empirically unaddressed by this thesis.

Another limitation of this thesis is its explanatory scope, which is restricted by the methodological design. The generative mechanisms this research identified, platform governance and CFM normative regulation, are those operating within its delimited empirical scope. The domain of the Real, however, extends beyond what any single study can observe, as Bhaskar's (2008) stratified ontology establishes. This investigation therefore did not intend, nor would it have been possible, to examine all generative mechanisms producing structural tensions within the Brazilian medical profession, whether rooted in history, in labor practices, or in physicians' relations with other organizations such as educational institutions, private health insurers, hospitals, and the state.

Another limitation concerns the intermediary actors that shape conditions of medical visibility. This thesis analyzes that relationship as its primary axis, but medical mega-influencers' visibility is also mediated by medical marketing agencies, reach amplification services, advertising platforms, and other intermediaries whose role in shaping the conditions of digital medical authority is beyond the empirical scope of this study. Investigating these actors and their interactions would broaden the analysis from a bilateral to a fully networked view of medical visibility production and monetization.

Finally, the choice of Brazil as the empirical field, though analytically justified by the specific conditions of credential saturation and market expansion that intensify the structural incentives for platformization, raises the question of contextual specificity. The Brazilian model of professional regulation, organized through state-delegated councils with compulsory membership rather than through voluntary associations, may produce distinct patterns of normative compliance and impunity that do not fully transfer to other national contexts.

7.6 Agenda for future research

The limitations of this thesis also indicate its contributions to future research. Four directions merit priority attention. The first one is to elevate the Performative Reconfiguration of Professional Authority from a substantive to a formal theory by conducting comparative analysis across professions. Lawyers, architects, healthcare professionals in general, and accountants all possess institutionalized credentials, collegial regulatory bodies, and jurisdictional claims that are structurally analogous to those of medicine, though with different degrees of social closure and market concentration. Investigating whether the Performative Credential operates as a general mechanism of authority in platformized professional practice and whether the vector structure of Performative Reconfiguration of Professional Authority reproduces in these fields would allow the substantive theory generated here to be tested against cases with different institutional configurations. The documented presence of regulated health professionals in the same platform rankings from which this corpus was extracted provides a natural starting point for comparative research within the health field.

The second direction concerns the reception side of the phenomenon, specifically how audiences, patients, and health consumers actually evaluate and act upon the information and products generated by medical mega-influencers. The contemporary health consumer navigates a media environment in which the Performative Credential, millions of followers, production quality, parasocial intimacy, and digital accessibility compete with the traditional

credential, the ‘white coat,’ the hospital affiliation, and the doctoral degree as signals of epistemic trustworthiness. Research on this question would require direct engagement with audiences through interviews, surveys, or experimental designs and would complement the production-side analysis of this thesis by completing the circuit through which ‘datafied’ medical authority has its most consequential social effects.

The third direction involves physicians who are entering the digital space without the accumulated authority of established specialists. The platform may function not as a supplement to an existing professional career but as the primary site of professional identity construction for physicians who graduated in a saturated market without a registered specialty. The dynamics of Performative Reconfiguration of Professional Authority for this cohort, for whom the Performative Credential may precede rather than convert a traditional credential, are likely to differ structurally from those documented among mega-influencers and deserve dedicated investigation.

The fourth direction is the regulatory response itself as an object of sociological inquiry. This thesis documents the structural inadequacy of the CFM’s current approach but does not explain the organizational, political, and institutional conditions that produce and sustain it. Research questions include how the council manages its dual role as regulatory body and corporate interest group; under what institutional conditions cross-agency coordination between the CFM and Anvisa becomes feasible; and whether the enforcement gap is better characterized as an absence of political will, a resource constraint, or a structural consequence of the council’s constitution. These questions require methods, such as organizational ethnography and elite interviews, that fall outside the scope of this thesis.

7.7 Final considerations

This thesis began with a puzzle about why a physician with a legally protected jurisdiction, a formally credentialed monopoly over one of the most universally demanded services in human society, and a century of institutional consolidation would choose to submit to the governance logic of a private platform, to optimize their content for algorithmic recommendation, to build a personal brand through parasocial intimacy, and to convert their clinical authority into a commercial asset. The answer, this research argues, is that the choice is not entirely free. The structural conditions that produce the saturation of the Brazilian medical market, the surplus of credentialed professionals lacking specialty differentiation, the emergence of platforms as alternative credential markets, and the invisible management that

platforms exercise over all content creators, regardless of their primary occupation, create a generative mechanism that conditions without determining the response of individual agents.

What the agents do with this condition is what the theory of Performative Reconfiguration of Professional Authority explains. They do not resolve the incompatibility between regimes; they reconfigure authority performatively, continuously, and in dialectically opposing directions, depending on the teleological orientation they adopt. The same platforms, the same instruments, and the same professional markers produce opposite effects, functioning in one direction as a conversion machine that ‘datafies’ clinical authority into supplements, protocols, and infoproducts, and in the other as a broadcasting infrastructure that physicians with accumulated institutional authority use to defend scientific consensus at a scale that no traditional media appearance could match.

The structural elaboration that results from this process, the datafication of medical authority, the emergence of a substitute validation system, and the erosion of the conditions that make Vector 2 possible through the generalization of Vector 1 are not the product of individual malfeasance. It is the emergent effect of a business model that extracts value from all content without distinguishing between the physician who uses their credential to sell magnesium protocols and the physician who uses the same credential to fight vaccine misinformation. Platforms profit from both types of physicians, and their recommendation systems amplify content from both according to the same criterion: engagement. This is the sense in which the datafication of medical authority is a structural condition, not a failure of individual ethics. Anchored in the regulation of individual intent and procedural compliance, the CFM’s response targets the wrong level of the phenomenon.

If a profession that has a century-long history of successfully defending its market against competitors, formalized corporate regulation, legally protected jurisdiction, and extraordinary social prestige is unable to evade the governance logic of platforms, if even the archetype of professional closure is being reconfigured by engagement metrics, then the ability of the platform business model to capture, manage, and reshape the world of specialized work is far more profound than existing theories of professions, labor, and organizations have yet acknowledged. Its expansion into previously unaddressed domains, such as public health, may be just the beginning.

There is, however, a dimension of this transformation beyond what the evidence assembled here can demonstrate, yet its theoretical framework makes it legible. As explained in Chapter 3, professions as a social form became possible when knowledge became a sociocultural authority independent of religious and monarchical institutions and a capitalist

market allowed the private provision of knowledge-based services. Both conditions presuppose public trust in scientific knowledge as a legitimate basis for professional authority. That trust is not a natural given. This trust is a social achievement that has been historically constructed and institutionally maintained; its erosion would threaten the professions as such, not merely their organizational forms. Platformization accelerates an epistemic delegitimation that social media did not originate but amplifies to an unprecedented scale. A medical mega-influencer who uses Vector 1 to contest laboratory reference ranges or vaccination efficacy not only violates a professional norm, but also degrades the epistemic foundation on which all professions are based.

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